

ACTIVITIES BY PHASE

Who leads each activity in each phase

Every activity has a West Monroe lead and a named Northfield owner, so decisions and handovers never wait on a role that is unclear.

HOW WE WORK

Ownership rule

- One owner on each side
- Owner signs the phase gate
- Escalate within two days

ACTIVITY	PHASE	WEST MONROE	NORTHFIELD	WKS
Application scoring	RATIONALIZE	Portfolio lead	App owners	4
Disposition workshops	RATIONALIZE	Strategy lead	CIO office	4
Target design	RATIONALIZE	Architect	Architecture	4
Funding case	RATIONALIZE	Lead partner	Finance	2
Landing zone	BUILD	Cloud lead	Infrastructure	12
Billing refactor	BUILD	Tech lead	Revenue cycle	10
Data reconciliation	BUILD	Data lead	Data stewards	12
Wave cutover	BUILD	Delivery lead	Patient access	6
Hypercare	SUSTAIN	Support lead	Service desk	12
Quarterly tuning	SUSTAIN	Cloud lead	Infrastructure	4
Governance charter	SUSTAIN	Strategy lead	CIO office	4
Modernization backlog	SUSTAIN	Portfolio lead	Product leads	Ongoing

ADR

014

STATUS

ACCEPTED

DATE

12 Sep 2026

DECIDERS

Architecture review board

SUPERSEDES

ADR-009 Point-to-point HL7 feeds

ARCHITECTURE DECISION RECORD

ADR-014: Route clinical events through a shared bus

CONTEXT

Northfield runs 14 clinical and revenue systems, and every pair that exchanges data has its own interface. The 61 feeds fail in different ways, and each new system adds more.

DECISION

Publish clinical and revenue events to one managed event bus. Systems subscribe to the events they need, so a new feed no longer changes the sending system.

POSITIVE CONSEQUENCES

- One place to monitor and replay every event
- New systems connect in weeks, not quarters

NEGATIVE CONSEQUENCES

- The platform team must run a new service around the clock
- Interface teams need training in event design

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ADR-009 Point-to-point HL7 feeds

Review in 12 months or when the platform team changes.

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POSITIVE CONSEQUENCES

- One place to monitor and replay every event
- New systems connect in weeks, not quarters
- Failed deliveries are visible in one dashboard

NEGATIVE CONSEQUENCES

- The platform team must run a new service around the clock
- Interface teams need training in event design
- Short period of running both models

ADR-014: Route clinical events through a shared event bus

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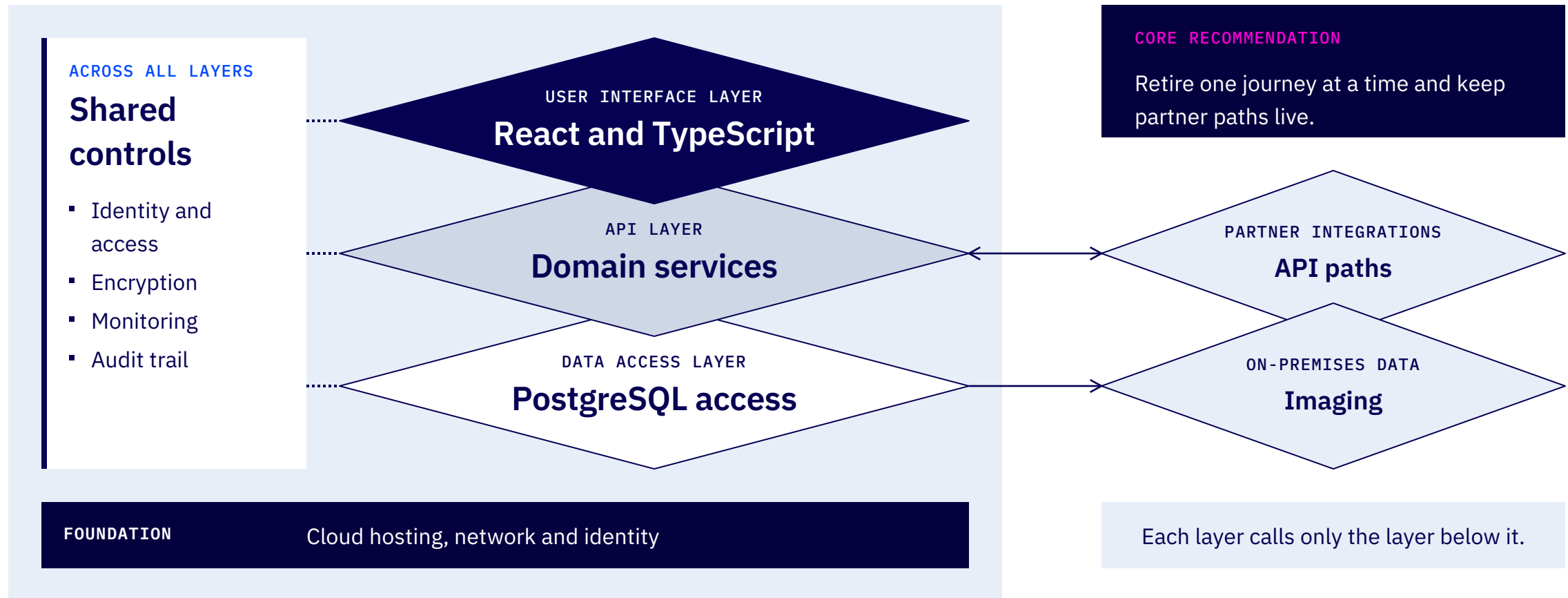
Review in September 2027, or sooner if feed incidents rise.

Agenda for the design-sprint day

TIME	SESSION	FINANCE AND DATA CONTRIBUTION	SHARED RESULT
8:30–9:30	1. Joint framing	- Finance: bring close calendar and pain points - Data: test feasibility and source availability	One scoped opportunity with a visible path
9:35–11:30	2. Parallel definition	- Finance: define rules, controls, acceptance - Data: profile sources and prove the data flow	Agreed rules and a working data sample
12:15–1:00	3. Handoff	- Finance: explain intent and open questions - Data: challenge ambiguity; propose a slice	Agreed interface, scope and demo slice
1:00–3:15	4. Build and review	- Finance: review each increment against rules - Data: convert feedback into bounded changes	Traceable feedback and acceptance evidence
3:15–4:30	5. Scale-up	- Finance: name operating-model implications - Data: name platform and ownership implications	Next actions and a reusable learning record

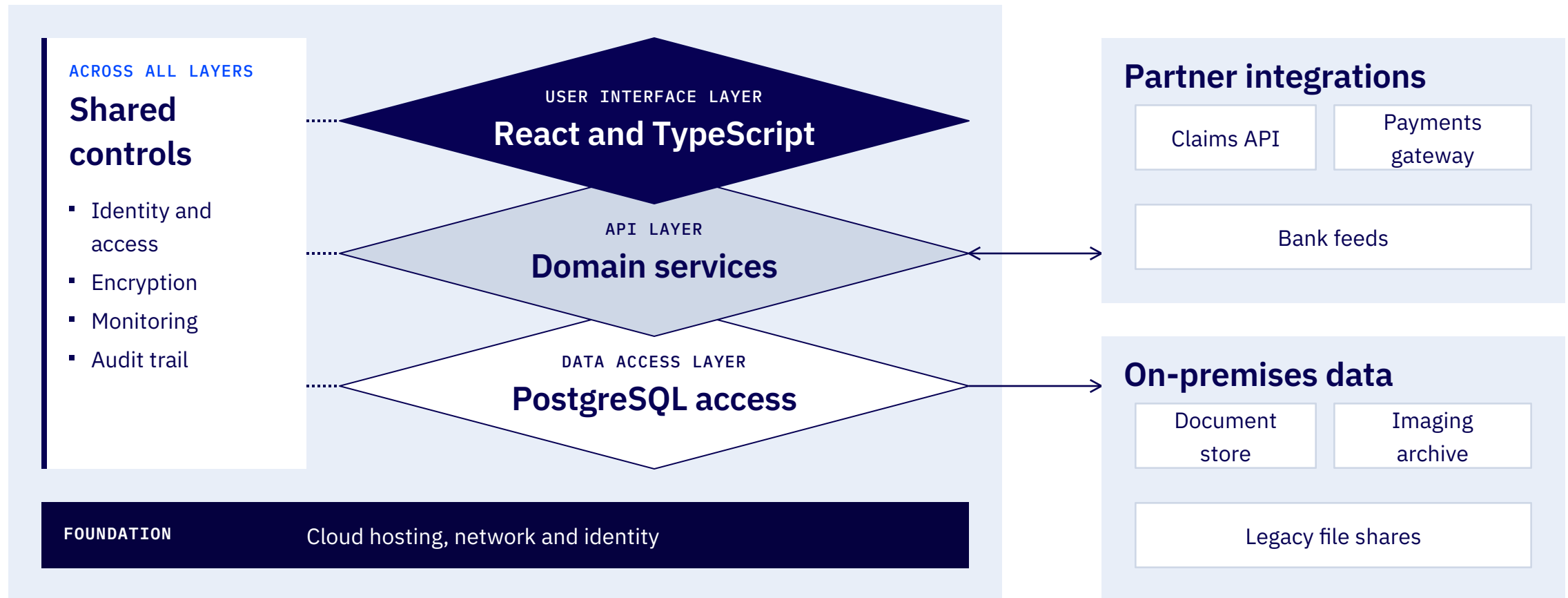
Outcome of the day: one agreed wave 1 scope, a tested data slice and a named owner for each decision.

Three tiers let us change one layer in isolation



Source: West Monroe reference architecture.

Three tiers change in isolation and reach other systems



Source: West Monroe reference architecture.

One layered platform replaces the old close tools

CLOSE PLATFORM

Build once

Each layer serves every close process, so one moves without a rebuild.



Experience

Close cockpit and mobile approvals



API layer

Business rules and posting services



Data access

One governed path to close data



Source systems

ERP, sub-ledgers and bank feeds



Foundation: cloud landing zone, identity, network and storage



ACROSS LAYERS 1-4

Shared controls

- Security
- Governance
- Observability
- Audit trail

Source: West Monroe reference architecture.

SOLUTION ARCHITECTURE

One layered platform replaces the old close tools

Build once

Each layer serves every close process, so one moves without a rebuild.

ACROSS LAYERS 1-4

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Source: West Monroe reference architecture.

CLOSE PLATFORM

1 EXPERIENCE

Close cockpit and mobile approvals

2 API LAYER

Business rules and posting services

3 DATA ACCESS

One governed path to close data

4 SOURCE SYSTEMS

ERP, sub-ledgers and bank feeds

FOUNDATION

Cloud landing zone, identity, network and storage

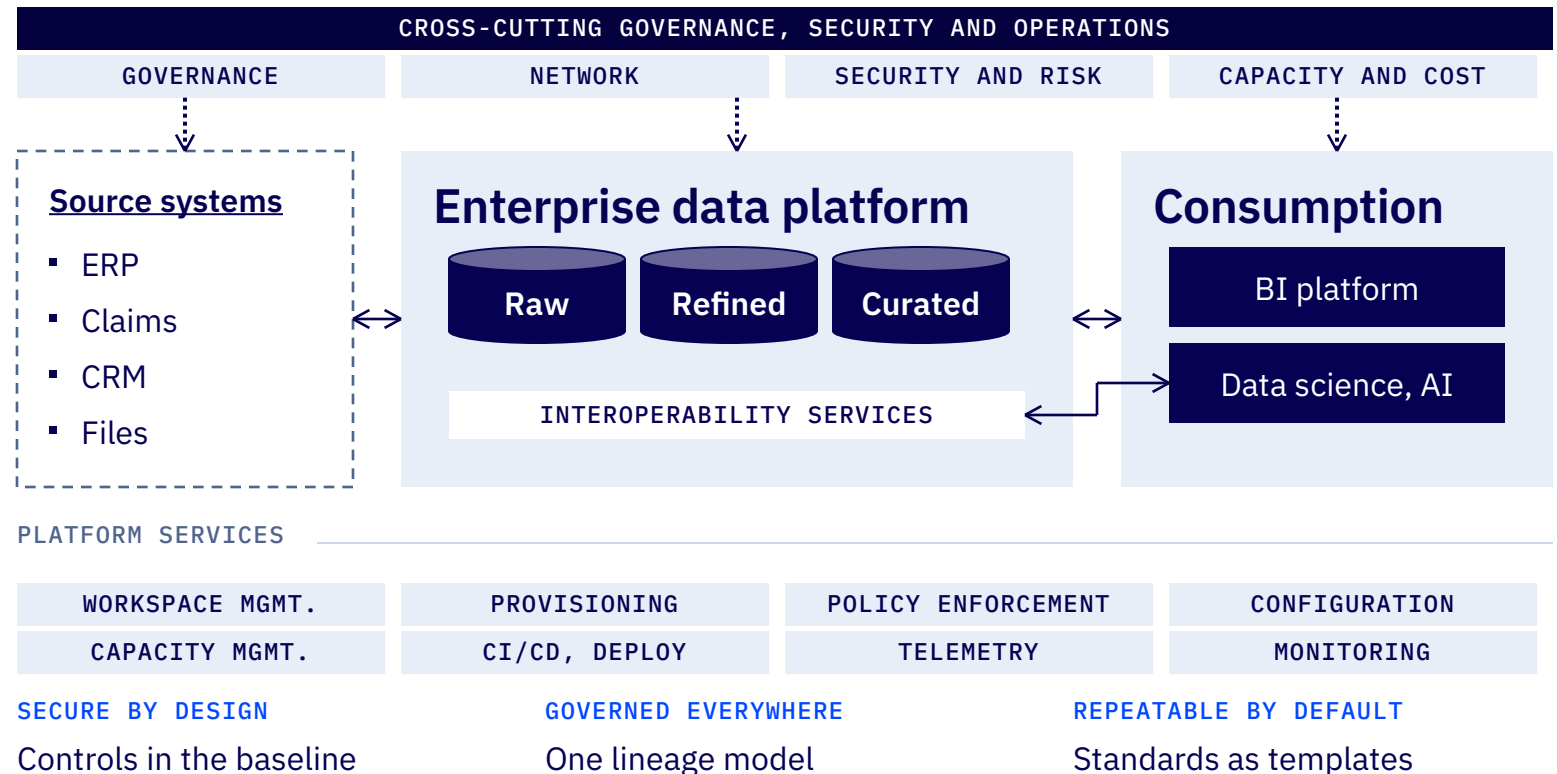
A secure, governed reference architecture lets each workload land on the right platform

Phase 1 proof points

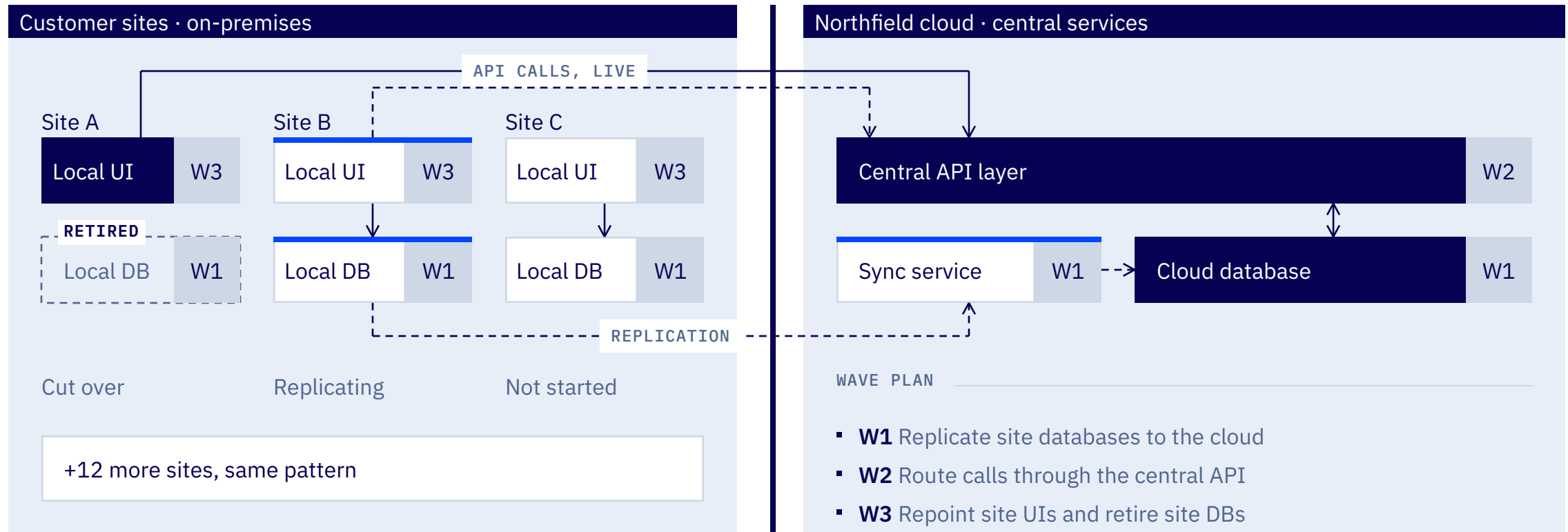
- Workspace and environment pattern approved
- Code promotion demonstrated
- Lineage metadata visible for each store
- Telemetry externalized to shared monitoring
- Priority patterns demonstrated

Dependency disposition

- Gaps logged with owner and interim treatment
- Impact on general availability stated
- Reviewed at each steering meeting

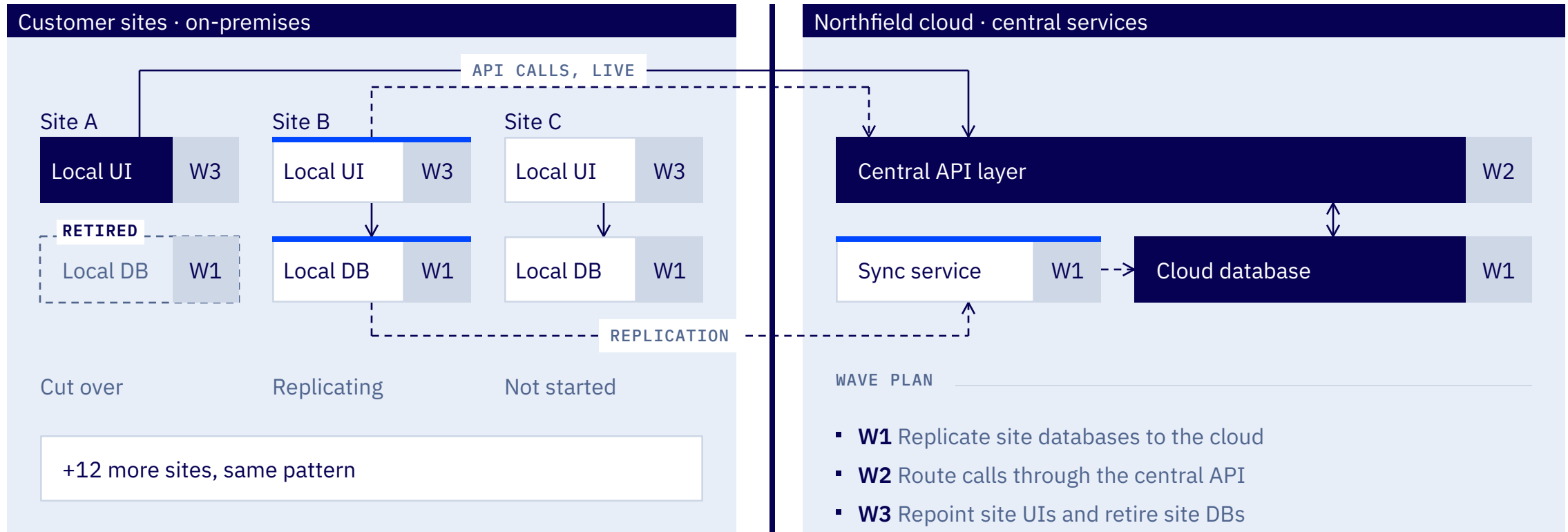


Data and API move to the cloud while UIs stay on site



Legacy / on-prem
Coexisting
Migrated
Retired / planned
W1 Wave
 -----> Sync / routing

Data and API move to the cloud while UIs stay on site



One row carries the largest gap to target

CAPABILITY	MAIN	LAKESIDE	RIDGEVIEW	HARBOR
Patient access	Small	Moderate	Moderate	Large
Scheduling	Moderate	Moderate	Large	Large
Revenue cycle	Small	Small	Moderate	Large
Documentation	Small	Moderate	Moderate	Moderate
<u>Care coordination</u>	Moderate	Moderate	Large	Critical
Supply chain	Small	Moderate	Large	Large
Data and analytics	Moderate	Moderate	Large	Large

- Nine other cells sit in the large band, all at Ridgeview or Harbor.
- Main campus has no gap above moderate.

BIGGEST GAP

Care coordination at Harbor

Referral status lives in faxes and phone notes, so no one sees a patient's whole path.

No gap Small Moderate Large Critical

Source: Northfield Health System assessment, Sep 2026.

Three gaps drive most of the distance to target

CAPABILITY	MAIN CAMPUS	LAKESIDE	RIDGEVIEW	HARBOR
Patient access	Small	Moderate	Moderate	Critical
Scheduling	Moderate	Moderate	Critical	Critical 2
Revenue cycle	Small	Small	Moderate	Critical
Documentation	Small	Moderate	Moderate	Moderate
Care coordination	Moderate	Moderate	Critical	Critical 1
Supply chain	Small	Moderate	Critical	Critical
Data and analytics	Moderate	Moderate	Critical 3	Critical

No gap
Small
Moderate
Large
Critical

Source: Northfield Health System assessment, Sep 2026.

01

Care coordination at Harbor

No shared care plan or referral tracking; clinics hand off by phone and fax.

02

Scheduling at Harbor

Templates differ by clinic, so open slots never reach the central call center.

03

Data and analytics at Ridgeview

Reports are built by hand each month and cannot be compared with other sites.

-0.5

off the weighted gap score.

CAPABILITY ASSESSMENT

The front door is the weakest group

- **Strong core.** Revenue rows reach level 3 or better at the main campus.
- **Thin edges.** Harbor sits at level 1 or below in most front-door and care rows.

WEAKEST GROUP

1.7

is the four-site average for the front door, the lowest of the four groups.

	CAPABILITY	MAIN	LAKE SIDE	RIDGEVIEW	HARBOR
FRONT DOOR	Patient access	3	2	1	1
	Scheduling	3	2	2	1
	Referral management	2	2	1	0
REVENUE	Patient billing	3	3	2	1
	Revenue cycle	4	3	3	2
	Denials management	3	2	1	1
CARE DELIVERY	Clinical documentation	4	3	2	1
	Care coordination	2	2	1	0
	Supply chain	3	3	2	2
	Quality reporting	3	2	1	0
	Population health	2	2	1	0
ENTERPRISE	Workforce management	3	2	2	1
	Data and analytics	3	2	1	1
	Cybersecurity	4	3	3	2

■ Absent
■ Ad hoc
■ Defined
■ Managed
■ Optimized

Source: Northfield Health System assessment, Sep 2026.

CAPABILITY ASSESSMENT

Strong at the core, thin at the edges

- **Strong core.** Revenue cycle and documentation reach level 3 or better at the main campus.
- **Uneven spread.** Only the main campus holds a level 3 or 4 in more than half the rows.
- **Thin edge.** Harbor has no capability above level 2, and two sit at level 0.

LIFT FIRST

3

capabilities at Ridgeview and Harbor.

CAPABILITY	MAIN CAMPUS	LAKE SIDE	RIDGEVIEW	HARBOR
Patient access	3	2	1	1
Scheduling	3	3	2	1
Revenue cycle	4	3	2	2
Clinical documentation	4	3	3	2
Care coordination	2	2	1	0
Supply chain	3	2	1	1
Workforce management	2	2	1	1
Data and analytics	3	2	1	0
Cybersecurity	3	3	2	1
Patient billing	3	2	2	1
Quality reporting	3	3	2	1

Absent
Ad hoc
Defined
Managed
Optimized

Source: Northfield Health System assessment, Sep 2026.

PATIENT APP CAPABILITIES

Access is live; care and billing carry the backlog

- **Live** booking, billing basics and the medication list.
- **In build** care plans, messaging and payment plans.
- **Open** kiosk, wayfinding and the community programs.

■ Live

■ In build

■ Planned 2026

□ Not started

Access

Find a provider

Self-scheduling

Waitlist and cancellations

Virtual visit join

Pre-visit forms

Kiosk check-in

Wayfinding

Care

Care plans

Medication list

Lab results

Secure messaging

Remote monitoring

Visit summaries

Allergy list

Billing

Statements

Pay online

Payment plans

Cost estimates

Insurance card capture

Financial aid

Autopay

Engagement

Reminders

Proxy access

Feedback surveys

Health content

Community programs

Notifications

Language choice

One model connects customers, capabilities and value

Customer	Patients			Payers		Employers		
	Self-pay · small balance			Commercial · large		Government · largest		
Channels	Portal	Phone	Text	Email	Mail	Kiosk	In person	
Enabling capabilities	Eligibility and intake			Charge capture		Claim status tracking		
	Registration and scheduling			Coding and edits		Denial workflow		
	Patient accounting platform							
Content and data	Enterprise data platform					Revenue analytics		
	Patient	Charge	Contract	Remit	Statement	Payroll	Ledger	
Operating model	Integrated channels			Revenue management		Organizational agility		
	Integrated technology			Message personalization		Process innovation		
Value drivers	CASH ACCELERATION - Days in A/R - Clean-claim rate			COST TO COLLECT - Touches per account - Rework volume		PATIENT EXPERIENCE - Billing clarity - Pay-by-text uptake		

Every layer has a clear build owner



Source: Sample names for layout only; replace with the client product inventory.

Sprints 4 to 6 carry the heaviest commitments

TEAM	S1	S2	S3	S4	S5	S6	S7	S8	CAPACITY
Scheduling	24	28	32	34	36	36	30	22	32
Patient portal	20	24	28	30	34	38	34	26	30
Revenue cycle	16	22	26	30	32	34	30	24	28
Integration	28	32	36	40	42	40	34	24	36
Data platform	12	16	20	24	28	30	26	18	26

STORY POINTS Under 5 5–14 15–24 25–34 35 or more

01 Sprint 4 to 6

Every team commits above 24 points while go-live prep peaks.

02 Integration

Commits above its 36-point capacity in sprints 4 to 6.

03 Sprint 8

Load eases to 26 points or fewer everywhere before the freeze.

Source: Northfield delivery plan, Sep 2026.

The run-rate ramps to \$210K a month, then holds

Team run-rate by month

TEAM MONTHLY RUN-RATE



- 01 Ramp-up, months 1–2**
Core team starts; specialists join as workstreams open.
- 02 Steady state, months 3–6**
Full team at \$210K a month.
- 03 Roll-off, months 7–8**
Capacity steps down as Northfield takes over.

\$1.32M

estimated total, eight months

Six workstreams deliver the five-day close

WHY IT MATTERS

A close that runs on rules

Close takes 12 days because work waits for people. We resequence it so entries are checked as they post.

01 Close calendar

One calendar for 14 business units, built backward from day five.

02 Reconciliations

Accounts reconciled during the month, with exceptions routed on day one.

03 Intercompany

Overnight matching replaces email and spreadsheets across eleven entities.

04 Review

Rules check entries as they post; controllers see only the exceptions.

05 Reporting

Flash results ready on day three, the full package on day five.

06 Controls

Evidence is captured as work happens, so audit asks stop reopening the close.

Source: West Monroe close calendar analysis, FY26, 14 business units.

Eight changes deliver the five-day close

WHY IT MATTERS

A close that runs on rules

Close takes 12 days because work waits for people. We resequence it so entries are checked as they post.

01 Reconcile

One template and owner for every balance-sheet account.

02 Matching

Intercompany balances match overnight across eleven entities.

03 Early review

Controllers review entries as they post, not in one push on day nine.

04 Ownership

Every close task has a named owner, a due time and an escalation path.

05 Flash report

Flash results are ready on day three and the full package on day five.

06 Controls

Evidence is captured as work happens, so audit requests stop reopening the close.

07 Calendar

One calendar for 14 business units, built backward from the day-five target.

08 Exceptions

Breaks reach the right owner the same day, with the supporting evidence attached.

Four groups of change deliver the five-day close

WHY IT MATTERS

A close that runs on rules

Close takes 12 days because work waits for people. We resequence it so entries are checked as they post.

CALENDAR

01 Calendar

One calendar, built back from day five.

02 Exceptions

Breaks reach an owner the same day.

RECONCILIATION

03 Reconcile

One template and owner per account.

04 Matching

Intercompany balances match overnight.

REVIEW

05 Early review

Entries are reviewed as they post.

06 Ownership

Every task has an owner and a due time.

REPORTING

07 Flash report

Flash on day three, full pack on day five.

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PEOPLE AND GOVERNANCE

03 Early review

Controllers review entries as they post, not in one push on day nine.

04 Ownership

Every close task has a named owner, a due time and an escalation path.

05 Flash report

Flash results are ready on day three and the full package on day five.

06 Controls

Evidence is captured as work happens, so audit requests stop reopening the close.

CLIENT

Northfield Health System

Four-entity hospital group

CHALLENGE

A 15-day close, with intercompany balances matched by email.

RESULT

6 days

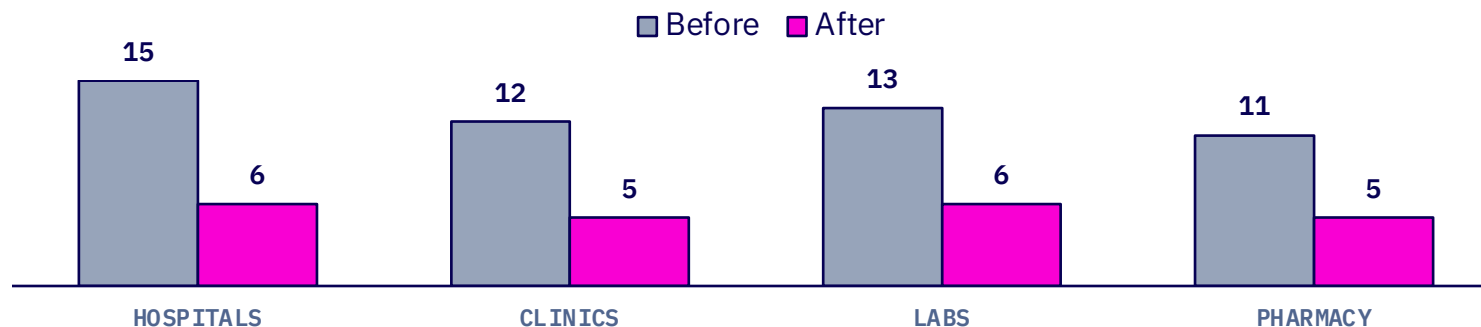
to close, from 15

CASE STUDY • HEALTHCARE

Every entity now closes in 6 days

Days to close, before and after

DAYS TO CLOSE



WHAT WE DID

- Automated reconciliations for the busiest accounts.
- Set rules at posting, so errors stop before review.
- Moved review to day one, entity by entity.

RESULTS

92%

matches automated

\$2.1M

saved a year

Source: Sample figures for layout only; replace with current firm data before use.

Each audience hears a message built for its day-to-day

01 Physicians

MESSAGE

What changes for my schedule

CHANNEL

Huddle and short video

CADENCE

Every 2 weeks

OWNER

Medical director

02 Frontline staff

MESSAGE

Steps, dates, who to call

CHANNEL

Floor-walkers, huddle cards

CADENCE

Weekly

OWNER

Practice managers

03 Leaders

MESSAGE

Progress, risks, decisions

CHANNEL

Steering briefing, dashboard

CADENCE

Monthly

OWNER

Program sponsor

04 Patients

MESSAGE

How to book, what to expect

CHANNEL

Email, text, lobby signs

CADENCE

From T-4 weeks

OWNER

Patient experience

Rule: staff hear from their own manager first.

Source: Northfield Health System communications plan, Sep 2026.

Six questions staff ask, with straight answers

01 Will I lose my job?

No roles are eliminated. Some tasks move; the role cards show which.

WHERE TO LOOK

Role cards on the intranet

02 When is my training?

Eight hours in the four weeks before your site goes live, scheduled by your manager.

WHERE TO LOOK

Your manager's schedule email

03 If the system is down?

A paper downtime kit and a hotline run for the first two weeks.

WHERE TO LOOK

Downtime kit at every desk

04 Who do I call?

Your floor-walker first, then the service desk hotline.

WHERE TO LOOK

Number on the huddle card

05 Keep my old templates?

Templates are rebuilt once by each clinic; your lead signs off.

WHERE TO LOOK

Template sign-off sheet

06 How will patients know?

Email, text and lobby signs begin four weeks before your site goes live.

WHERE TO LOOK

Lobby signs and website

Source: Northfield Health System change team, Sep 2026.

KEY MESSAGES

Three messages every leader repeats

01 Why now

Patients wait 11 days for a new appointment and hold times average eight minutes.

Three schedule systems cannot share open slots.
Staff lose time moving between screens.

02 What changes

One platform, one booking workflow, one set of rules for all nine sites.

Your manager shows you the steps before go-live.

Patient care itself does not change.

03 How we help

Role-based training, a champion on every floor and extra help for four weeks after launch.

Questions go to your floor-walker first.

Proof: 11-day wait today, 3-day goal.

Source: Northfield Health System communications plan, Sep 2026.

THE STORY IN ONE LINE

One schedule, easier access for every patient.

Use these words in every briefing, huddle and email. Local examples welcome; new claims are not.

Every audience hears on a countdown to go-live

AUDIENCE	T-12 WKS	T-8 WKS	T-4 WKS	T-1 WK	GO-LIVE	T+4 WKS
Executives	Case and funding	Steering update	Readiness vote	Go/no-go brief	Launch note	Results review
Physicians	Vision video	Peer demo	Template sign-off	Cheat sheet	Hotline	Fix list
Schedulers	Town hall	Training dates	Sandbox practice	Huddle card	Floor-walkers	Tips and wins
Nurses	Unit meeting	Role cards	Unit demo	Downtime kit	Hotline	Pulse check
Patients	None yet	None yet	Website notice	Email and text	Lobby signage	Survey

- **Rule.** No audience hears a date from a channel it does not already trust.
- **Rule.** Frontline staff hear first from their own manager, never from a broadcast.
- **Rule.** Patients hear only after staff are trained at their site.

Source: Northfield Health System communications plan, Sep 2026.

TAKEAWAY

Four of five clinical systems have never tested a recovery.

Backups exist; the untested failover is the exposure.

RESILIENCE

Recovery is the weakest control

SYSTEM	BACKUP	FAILOVER	TESTED	DOCUMENTED
NorthChart EHR	1	2	4	1
Lab information	2	3	3	1
Pharmacy	1	2	2	1
Imaging archive	3	4	4	2
Scheduling	1	1	3	0

None Low Some High Severe

WHAT WE RECOMMEND

- Test failover for the imaging archive and the EHR before go-live.
- Document recovery steps for the imaging archive, which scores worst on every control.

OUR DIFFERENCE

You will feel the difference in how we work

Our cross-functional teams combine hands-on finance, data and change experience with the technical depth to solve the whole problem.

We work alongside your team as player-coaches, moving from architecture and guardrails through engineering and operations to go-live.

HANDOVER TARGET

Day 90

Your team runs the platform.

OTHER FIRMS

- One-time benefit capture
- Technology-focused
- Generic blueprint
- Siloed workstreams
- Vendor-led delivery

HOW WE WORK

- Iterative path to go-live
- Platform-product focus
- Integrated architecture
- Player-coach partnership
- Cross-discipline teams

WHAT THAT MEANS FOR YOU



We build the capability to own, run and evolve the platform.



We apply data, cloud, security and change expertise.



We set measurable goals for speed, adoption and cost.



Cross-functional teams work together through delivery.



We target go-live outcomes, not only fixed scope.



Healthcare payer and platform experience shapes decisions.

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We apply data, cloud, security and change expertise.



We set measurable goals for speed, adoption and cost.

Shared context supports both **Build and Sustain**

SHARED FOUNDATION

- **Source-backed evidence** from code and data.
- **Validated process context** owned by the business.
- **Approved decisions** and current artifacts.
- **Review and test results** kept beside the evidence.

Governed retrieval connects the foundation to bounded work.

Build

HOW CONTEXT IS USED

Retrieve process, rules, requirements and open questions for a bounded design, coding, testing or cutover task.

VALUE OPPORTUNITY

Warmer handoffs, faster prototyping, stronger traceability and earlier visibility of gaps.

Sustain

HOW CONTEXT IS USED

Connect bug reports and change requests to documented intent, architecture and tests for impact analysis.

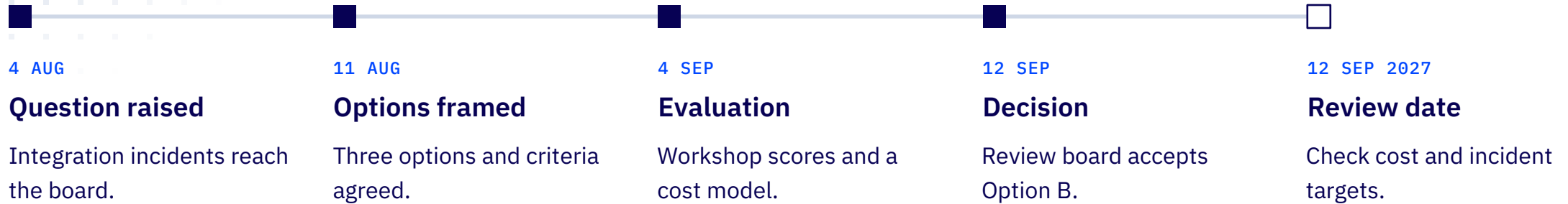
VALUE OPPORTUNITY

Faster investigation, better knowledge preservation and less maintenance effort over time.

The context layer compounds in value only while teams keep it current, governed and source-linked.

HOW THE DECISION WAS MADE

Six weeks from question to a decision on the event bus



WHO TOOK PART

MO **Maya Okafor**
Chief architect

LS **Lena Soto**
Clinical informatics

DH **Dev Hartley**
Platform lead

RK **Rui Kato**
Finance partner

EVIDENCE REVIEWED

- Integration incident log, last 12 months
- Cost model for all three options
- Architecture workshop scores
- Reference check with two peer systems
- Vendor briefings for Option C

HOW THE DECISION WAS MADE

Six weeks from question to a decision

- **Question raised** 4 AUG
Integration incidents reach the board.
- **Options framed** 11 AUG
Three options and criteria agreed.
- **Evaluation** 4 SEP
Workshop scores and a cost model.
- **Decision** 12 SEP
Review board accepts Option B.
- **Review date** 12 SEP 2027
Check cost and incident targets.

WHO TOOK PART

MO	Maya Okafor Chief architect	LS	Lena Soto Clinical informatics
DH	Dev Hartley Platform lead	RK	Rui Kato Finance partner

EVIDENCE REVIEWED

- Integration incident log, last 12 months
- Cost model for all three options
- Architecture workshop scores

AI shifts buy versus build toward strategic fit

BUY

Accelerate standard capabilities

BEST WHEN

- Needs are common
- Speed beats customization
- Vendors carry the roadmap

TRADE-OFFS

- Fit gaps force compromise
- Roadmaps constrain change
- Switching costs build up

BUILD

Own differentiated capabilities

BEST WHEN

- Workflows give an edge
- Data and roadmap control
- Talent is already in place

TRADE-OFFS

- Needs durable capability
- You own maintenance
- Hiring takes time

MODERNIZATION ECONOMICS

AI lowers cost at every stage

PLAN AND UNDERSTAND

Faster discovery, mapping and requirements
Weeks of analysis become days.

IMPLEMENT AND BUILD

Accelerate coding, testing and migration
Agents draft, people review.

MAINTAIN LEGACY

Cut run cost and reliance on scarce skills
Fewer specialists on call.

As AI lowers planning, build and run costs, choose by strategic fit, differentiation and ownership.

COMMERCIALS

\$720K rests on four assumptions

PHASE	TIMING	SHARE	FEE
Diagnose	Weeks 1–4	30%	\$216K
Design	Weeks 5–10	40%	\$288K
Roadmap	Weeks 11–14	30%	\$216K
Total fixed fee		100%	\$720K

HOW IT IS PAID

- Invoiced as each phase is accepted, net 30
- Expenses at cost, capped at 8% of fees

ASSUMPTIONS

14 business units in scope

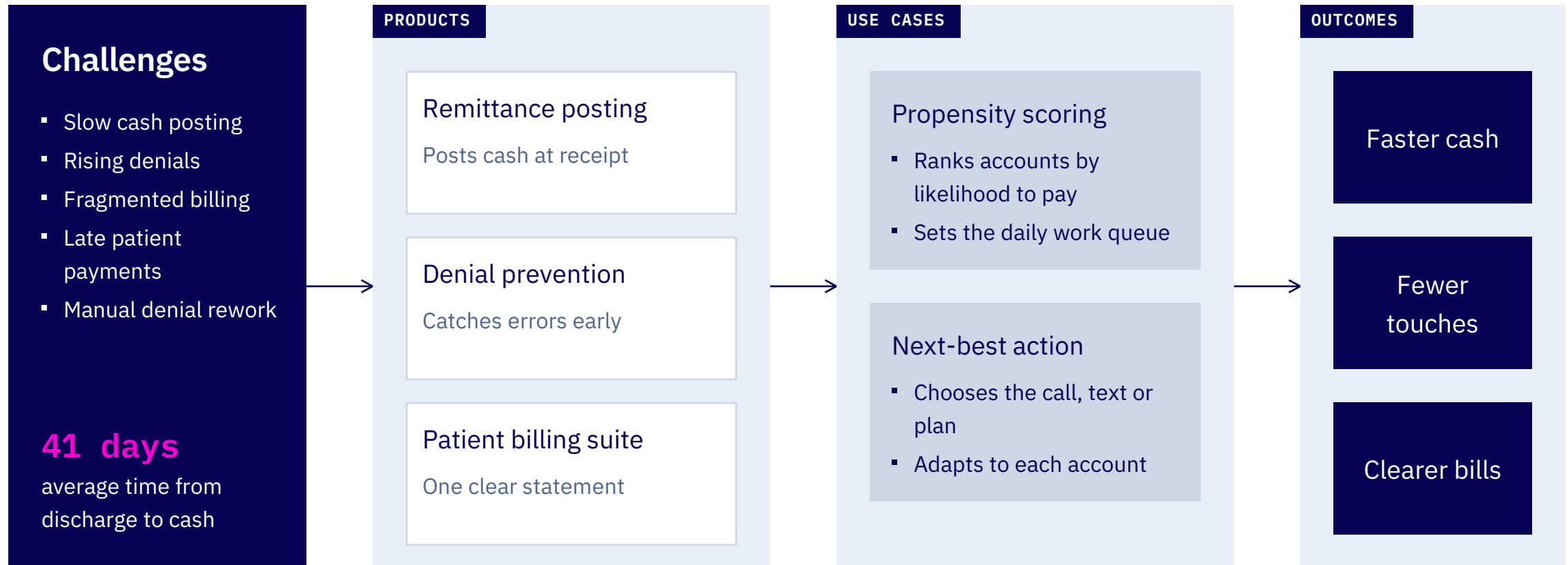
Data access in week 1

Decisions within 5 business days

One ERP instance, no new modules

Changes follow the change process.

From revenue-cycle challenges to measurable outcomes



Start from the challenge, not the tool: each product pays off when a use case makes it a repeatable action.

Custom software scales what makes you unique

WHY BUILD

Make software conform to the business

- 01 Unique processes**
Encode the workflows that set you apart.
- 02 Familiar language**
Keep the vocabulary your people use.
- 03 Selective change**
Modernize only what needs to change.

CHANGE ON YOUR TERMS

Less translation, less retraining.



ADVANTAGE 01

Scale what makes you unique

Turn distinctive processes into repeatable operations.

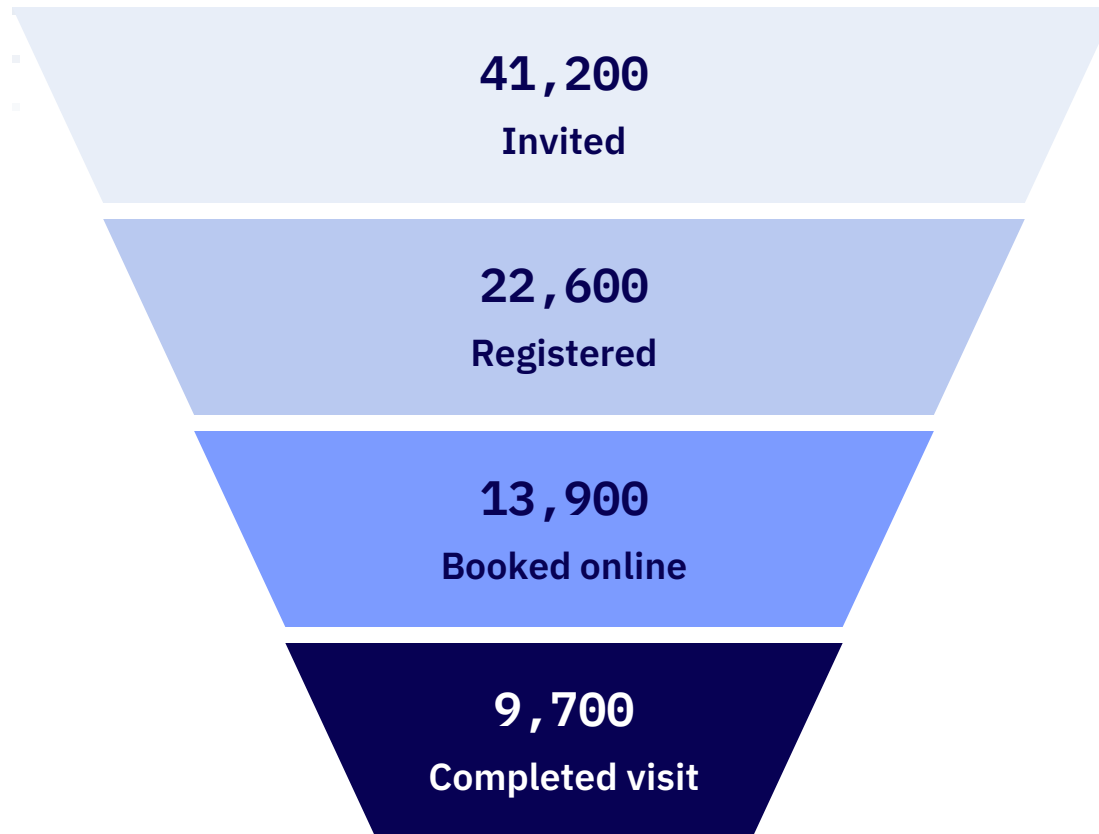
ADVANTAGE 02

Modernize without upheaval

Keep familiar roles and vocabulary.

Build when differentiation matters and you want to choose where the business changes.

Three insights explain where patients drop away



Insight 1: Registration loses 45% of invitees

Identity checks fail on the first try for one in five.

Insight 2: Booking is a phone habit

Patients over 65 book 3x more by phone than online.

Insight 3: Visits convert once booked

70% of online bookings are completed, above phone.

Source: Sample figures for layout only; replace with current firm data before use.

Front door scores well on process, poorly on data

CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	WHAT WE FOUND
Patient access Front door	3	2	2	3	2	- Online booking covers 40% of visits - Two registration workflows by site - Add a single eligibility check
Scheduling Front door	3	3	2	3	3	- Templates differ by clinic - Self-scheduling live at three sites - Standardize visit-type rules
Referral management Front door	2	1	1	2	2	- Referrals arrive by fax and portal - No shared status view - Stand up a referral queue
Revenue cycle Revenue	4	5	3	3	4	- Rules engine at posting is mature - Close takes eight days, not five - Month-end review is manual

1 Ad hoc 2 Repeatable 3 Defined 4 Managed 5 Optimized

Source: Sample ratings for layout only; replace before use.

Revenue and care delivery: strong cores, weak handoffs

CAPABILITY HEAT MAP WITH NOTES (CONTINUED, 2 OF 3)

CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	WHAT WE FOUND
Patient billing Revenue	3	3	3	2	3	- Statements are clear and on time - Payment plans sit outside the platform
Denials management Revenue	2	2	2	2	1	- Denial reasons are not coded - Rework is owned by no one
Clinical documentation Care delivery	4	4	3	4	3	- Templates are strong and reused - Voice capture is live in two units
Care coordination Care delivery	2	2	1	3	2	- Handoffs run on phone and email - Care plans live in three tools
Supply chain Operations	3	2	2	2	3	- Contracts are tracked by site - Item master has duplicates
Workforce management Operations	2	3	2	2	3	- Rosters are built by hand - Credentialing is on time

1 Ad hoc
2 Repeatable
3 Defined
4 Managed
5 Optimized

Source: Sample ratings for layout only; replace before use.

Referrals, denials and care coordination score lowest

	CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	WHAT WE FOUND
FRONT DOOR	Patient access	3	2	2	3	2	- Two registration flows across sites - Eligibility is checked after arrival
	Referral management	2	1	1	2	2	- Referrals arrive by fax and portal - No shared status view
REVENUE	Revenue cycle	4	5	3	3	4	- Rules engine at posting is mature - Month-end review is still manual
	Denials management	2	2	2	2	1	- Denial reasons are not coded - Rework has no owner
CARE DELIVERY	Clinical documentation	4	4	3	4	3	- Templates are strong and reused - Voice capture live in two units
	Care coordination	2	2	1	3	2	- Handoffs run on phone and email - Care plans live in three tools

1 Ad hoc
2 Repeatable
3 Defined
4 Managed
5 Optimized

Source: Sample ratings for layout only; replace before use.

Start with denials and referrals, then lift the rest

	CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	HORIZON	WHAT WE FOUND
FRONT DOOR	Patient access	3	2	2	3	2	NEXT	- Two registration flows across sites - Eligibility is checked after arrival
	Referral management	2	1	1	2	2	NOW	- Referrals arrive by fax and portal - No shared status view
REVENUE	Revenue cycle	4	5	3	3	4	LATER	- Rules engine at posting is mature - Month-end review is still manual
	Denials management	2	2	2	2	1	NOW	- Denial reasons are not coded - Rework has no owner
CARE DELIVERY	Clinical documentation	4	4	3	4	3	LATER	- Templates are strong and reused - Voice capture live in two units
	Care coordination	2	2	1	3	2	NEXT	- Handoffs run on phone and email - Care plans live in three tools

1 Ad hoc
2 Repeatable
3 Defined
4 Managed
5 Optimized

Now this quarter, Next after, Later backlog

Source: Sample ratings for layout only; replace before use.

CAPABILITY	PROC	TECH	DATA	PPL	GOV	NOTES
Patient access	3	2	2	3	2	- Two registration flows - Eligibility checked late
Scheduling	3	3	2	3	3	- Rules differ by clinic - Self-scheduling at 3 sites
Referral management	2	1	1	2	2	- Faxes and portal mixed - No shared status view
Revenue cycle	4	5	3	3	4	- Rules engine is mature - Month-end review is manual
Patient billing	3	3	3	2	3	- Statements are on time - Plans sit outside billing
Denials management	2	2	2	2	1	- Denials are not coded - Rework has no owner
Clinical documentation	4	4	3	4	3	- Templates well reused - Voice capture in 2 units

CAPABILITY ASSESSMENT

Seven capabilities and the reason behind each score

- **Process leads.** It is the top score in five of seven rows.
- **Data lags.** Referral management scores 1 on technology and data.
- **Floor.** Denials has a 1 on governance.

- 1 Ad hoc
- 2 Repeatable
- 3 Defined
- 4 Managed
- 5 Optimized

Source: Sample ratings for layout only; replace before use.

Enterprise capabilities trail on data and people

- **Security holds.** Governance reaches 5 in cybersecurity.
- **Data is the gap.** Master data has no owner in three of four rows.
- **People next.** Workforce and analytics both score 2.

1 Ad hoc

2 Repeatable

3 Defined

4 Managed

5 Optimized

CAPABILITY HEAT MAP WITH NOTES (CONTINUED, 3 OF 3)

CAPABILITY	PROC	TECH	DATA	PPL	GOV	NOTES
Supply chain	3	2	2	2	3	. Item master has duplicates . Contracts tracked by site . Clean item master first . Pool volumes across sites
Workforce management	2	3	2	2	3	. Rosters are built by hand . Credentialing is on time . Open reqs data is stale . Automate roster build
Data and analytics	2	3	2	2	2	. Reports rebuilt per team . Master data has no owner . Name data stewards . Publish a metric catalog
Cybersecurity	3	4	3	3	5	. Controls tested yearly . Access review quarterly . Vendor risk is tracked . Add live monitoring

Source: Sample ratings for layout only; replace before use.

Four capabilities need attention first

CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	PRIORITY	WHAT WE FOUND
Referral management Front door	2	1	1	2	2	P1	- Referrals arrive by fax and portal - No shared status view
Denials management Revenue	2	2	2	2	1	P1	- Denial reasons are not coded - Rework has no owner
Care coordination Care delivery	2	2	1	3	2	P2	- Handoffs run on phone and email - Care plans live in three tools
Supply chain Care delivery	3	2	2	2	3	P3	- Item master has duplicates - Contracts tracked by site, not system
Revenue cycle Revenue	4	5	3	3	4	P4	- Rules engine at posting is mature - Month-end review is still manual

1 Ad hoc
2 Repeatable
3 Defined
4 Managed
5 Optimized

P1 fix now, P2 next, P3 plan, P4 hold

Source: Sample ratings for layout only; replace before use.

CARE DELIVERY DETAIL

Care delivery and operations: one card per row

LOCATOR

A1					
A2					
A3					
A4					
A5					
A6					
A7					
A8					
A9					

1 Ad hoc 5 Optimized

Source: Sample ratings for layout only; replace before use.

A7 Clinical documentation

PRO	TEC	DAT	PPL	GOV
4	4	3	4	3

- Templates are strong and reused
- Voice capture live in two units
- Scale ambient notes to eight units

A8 Care coordination

PRO	TEC	DAT	PPL	GOV
2	2	1	3	2

- Handoffs run on phone and email
- Care plans live in three tools
- Pilot a shared care plan in oncology

A9 Supply chain

PRO	TEC	DAT	PPL	GOV
3	2	2	2	3

- Item master has duplicates
- Contracts tracked by site, not system
- Clean the item master first

Denials management is the weakest of the revenue rows

A6

Denials management

PRO	TEC	DAT	PPL	GOV
2	2	2	2	1

Overall 2 of 5

WHAT DRIVES THE SCORE

- Denial reasons are not coded
- Rework has no owner
- Appeals follow no playbook

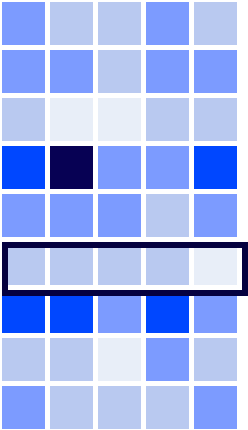
WHAT WE'D DO

- Code every denial at source
- Name one denials owner
- Write the appeal playbook

ACROSS A4 TO A6 (CONTINUED, 2 OF 2)

- **Process outruns data.** Revenue cycle scores 4 on process but 3 on data.
- **Denials are the gap.** A6 is the only revenue row with a 1.
- **Lowest cell.** Governance in denials management scores 1.
- **Sequence.** Code denials first; rules and billing changes follow.

LOCATOR



ROW A6

1 Ad hoc 2 Repeatable 3 Defined 4 Managed 5 Optimized

Source: Sample ratings for layout only; replace before use.

Revenue cycle and billing: process is ahead of the platform

A4

Revenue cycle

PRO	TEC	DAT	PPL	GOV
4	5	3	3	4

Overall 4 of 5

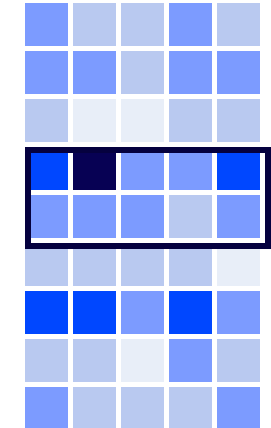
WHAT DRIVES THE SCORE

- Rules engine at posting is mature
- Month-end review is still manual
- Close takes eight days, not five

WHAT WE'D DO

- Extend rules to the remaining sites
- Automate the day-one review
- Publish a daily close status

LOCATOR



ROWS A4–A5

A5

Patient billing

PRO	TEC	DAT	PPL	GOV
3	3	3	2	3

Overall 3 of 5

WHAT DRIVES THE SCORE

- Statements are clear and on time
- Payment plans sit outside the platform
- Lockbox posting runs on a spreadsheet

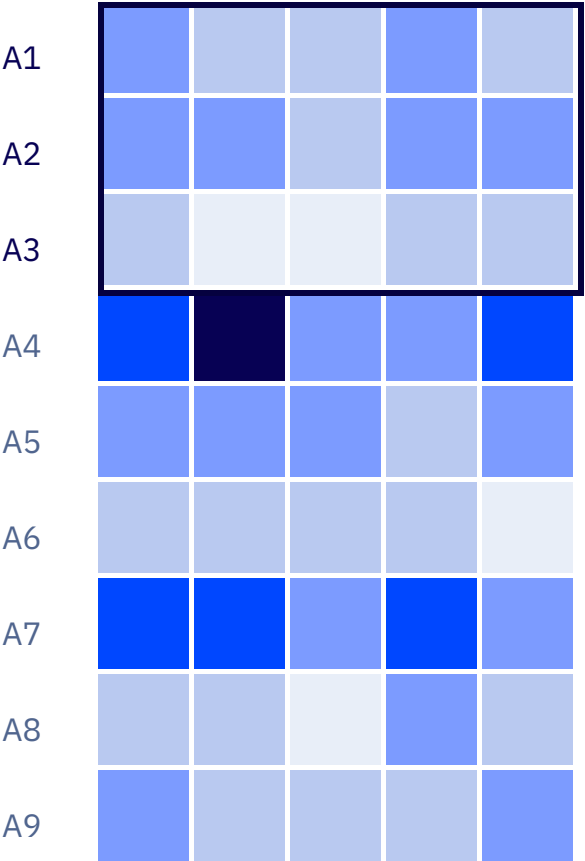
WHAT WE'D DO

- Move plans into the billing system
- Retire the lockbox spreadsheet
- Add text-to-pay for balances

1 Ad hoc 2 Repeatable 3 Defined 4 Managed 5 Optimized

Source: Sample ratings for layout only; replace before use.

LOCATOR



1 Ad hoc 5 Optimized

FRONT-DOOR DETAIL

Front-door rows in depth

A1 Patient access

PRO	TEC	DAT	PPL	GOV
3	2	2	3	2

- **Drives it.** Two registration flows across sites; eligibility checked after arrival.
- **We'd do.** Run one eligibility check at booking; train front desks on a single script.

A2 Scheduling

PRO	TEC	DAT	PPL	GOV
3	3	2	3	3

- **Drives it.** Self-scheduling live at three sites; visit-type rules differ by clinic.
- **We'd do.** Standardize visit-type rules; automate the waitlist.

A3 Referral management

PRO	TEC	DAT	PPL	GOV
2	1	1	2	2

- **Drives it.** Referrals arrive by fax and portal; no shared status view.
- **We'd do.** Stand up one referral queue; track leakage by specialty.

Source: Sample ratings for layout only; replace before use.

Revenue cycle is the group to look at first

CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	OVERALL
A1 Patient access	3	2	2	3	2	2
A2 Scheduling	3	3	2	3	3	3
A3 Referral management	2	1	1	2	2	2
A4 Revenue cycle	4	5	3	3	4	4
A5 Patient billing	3	3	3	2	3	3
A6 Denials management	2	2	2	2	1	2
A7 Clinical documentation	4	4	3	4	3	4
A8 Care coordination	2	2	1	3	2	2
A9 Supply chain	3	2	2	2	3	2

1 Ad hoc 2 Repeatable 3 Defined 4 Managed 5 Optimized

Source: Sample ratings for layout only; replace before use.

- **Reference badges.** Each row keeps its code on the slides that follow.
- **Overall.** The rounded mean of five dimensions.

NEXT TWO SLIDES

Detailed notes follow for A4–A6

A4 Revenue cycle

A5 Patient billing

A6 Denials management

Revenue cycle is the group to look at first

	CAPABILITY	PROCESS	TECHNOLOGY	DATA	PEOPLE	GOVERNANCE	OVERALL
FRONT DOOR	A1 Patient access	3	2	2	3	2	2
	A2 Scheduling	3	3	2	3	3	3
	A3 Referral management	2	1	1	2	2	2
REVENUE	A4 Revenue cycle	4	5	3	3	4	4
	A5 Patient billing	3	3	3	2	3	3
	A6 Denials management	2	2	2	2	1	2
CARE DELIVERY	A7 Clinical documentation	4	4	3	4	3	4
	A8 Care coordination	2	2	1	3	2	2
	A9 Supply chain	3	2	2	2	3	2

1 Ad hoc 2 Repeatable 3 Defined 4 Managed 5 Optimized

Source: Sample ratings for layout only; replace before use.

- **Reference badges.** Each row keeps its code on the slides that follow.
- **Overall.** The rounded mean of five dimensions.

NEXT TWO SLIDES

Detailed notes follow for A4–A6

A4 Revenue cycle

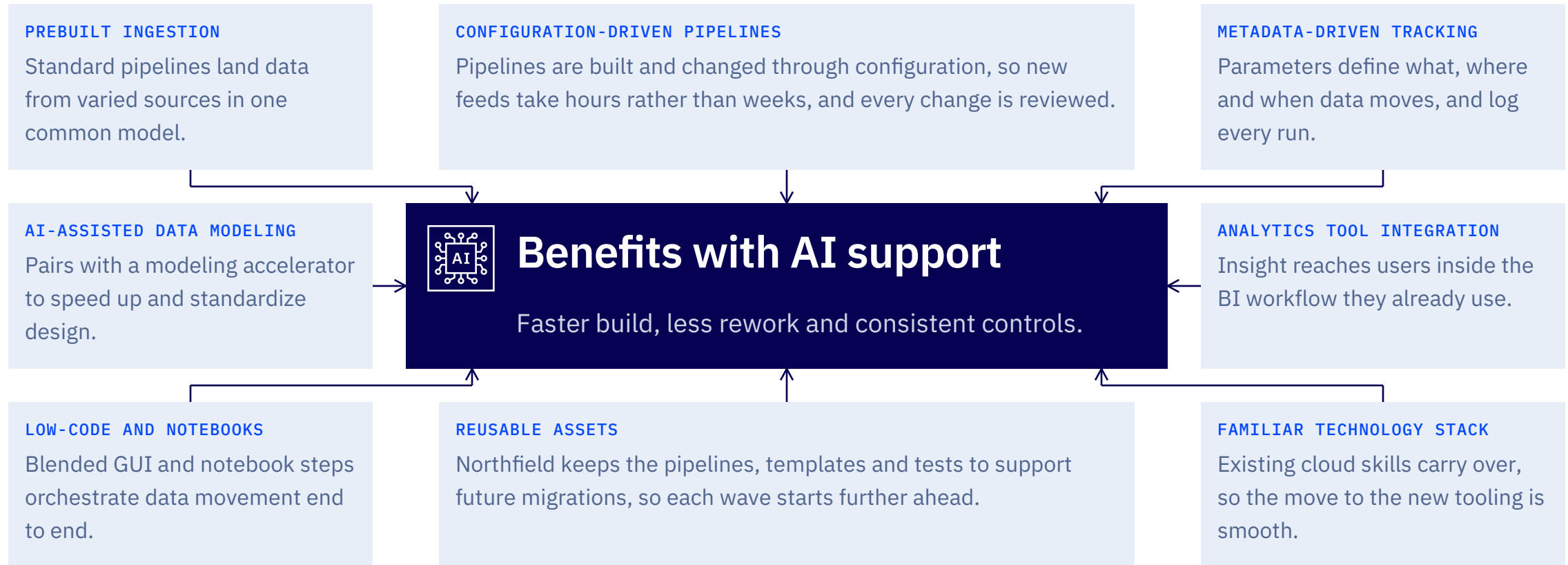
A5 Patient billing

A6 Denials management

Four domains set the investment order



Reusable patterns speed up the platform build



Source: West Monroe engineering patterns, Northfield delivery plan.

Front-desk schedulers are hit hardest on process, technology and role

HIGH IMPACT

01 Process

Walk-in and phone booking merge into one scheduling workflow with new double-booking rules.

Support: Redesign sessions with five clinics.

HIGH IMPACT

02 Technology

Three legacy schedule screens are replaced by a single web platform with new login.

Support: Sandbox practice from week 6.

HIGH IMPACT

03 Role

Schedulers own referral holds and waitlist outreach that used to sit with nurses.

Support: Updated role cards and job aids.

MEDIUM IMPACT

04 Skills

Template building and rules-based overbooking are new skills for the team.

Support: Eight hours of role-based training.

Scale: 142 schedulers, nine sites

Limit: 61 work part time

Source: Northfield Health System change impact assessment, Sep 2026.

Each month of delay gives up \$217K of value

VALUE FORGONE EACH MONTH

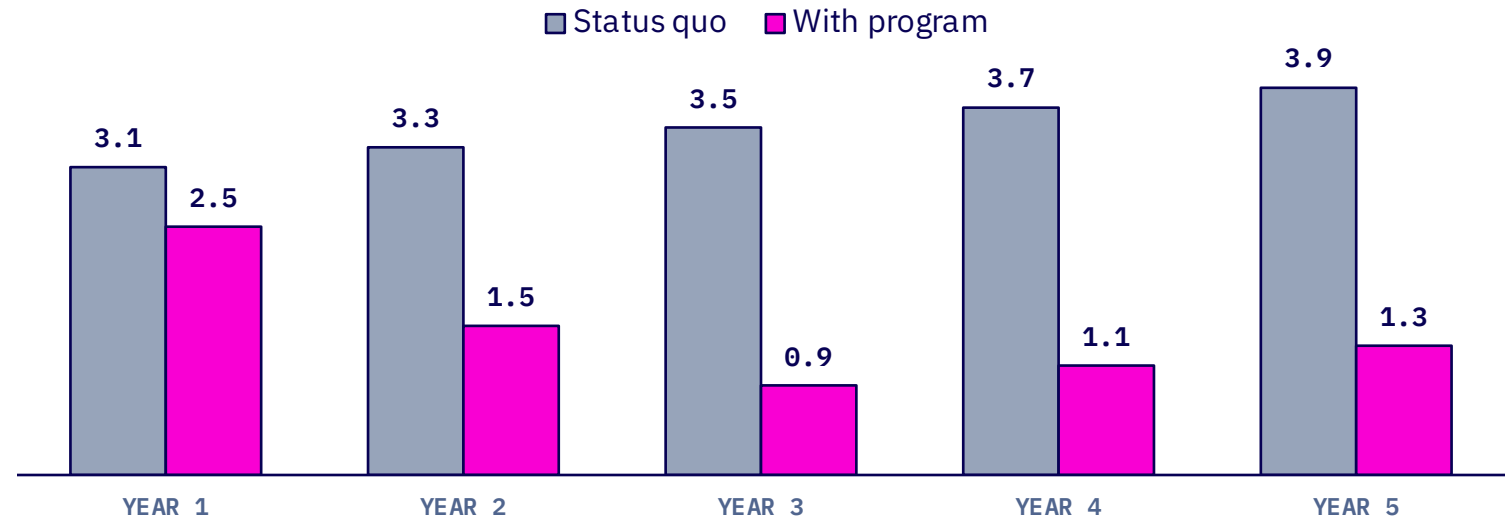
\$217K

\$2.6M a year, divided by 12

Six-month delay costs **\$1.3M**Payback moves to **month 37**A year of delay costs **\$2.6M**

Avoidable cost if nothing changes

\$M A YEAR



- Manual close hours grow 6% a year with volume
- Denied claims keep writing off 9.1% of billed revenue

Source: Sample figures for layout only; replace with current program data before use.

Twenty-four accelerators by segment and phase

Read down a column to follow one phase through the PDLC, or across a row to see everything available in a segment.

HOW TO USE

Two accelerators each

- Start with the first in every cell
- Add the second as scope grows
- Both ship with playbooks

Source: Sample names for layout only; replace with the current catalog before use.

	Rationalize	Build	Sustain
Identify	Portfolio Analyzer Dependency Mapper	Code Scanner Interface Profiler	Estate Monitor Drift Alerts
Define	Disposition Modeler Risk Scorer	Migration Planner Cutover Playbooks	Service Playbooks SLO Designer
Build	Target Blueprints Architecture Linter	Replatform Kit Test Harness	Release Factory Runbook Automator
Optimize	Value Tracker Benefit Dashboard	Data Mover Reconciler	FinOps Optimizer Capacity Forecaster

Four stakeholders need a sponsor push

19 days

average wait across the four

BLOCKED • 34 DAYS

Chief Medical Officer · Clinical ops

- Ask: CEO office holds 45 minutes
- Open: who owns documentation standards

NO RESPONSE • 21 DAYS

Patient Access Supervisors

- Ask: COO releases team for 60 minutes
- Open: where registration errors are found

RISK IF UNRESOLVED

2

findings stay unvalidated at the readout on Nov 3.

RESCHEDULED • 12 DAYS

VP Supply Chain · Finance

- Ask: shared Thursday slot
- Open: spend outside contracts

RESCHEDULED • 9 DAYS

Chief Digital Officer · IT

- Ask: sponsor note on priority
- Open: platform roadmap fit

ASKS THIS WEEK

- CEO office: 45 minutes for the CMO
- COO: release registrars for 60 minutes
- CFO: route the board chair request

Source: Sample names for layout only; replace with engagement stakeholders before use.

Three stakeholders have waited more than ten days

STAKEHOLDER	STATUS	DAYS	REASON
Chief Medical Officer Clinical ops	 Blocked	34	Calendar blocked by assistant
Patient Access Supervisors Patient access	 No response	21	No reply to three emails
VP Supply Chain Finance	 Rescheduled	12	Rescheduled twice
Chief Digital Officer IT	 Rescheduled	9	Rescheduled; sponsor note due
Board finance chair Finance	 Awaiting date	7	Awaiting CFO routing
Dir., Revenue Integrity Revenue cycle	 Awaiting date	5	Rescheduled once

Source: Sample names for layout only; replace with engagement stakeholders before use.

ESCALATION ASKS

3

sponsor actions needed this week

- CEO office to hold 45 minutes for the CMO
- COO to release registrars for 60 minutes
- CFO to route the board chair request

Until then: clinical and front-end findings rest on documents only.

Revenue cycle sees denials start at authorization

STAKEHOLDER	DEPARTMENT AND DATE	FORMAT	INTERVIEWERS
J. Alvarez VP Revenue Cycle	Revenue cycle Sep 8, 60 minutes	Onsite, Main campus	K. Ortiz, W. Lee

TOPIC 1

Denials

- Authorization gaps cause 40% of denials
- Appeals take 45 days



We find it at billing, three weeks late.

VP REVENUE CYCLE

Source: Sample names for layout only; replace with engagement stakeholders before use.

TOPIC 2

Registration

- Front-desk errors found at billing
- No eligibility check at check-in

PAIN AND OPPORTUNITY

- Pain: manual rework
- Pain: no shared view
- Idea: one work queue

TOPIC 3

Tooling

- Three work queues, no single view
- Reports rebuilt by hand each month

FOLLOW-UPS AND QUESTIONS

- Denial extract: received
- Appeal staffing: pending Oct 6
- Open: who owns payer contracts?

Five findings from the revenue cycle interview



J. Alvarez
VP Revenue Cycle

DEPARTMENT AND DATE
Revenue cycle · Sep 8

INTERVIEWERS
K. Ortiz, W. Lee

“

**We find it at billing,
three weeks late.**

J. ALVAREZ

- 01** **Denials start at authorization**
40% of denials trace to missing authorizations. So what: fix intake first.
- 02** **Errors surface at billing**
Front-desk errors are found three weeks late. So what: add check-in checks.
- 03** **Three queues, no shared view**
Teams work denials in separate queues. So what: build one queue by payer.
- 04** **Appeals average 45 days**
Two analysts hold the appeal know-how. So what: cross-train and staff the tail.
- 05** **Reports are hand-built**
Monthly reports restart in spreadsheets. So what: automate the denial extract.

FOLLOW-UP	STATUS
Denial extract	■ Received
Appeal staffing	■ Pending
Contract owner	□ Open

Source: Sample names for layout only; replace with engagement stakeholders before use.

Patient access leads agree on registration as the first fix

Group session · Sep 25 · Onsite, Main campus · 90 min · K. Ortiz, A. Cole

DIRECTOR, PATIENT ACCESS

M. Chen

- Errors begin at the front desk
- Three scheduling tools in use
- Wants real-time eligibility

PATIENT ACCESS SUPERVISOR

B. Kowalski

- Staff re-key data twice
- Turnover is highest at intake
- Prior-auth data is slow to arrive

SCHEDULING LEAD

U. Lindgren

- Templates differ by clinic
- Cancellations are not tracked
- Open to a shared scheduling tool

3 of 3

name registration quality as the first fix

OPPORTUNITIES

- Check eligibility at check-in
- One scheduling template set
- Shared error dashboard

FOLLOW-UPS AND OPEN QUESTIONS

- Registration error log: received
- Prior-auth data: overdue 3 days
- Open: who owns template changes?

Source: Sample names for layout only; replace with engagement stakeholders before use.

Revenue cycle sees denials start at authorization



J. Alvarez

VP Revenue Cycle

DEPARTMENT

Revenue cycle

DATE AND FORMAT

Sep 8 · Onsite · 60 min

INTERVIEWERS

K. Ortiz, W. Lee

01 Denials start at authorization

40% of denials trace to missing authorizations. So what: fix intake before appeals.

02 Errors surface at billing

Front-desk errors are found about three weeks after the visit. So what: add a check-in step.

03 Three work queues, no single view

Teams hide backlog in separate queues and hand-built reports. So what: one queue by payer.

04 Appeals depend on two analysts

Appeals average 45 days and the know-how sits with two people. So what: cross-train now.

Source: Sample names for layout only; replace with engagement stakeholders before use.



J. Alvarez

VP Revenue Cycle

DEPARTMENT

Revenue cycle

DATE AND FORMAT

Sep 8 · Onsite · 60 min

INTERVIEWERS

K. Ortiz, W. Lee

STAKEHOLDER INTERVIEWS

Denials start at authorization

- 01 Denials start at authorization**
Evidence: 40% of denials trace to authorization gaps. Implication: fix intake, not appeals.
- 02 Errors surface late**
Evidence: front-desk errors found at billing, three weeks on. Implication: add a check-in eligibility step.
- 03 No single work view**
Evidence: three queues, reports rebuilt by hand monthly. Implication: one queue by payer.
- 04 Appeal know-how is concentrated**
Evidence: two analysts hold the practice behind 45-day appeals. Implication: cross-train first.
- 05 Payer contracts have no owner**
Evidence: nobody could name one owner. Implication: assign an owner before redesign.

FOLLOW-UPS AND QUESTIONS

- **Received:** denial extract; appeal staffing due Oct 6
- **Open:** who owns payer contracts?

Source: Sample names for layout only; replace with engagement stakeholders before use.

J. Alvarez, VP Revenue Cycle: Sep 8 interview

DEPARTMENT

Revenue cycle

DATE AND FORMAT

Sep 8 · Onsite · 60 min

INTERVIEWERS

K. Ortiz, W. Lee

TOPIC	KEY POINT	PAIN POINT OR OPPORTUNITY
Denials	40% of denials trace to authorization gaps	Rework after day 30 is manual
Registration	Front-desk errors surface at billing	No eligibility check at check-in
Tooling	Three work queues, no single view	Reports rebuilt by hand monthly
Appeals	Appeals average 45 days to resolve	Two analysts hold the know-how

Source: Sample names for layout only; replace with engagement stakeholders before use.

“

We find it at billing, three weeks late.

J. ALVAREZ

OPPORTUNITIES

- One work queue by payer
- Eligibility check at check-in

FOLLOW-UPS AND QUESTIONS

- **Denial extract:** received Sep 22
- **Appeal staffing:** pending, due Oct 6
- **Open:** who owns payer contracts?

Fifteen more interviews run in three waves through October

BY OCT 16

Wave 1 · Executives

- Chief Nursing Officer
 - Oct 7 · Clinical priorities
- Chief Operating Officer
 - Oct 8 · Operating model
- VP Supply Chain
 - Oct 9 · Spend scope
- Chief Digital Officer
 - Oct 13 · Roadmap fit
- Board finance chair
 - Oct 15 · Funding view

BY OCT 23

Wave 2 · Operational leaders

- Director, Scheduling
 - Oct 17 · Template rules
- Director, Billing
 - Oct 20 · Statement process
- Mgr, Credentialing
 - Oct 21 · Payer enrollment
- Dir., Quality
 - Oct 22 · Reporting load
- Mgr, Cash Posting
 - Oct 23 · Posting lag

BY OCT 30

Wave 3 · Front-line and SMEs

- Registration leads (3)
 - Oct 24 · Workflow walk
- Coding specialists (4)
 - Oct 27 · Query volume
- Denials analysts (3)
 - Oct 28 · Appeal tooling
- Service desk lead
 - Oct 29 · Incident trends
- Unit clerks (3)
 - Oct 30 · Charge capture

Source: Sample names for layout only; replace with engagement stakeholders before use.

- **Finance and compliance.** Every request met inside a week of the interview.
- **Clinical operations.** One request blocked pending CMO sponsor action.
- **Patient access.** Prior-authorization data is slow to arrive.
- **IT.** Two inventories are in progress, due next week.

REQUESTS MET

5 of 12

with four pending and three open.

STAKEHOLDER INTERVIEWS

Finance and compliance answered fastest

PERSON	DEPARTMENT	MET	FOLLOW-UP REQUEST	STATUS
J. Alvarez VP Revenue Cycle	Revenue cycle	Sep 8	Denial reason-code extract	Received
M. Chen Director, Patient Access	Patient access	Sep 9	Registration error log	Received
R. Okafor Chief Information Officer	IT	Sep 10	Application inventory	Pending
S. Patel Dir., Enterprise Apps	IT	Sep 11	Interface and integration map	Pending
L. Brandt Chief Financial Officer	Finance	Sep 15	FY26 budget by department	Received
D. Moreau Controller	Finance	Sep 16	Month-end close calendar	Received

Source: Sample names for layout only; replace with engagement stakeholders before use.

Leaders agree on the problem and differ on the order

REVENUE CYCLE

J. Alvarez, VP Revenue Cycle

- Denials rise on missing authorization
- Rework is manual after day 30
- Wants one work queue

PATIENT ACCESS

M. Chen, Director

- Registration errors start at the front desk
- Three scheduling tools in use
- Asks for real-time eligibility

IT

R. Okafor, CIO

- Two platforms near end of support
- Integration is the main risk
- Prefers phased cutover

FINANCE

L. Brandt, CFO

- Close takes 12 days
- Sees denials as a cash lever
- Needs a case by January

COMPLIANCE

K. Ito, Chief Compliance Officer

- Audit findings repeat each year
- Wants controls built in
- Open to automation

HIM

P. Silva, Director

- Coding backlog averages 9 days
- Documentation gaps drive denials
- Short two coders

Source: Sample names for layout only; replace with engagement stakeholders before use.

What eight leaders told us

- **J. Alvarez, VP Revenue Cycle.** Denials start at authorization; rework after day 30 is manual.
- **M. Chen, Patient Access.** Registration errors begin at the front desk; three scheduling tools.
- **R. Okafor, CIO.** Two platforms near end of support; integration is the main risk.
- **L. Brandt, CFO.** Close takes 12 days; wants a funded case by January.
- **K. Ito, Compliance.** Audit findings repeat; wants controls built into workflow.
- **P. Silva, HIM.** Coding backlog averages 9 days; short two coders.
- **T. Wallace, Clinical Ops.** Staffing by unit varies; nurses re-key charge data.
- **D. Moreau, Controller.** Manual journals at close; reconciliation lags a week.

4 of 8

name authorization and registration quality as the first fix

3 of 8

want controls and automation built into the workflow

2 of 8

need a funded case before January

HOW TO READ THIS

Four of eight put authorization and registration first

Finance, IT and compliance want a funded case, a phased cutover and built-in controls before any tool change.

Source: Sample names for layout only; replace with engagement stakeholders before use.

COMMERCIALS

A \$1.8M investment pays back in 14 months

\$1.8M

total investment

\$2.6M

annual run-rate benefit

14

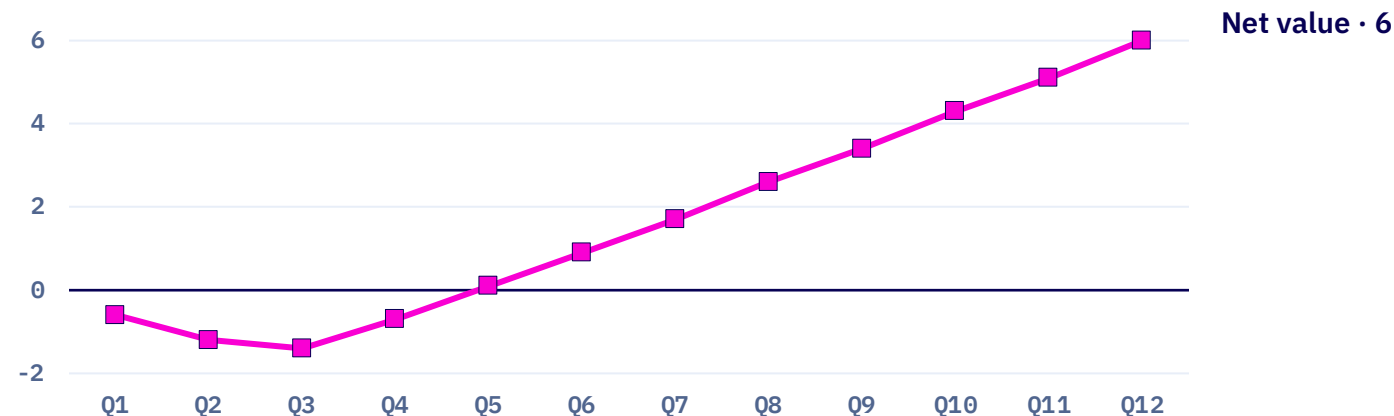
months to payback

330%

three-year return

Cumulative net value

\$M, BY QUARTER



WHAT IT MEANS

- Investment peaks in quarter 3
- Net value turns positive in quarter 5
- \$6.0M net value by year three

Source: Illustrative Northfield business case; replace with validated figures before use.

Each phase ends in a concrete outcome

01 Rationalize

Decide what to modernize, in what order, and why it pays back.

- Portfolio assessment
- Application rationalization
- Target architecture
- Business case

OUTCOME

A funded roadmap: every application has a disposition and a wave.

02 Build

Move the chosen applications and data onto the new platform, safely.

- Re-platform
- Refactor
- Data migration
- Cutover

OUTCOME

Applications live on the new platform with their data reconciled.

03 Sustain

Run the new estate well and keep it from aging into the next legacy.

- Run and operate
- Optimize
- Modernization governance
- Continuous modernization

OUTCOME

A run model and a pipeline that keep the estate current.

Every phase opens on **criteria** and closes at a gate

1 Rationalize

ENTRY CRITERIA

- Mandate and sponsor named
- Application data accessible
- Budget for discovery released

EXIT CRITERIA

- Every application dispositioned
- Business case agreed by finance

GATES

G1 Portfolio decision

G2 Architecture and funding

2 Build

ENTRY CRITERIA

- Roadmap and funding approved
- Target architecture signed
- Wave 1 team in place

EXIT CRITERIA

- Rollback rehearsed and passed
- Data reconciled to the penny

GATES

G3 Cutover readiness

G4 Steady-state handover

3 Sustain

ENTRY CRITERIA

- Cutover complete, hypercare done
- Runbooks accepted by support
- Run budget transferred

EXIT CRITERIA

- Service levels met two quarters
- Next-wave candidates ranked

GATES

Quarterly health review

Annual re-rationalization

THE IDEA

Decide, deliver, then keep it current

Most estates modernize once and then age. The lifecycle makes the third phase as deliberate as the first two.

12

subtracks across three phases

MODERNIZATION LIFECYCLE

Three phases, one lifecycle

Rationalize

Decide

What to modernize, in what order

- Portfolio assessment
- Application rationalization
- Target architecture
- Business case

Build

Deliver

Move applications and data safely

- Re-platform
- Refactor
- Data migration
- Cutover

Sustain

Run

Run it well, keep it current

- Run and operate
- Optimize
- Modernization governance
- Continuous modernization

Three phases, twelve subtracks, four gates

Northfield's scheduling and billing estate moves through the same three phases, each closed by a gate the steering group signs.

FOUR GATES

Exit gates

- G1 · Portfolio decision
- G2 · Architecture and funding
- G3 · Cutover readiness
- G4 · Steady-state handover

01 Rationalize

Portfolio assessment
Score 140 applications

Application rationalization
Retire, keep, move, rebuild

Target architecture
Scheduling, billing design

Business case
Cost, risk, benefit by wave

02 Build

Re-platform
Move to the landing zone

Refactor
Split the billing monolith

Data migration
Reconciled patient data

Cutover
Waves, rehearsals, go-live

03 Sustain

Run and operate
Monitoring and support

Optimize
Tune cost and capacity

Modernization governance
Standards and funding

Continuous modernization
Keep the estate current

Each phase has its own leads and Intellio accelerators

Northfield's team and the Intellio accelerators change with each phase, but the three leads stay constant.

HOW TO READ

Reading the rows

- Roles lead through the handoff
- Accelerators are tuned to the estate
- Each phase closes at a gate

Source: Sample names for layout only; replace with the current catalog before use.

01 Rationalize

LEAD ROLES

Engagement lead
Enterprise architect
Portfolio analyst
Finance partner

INTELLIO ACCELERATORS

Portfolio Scan
Disposition Scorecard
Reference Architecture Kit
Business Case Model

02 Build

LEAD ROLES

Delivery lead
Cloud engineer
Data engineer
Quality lead

INTELLIO ACCELERATORS

Landing Zone Blueprint
Refactor Toolkit
Data Reconciler
Cutover Runbook

03 Sustain

LEAD ROLES

Service owner
Site reliability lead
FinOps analyst
Change lead

INTELLIO ACCELERATORS

Operations Playbook
Cost Optimizer
Governance Cadence Kit
Modernization Radar

Four steps move Northfield from stage one to five

1 Assess

STAGE 1 → 2

- Baseline each team on seven capabilities
- Pick two pilot teams
- Set the target stage

DONE WHEN

Every team has a scored baseline

2 Pilot

STAGE 2 → 3

- Pair coaches with pilot teams
- Build shared workflows and prompts
- Name a workflow owner

DONE WHEN

Pilot teams reuse one shared workflow

3 Embed

STAGE 3 → 4

- Connect agents to PM tool, repo and CI
- Add risk-based approval gates
- Train the next teams

DONE WHEN

Handoffs run through agents with review

4 Extend

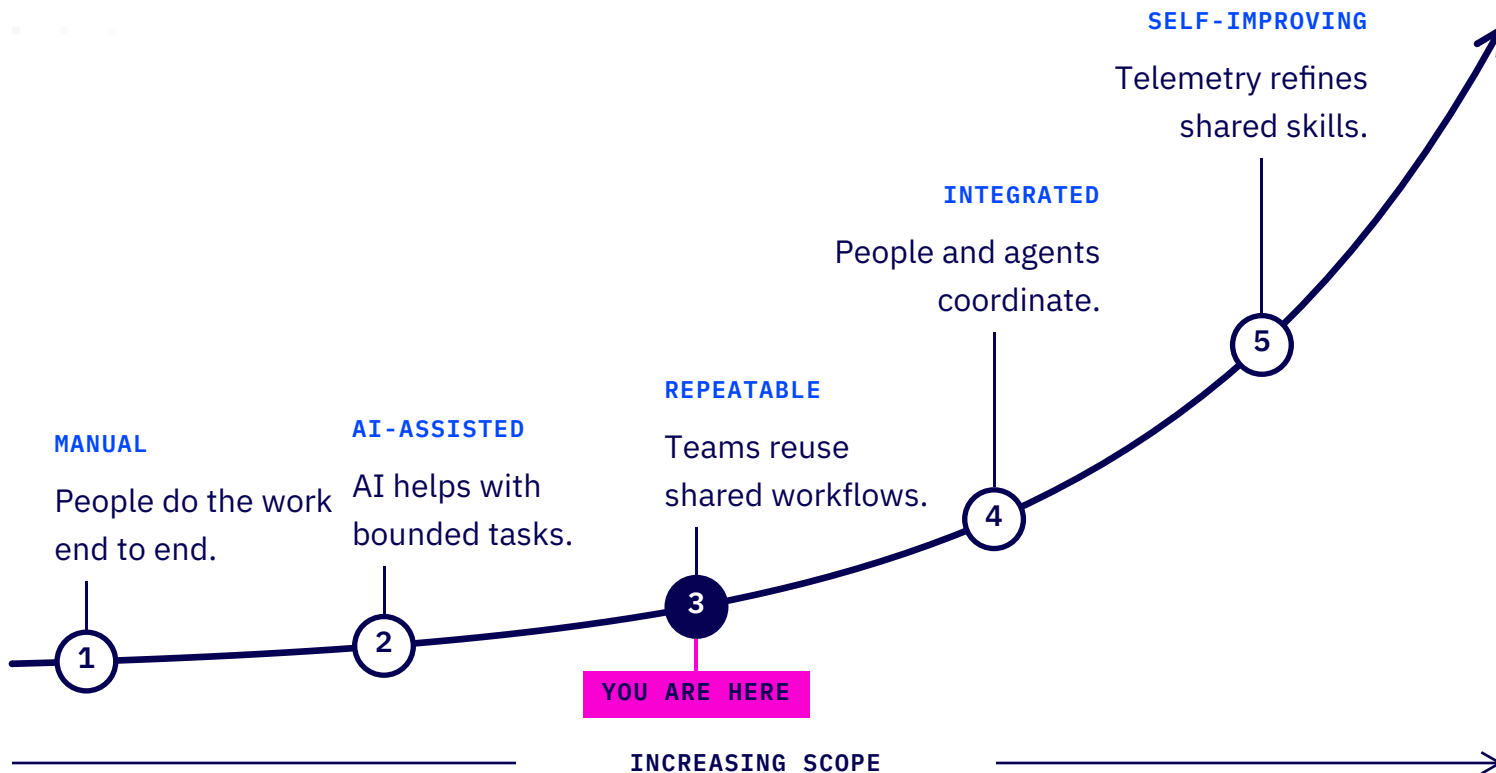
STAGE 4 → 5

- Add telemetry and monitoring agents
- Extend to engineering, SRE and support
- Hand the practice to Northfield

DONE WHEN

Skills improve from delivery data

Northfield is at stage three; stage four needs three moves



TO REACH STAGE FOUR

NAME ONE OWNER PER WORKFLOW

Each shared workflow needs an accountable owner and a review step.

TO REACH STAGE FOUR

DEFINE THE HANDOFFS

Write down where agents hand work to people and back.

TO REACH STAGE FOUR

MEASURE CYCLE TIME

Track one flow metric so gains show up in delivery.

Three lessons shape the climb to AI-native delivery



INSIGHT 1

WORKFLOWS BEFORE TOOLS

Gains stall until teams share workflows, not just licenses.

INSIGHT 2

REVIEW KEEPS PACE

Defined review lets agents take on more of the work safely.

INSIGHT 3

TELEMETRY IS THE UNLOCK

Self-improvement needs delivery data agents can learn from.

Maturity has two axes: how wide and how deep

Go deep, then go wide

Depth is how far AI is built into the workflow. Breadth is how many functions share it.

- Product teams lead at stage three
- Engineering and SRE follow one stage behind
- Support and go-to-market start at stage one

DEPTH OF INTEGRATION

	PRODUCT TEAMS	ENGINEERING AND SRE	WIDER ORGANIZATION
5 SELF-IMPROVING			
4 INTEGRATED FLOW	Target		
3 TEAM PRACTICE	Today	Target	
2 AI-ASSISTED		Today	Target
1 MANUAL			Today

BREADTH ACROSS THE ORGANIZATION

Shaded cells are stages already reached.

MATURITY MODEL

Most of the gain arrives after the inflection



INSIGHT 1

HABITS COME FIRST

Early stages cost effort and show little; hold the line until stage three.

INSIGHT 2

STANDARDS UNLOCK SCALE

A shared method is the inflection that makes later gains compound.

INSIGHT 3

MEASURES CLOSE THE LOOP

Optimizing starts when results feed back into the method.

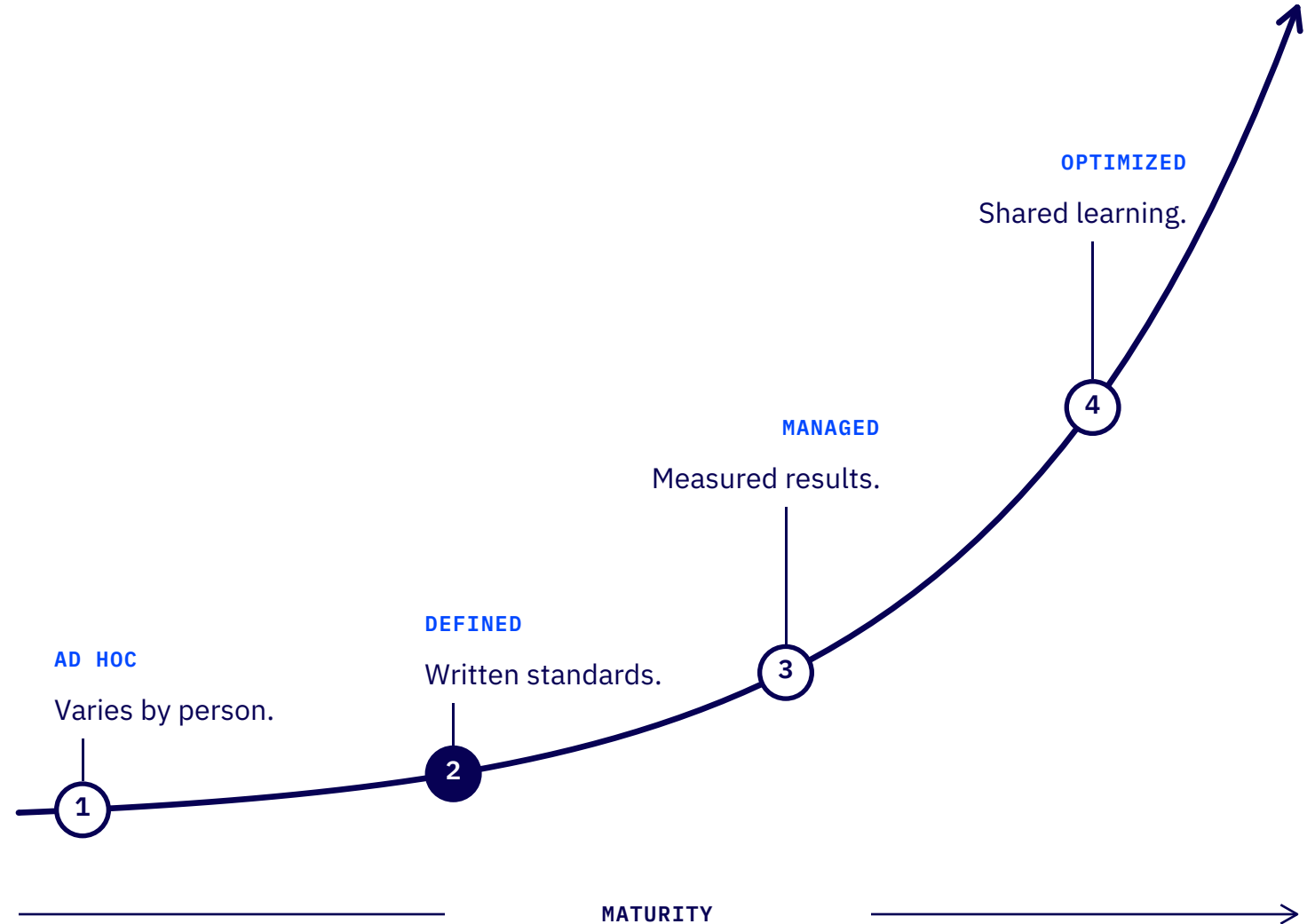
MATURITY MODEL

Most of the gain comes late in the climb

How to read it

The curve stays flat while teams build habits, then rises once the practice is shared.

- Early stages cost effort and show little
- Stage three is the inflection
- Northfield is at stage two today



One method finds and ranks how work should change

PROCESS ANALYSIS ACCELERATOR

What it is, what you get, why it works

WHAT IT IS

A repeatable method that combines process artifacts, operating data, interviews and observation to decide how work should change.

WHAT YOU GET

- Evidence-based view of workflows, handoffs and exceptions
- Prioritized moves to simplify or automate work
- Quantified capacity and value, with assumptions
- Future-state workflows with roles and measures

WHY IT WORKS

- Every recommendation grounded in standards
- Faster discovery through a repeatable method
- Initiatives linked to adoption and Finance
- Outputs trace back to source evidence

#	STEP	WHAT HAPPENS
01	Intake	Scans and registers every available document.
02	Process discovery	Maps the operation into areas and processes.
03	Current-state analysis	Builds process maps and characteristics.
04	Pain points and waste	Finds what optimization and AI can address.
05	Redesign options	Options to eliminate, simplify or automate.
06	Quantify and prioritize	Scores value, capacity, feasibility and risk.
07	Transformation waves	Packages priorities into sequenced initiatives.

Four roles, each with a clear job and a time commitment

01 Sponsor

TIME

2 hours a month

DOES

- Clears blockers
- Speaks at town halls
- Owns go/no-go
- Reviews adoption monthly

02 Change lead

TIME

Full time

DOES

- Runs the network
- Tracks adoption
- Reports weekly
- Escalates blockers

03 Site champion

TIME

2 hours a week

DOES

- Coaches peers
- Relays feedback
- Flags local risks
- Hosts a weekly huddle

04 Super user

TIME

1 hour a week

DOES

- First-line help
- Shares daily tips
- Logs issues
- Joins daily floor huddle

Ratio: one champion for every 35 staff.

Source: Northfield Health System change plan; sample figures for layout only.

A sponsor, a lead and 54 champions reach every floor



- 01 Sponsor**
Removes blockers, speaks at every town hall, owns the go/no-go vote.
- 02 Change lead**
Runs the network, tracks adoption and reports weekly to the steering committee.
- 03 Champions**
Two hours a week; coach peers, gather feedback, relay local risks.
- 04 Super users**
One hour a week; first-line help and a daily tip for their floor.

Each site champion leads a cell of four to five super users who sit on the floor and answer questions in real time.

Source: Northfield Health System change plan; sample figures for layout only.

AI changes how product and engineering operate

WHY AI CHANGES THE OPERATING MODEL

01

Speed changes the unit of work

Drafting, coding, testing and prototyping move faster, so teams can work in smaller, testable slices.

02

Ambiguity becomes the bottleneck

Unclear evidence, intent and acceptance criteria spread through delivery just as quickly.

03

The product interface is the advantage

Shared evidence, review and governance turn isolated gains into repeatable team performance.

HOW WE WILL RUN IT

Guide the process without taking over the work

- Short prompts, then participant work
- Named checkpoints and timeboxes
- Active coaching and unblocking
- Human approval at every gate
- Shared notes captured live
- Open risks logged for follow-up

WHY THIS MATTERS

AI changes how product and engineering operate

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WHAT PARTICIPANTS WILL UNDERSTAND

Six outcomes define the workshop

Frame the right opportunity

Pick a feasible problem

Turn evidence into intent

Stories and criteria

Agents across the SDLC

Plan, code, test, release

Product-to-R&D interface

Agree a demonstrable slice

Compress feedback loops

Prototype in parallel

Scale AI-enabled delivery

Adoption and governance

QUALIFICATIONS

We are deeply embedded across healthcare, with proven work in payer, provider and pharmacy

We deliver modernization through targeted offerings across the healthcare value chain.

TECHNOLOGY ALLIANCES

CLOUD PLATFORM	LAKEHOUSE
CRM	CORE ADMIN
INTEGRATION	BI TOOLS
CONTACT CENTER	IDENTITY
ERP	EHR
DATA QUALITY	GENERATIVE AI
SUPPLY CHAIN	PAYMENTS
REVENUE CYCLE	TELEHEALTH
CYBERSECURITY	INTEROPERABILITY

Industry offerings

PAYER

- Care management modernization
- Claims platform modernization
- Member digital strategy

PROVIDER

- Population health modernization
- Care experience design
- Clinical audit readiness

PHARMACY AND LIFE SCIENCES

- Clinical program management
- Manufacturing excellence
- Commercial enablement

Cross-functional capabilities

ADVANCED ANALYTICS AND AI

Generative AI accelerators · ML delivery · Data architecture · Cloud data strategy

EXPERIENCE STRATEGY

Service design · Persona development · CRM implementation · Market research

ENTERPRISE STRATEGY AND TECHNOLOGY

Application rationalization · IT cost optimization · ERP selection · Cybersecurity

PRODUCT AND WORKFORCE

Product strategy · Software engineering · DevOps · Org change management

Source: West Monroe engagement history.

OFFERINGS

Revenue cycle diagnostic

1 • WHAT WE WILL DO

Find where claims stall between registration and cash, size every leak and leave a fix list the revenue cycle team can start on. We work from 24 months of your own claims, not from benchmarks, and agree every number with Finance before it goes in the plan.

2 • HOW WE WILL DO IT • 6 WEEKS

01 • WEEKS 1 TO 2

Baseline

- Pull 24 months of claims by payer
- Map registration to cash steps
- Set KPI baselines with Finance

02 • WEEKS 3 TO 4

Diagnose

- Root-cause denials and underpayments
- Trace A/R aging by payer
- Validate leaks with billing leads

03 • WEEKS 5 TO 6

Size and plan

- Size each fix: low, base, high
- Sequence by effort and value
- Name owners and start weeks

\$145K fixed fee
6 weeks

Deliverables: KPI baseline, leak heat map, opportunity sizing, 90-day plan. Assumes a claims extract in week 1; no new tools.

Source: Sample figures for layout only; replace with current firm data before use.

PROOF • SAMPLE CLIENT

Sample case: 14 hospitals, 220 clinics.

18% fewer denials

\$11M annual cash opportunity

9d faster to cash

“We stopped debating the numbers and started fixing the top five leaks.”
VP Revenue Cycle, sample client

01 • FRAME • W1 TO 2

- Triage six sources; agree naming
- Set security and access standards
- Design the target model

02 • BUILD • W3 TO 7

- Pipelines for six sources, with tests
- Model the core with data owners
- Weekly review of lineage and quality

03 • PROVE • W8 TO 9

- Build two priority reports
- Validate numbers with Finance and clinical
- Fix gaps before go-live

04 • HAND OVER • W10

- Run book and monitoring
- Train 30 report builders
- Backlog for the next quarter

OFFERINGS

Data platform jumpstart, proven at a health system

1 • WHAT WE WILL DO

A governed starter platform on real Northfield data in ten weeks: six sources ingested, a tested core model and two live reports that finance and clinical leaders both use, built beside your team so you can extend it.

FIXED FEE

\$240K

fixed fee, 10 weeks
3 invoices, net 30

DELIVERABLES

- Platform blueprint and standards
- Tested core model, six sources
- Two live reports, run book

Team: lead, architect, two engineers, analyst. Assumes cloud tenant by week 2; up to six sources.

14

reports on one
platform

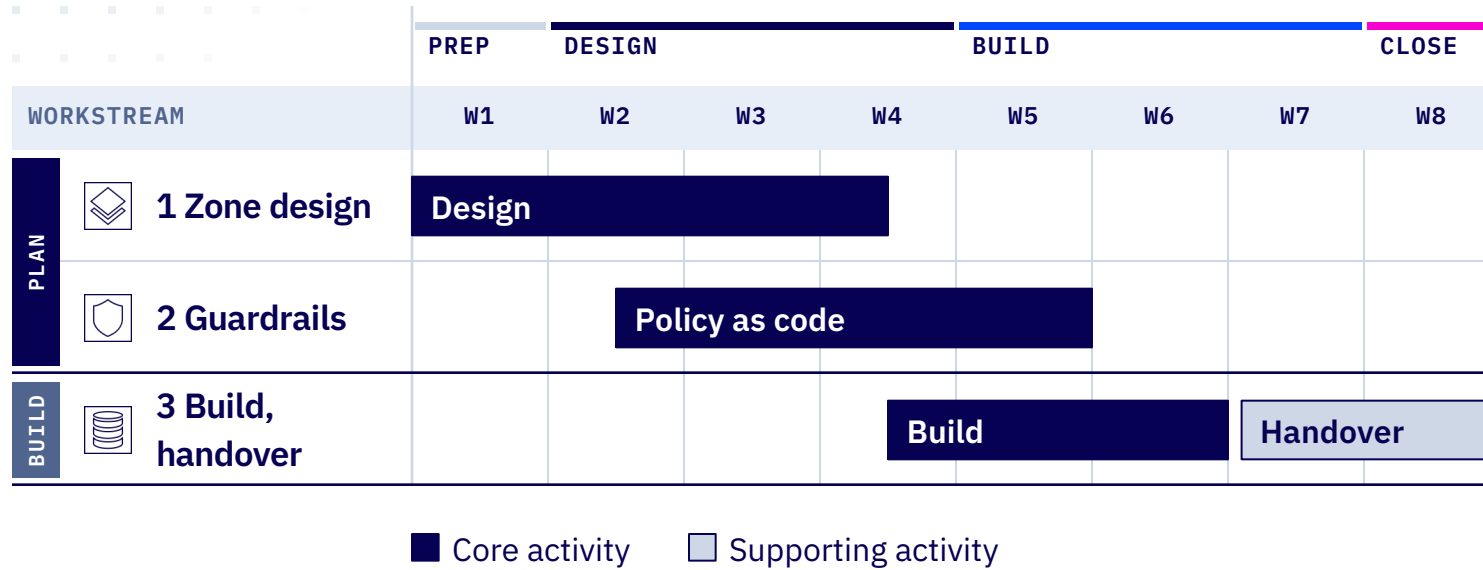
- 32%

less manual
rework

“One source we all trust.”
CFO, sample client

Source: Sample figures for layout only; replace with current firm data before use.

Cloud landing zone sprint: eight weeks to production



1 • ZONE DESIGN • W1 TO 4

- Network and account design
- Identity and tagging standards
- Three environments agreed

2 • GUARDRAILS • W2 TO 5

- Security baselines as code
- Drift and compliance checks
- Security lead sign-off

3 • BUILD, HANDOVER • W4 TO 8

- Pipelines tested in all three
- Operations team trained
- Readout and cost baseline

WHAT WE WILL DO

A secure, governed cloud foundation Northfield can build on: designed, built and tested in eight weeks, then handed over with runbooks.

FIXED FEE

\$210K

fixed fee, 8 weeks
Invoiced at weeks 4, 8

Team: lead 25%, architect, two engineers

DELIVERABLES

- Landing zone in production
- Policy-as-code library
- Runbooks and cost baseline

Assumes tenant and SSO ready in week 1

Source: Sample figures for layout only; replace with current firm data before use.

OFFERINGS

Six weeks to a clear scale or stop call

WHAT WE WILL DO

Test one AI use case on your own data before you commit to scale. We build and measure a working prototype, then give you a clear scale-or-stop call.

FIXED FEE

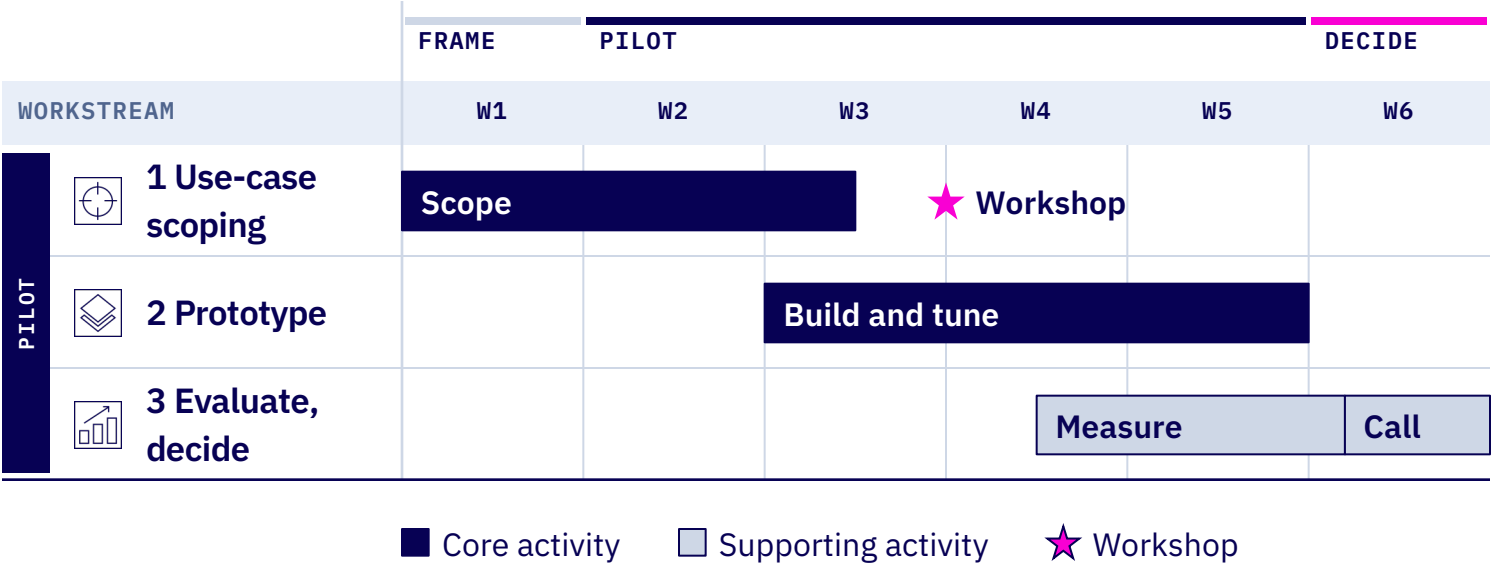
\$150K

fixed fee
6 weeks

DELIVERABLES

- Working prototype on Northfield data
- Evaluation scorecard
- Scale-or-stop recommendation

Source: Sample figures for layout only; replace with current firm data before use.



1 • SCOPING • W1 TO 3

- Pick one use case and success measure
- Map data, risks and clinical owners
- Workshop to agree the scope

3 • EVALUATE, DECIDE • W4 TO 6

- Score accuracy, safety and usability
- Test with 10 clinical users
- Scale-or-stop case with costed options

2 • PROTOTYPE • W3 TO 5

- Build on a slice of Northfield data
- Tune prompts, models and guardrails
- Weekly demo to the clinical lead

TEAM AND ASSUMPTIONS

- Lead 25%, engineer, analyst, advisor
- One use case chosen in week 1
- Clinical lead 4 hours a week

AI readiness assessment: one page, one fixed fee

1 • WHAT WE WILL DO

Health systems are piloting AI faster than they can govern it. In eight weeks we assess data, people, risk and use cases across Northfield, rank what to do first and leave a funded, sequenced first wave.

FIXED FEE

\$185K

fixed fee, 8 weeks
2 invoices, net 30

2 • HOW WE WILL DO IT • 8 WEEKS

01 • WEEKS 1 TO 2

Align

- Confirm sponsor goals and risk appetite
- 20 stakeholder interviews
- Agree scoring criteria

DELIVERABLES

Readiness scorecard, ranked use-case portfolio, governance model, 90-day roadmap

02 • WEEKS 3 TO 5

Assess

- Scan data, platform and talent against five domains
- Score each domain 1 to 5
- Compare with peer systems

03 • WEEKS 6 TO 7

Prioritize

- Long-list 30 use cases with clinical and finance leads
- Rank on value, risk and readiness
- Agree the top ten

ASSUMPTIONS

Data access and a data owner in week 1; sponsors decide in five days; no model build

04 • WEEK 8

Plan

- Governance and risk model with review gates
- 90-day roadmap with owners
- Business case and readout

Source: Sample figures for layout only; replace with current firm data before use.

OFFERINGS

Cloud cost review: four weeks, \$75K fixed

1 • WHAT WE WILL DO

A cloud cost review finds the 20 to 30 percent of Northfield's cloud spend that nobody owns. In four weeks we map every dollar to an owner, find the waste and commitment gaps, and leave a ranked savings backlog.

2 • HOW WE WILL DO IT • 4 WEEKS

01 • WEEK 1

Collect

- Read-only billing and usage access
- Pull 12 months of spend data
- Agree the account hierarchy

02 • WEEK 2

Map

- Tag and map spend to owners
- Flag untagged and shared cost
- Validate with account owners

03 • WEEK 3

Analyze

- Find idle and oversized assets
- Check commitment coverage
- Estimate savings per lever

04 • WEEK 4

Decide

- Rank savings by effort and risk
- Draft tagging and budget model
- Readout for the CFO and CIO

3 • FIXED FEE

\$75K

4 weeks

DELIVERABLES	TEAM	ASSUMPTIONS
Spend map by service and owner; ranked savings backlog	Lead, FinOps analyst, cloud engineer	Billing access in week 1; up to 40 accounts; savings are estimates

Source: Sample figures for layout only; replace with current firm data before use.

Finance close accelerator: what, how and what it costs

1 • WHAT

The offer and what you keep

Close slips because reconciliations, approvals and journal entries run in sequence. We map the close, remove the longest delays and put a five-day target calendar in place, so Finance spends days on analysis, not on chasing.

WHAT YOU KEEP

- Close map with a delay heat map
- Redesigned close calendar and RACI
- Approval rules, dashboard, playbook

2 • HOW

Three phases, ten weeks

01 • WEEKS 1 TO 3

Diagnose

- Time every close task end to end
- Interview 12 preparers, approvers
- Rank the 10 longest delays

02 • WEEKS 4 TO 7

Redesign

- Parallelize reconciliations
- Set approval thresholds and SLAs
- Pilot the calendar on one close

03 • WEEKS 8 TO 10

Embed

- Run a full close on the new design
- Train 40 preparers and approvers
- Hand over dashboard and playbook

3 • COST

\$120K

fixed fee

Ten weeks, three milestone invoices at weeks 3, 7 and 10

Fees exclude expenses; net 30

TEAM AND ASSUMPTIONS

Lead 25%, close SME and analyst full time. Needs ledger access in week 1.

Source: Sample figures for layout only; replace with current firm data before use.

OFFER	HOW WE DO IT	FIXED FEE
AI readiness scan Strategy	. Interview 8 leaders . Scan data, risk, talent . Rank top five use cases	\$65K 3 weeks
AI readiness assessment Strategy	. Score five domains vs. peers . Rank 30 use cases . Governance and 90-day plan	\$185K 8 weeks
Revenue cycle diagnostic Finance	. Baseline 24 months of claims . Root-cause denials and A/R . Size fixes; 90-day plan	\$145K 6 weeks
Data platform jumpstart Data	. Frame sources and standards . Build core model, six sources . Prove with two live reports	\$240K 10 weeks
Cloud landing zone sprint Cloud	. Design zone and guardrails . Build and test pipelines . Train and hand over	\$210K 8 weeks
Cloud cost review Cloud	. Pull billing and usage data . Map spend to owners . Rank savings by effort	\$75K 4 weeks

OFFERINGS

Six packaged offers, one fixed fee each

WHAT WE WILL DO

Pick a package, not a proposal. Each offer is a fixed fee for a fixed time, with named deliverables.

EVERY OFFER INCLUDES

- A named lead and team
- Weekly sponsor check-in
- Readout and handover pack

HOW OFFERS CHAIN

- Scan, then assessment, then pilot
- Diagnostic, then jumpstart

Assumes data access in week 1. Fees exclude expenses.

Source: Sample figures for layout only; replace with current firm data before use.

Cloud landing zone: pick the package that fits

1 • WHAT WE WILL DO

Three packages for a secure, governed cloud foundation: review it, build it, or build it and move the first workloads.

2 • HOW WE DO IT

- Review the estate against the design
- Design and build the landing zone, guardrails as code
- Prove it on workloads; hand over

ASSUMPTIONS

- Tenant and SSO ready in week 1
- Up to three environments
- Northfield holds the licenses

WHAT IS INCLUDED	A • REVIEW	B • SPRINT	C • PROGRAM
Current-state review	✓	✓	✓
Target landing zone design	✓	✓	✓
Security guardrails as code	—	✓	✓
Production landing zone	—	✓	✓
Runbooks and cost baseline	—	✓	✓
Migration wave plan	—	—	✓
First two waves migrated	—	—	✓
Hypercare, 60 days	—	—	✓
FIXED FEE Team: lead plus architects and engineers	\$75K 3 weeks, fixed	\$210K 8 weeks, fixed	\$480K 16 weeks, fixed

Source: Sample figures for layout only; replace with current firm data before use.

Three ways to start with AI, each a fixed fee

SKU 1 • SCAN

AI readiness scan

Where do we stand? A rapid read of data, risk and talent.

HOW WE DO IT

- Interview 8 leaders
- Scan data, risk and talent
- Rank the top five use cases

YOU KEEP

Scorecard, top five use cases, two-page summary

\$65K

fixed fee, 3 weeks · lead, analyst

SKU 2 • ASSESSMENT

MOST CHOSEN

AI readiness assessment

What should we do first? Full portfolio, governance and roadmap.

HOW WE DO IT

- Align on goals; 20 interviews
- Score five domains vs. peers
- Rank 30 use cases; plan 90 days

YOU KEEP

Scorecard, ranked portfolio, governance model, roadmap

\$185K

fixed fee, 8 weeks · lead, architect, 2 analysts

SKU 3 • PILOT

AI use-case pilot

Can it work here? Scope, build and test one use case.

HOW WE DO IT

- Scope one use case; set measures
- Build and tune on your data
- Evaluate; scale-or-stop call

YOU KEEP

Working prototype, scorecard, scale-or-stop case, handover pack

\$150K

fixed fee, 6 weeks · lead, engineer, analyst

Source: Sample figures and names for layout only; replace with current firm data and catalog before use.

OFFERINGS

Managed cloud: one team, one monthly run-rate

1 • WHAT WE WILL DO

Run the Northfield cloud estate for you: patching, security posture, cost and a monthly roadmap session, delivered by one named team on one monthly run-rate.

Up to 40 cloud accounts; three-month minimum, then 30 days' notice.

TEAM MONTHLY RUN-RATE
\$88K per month, whole team
In arrears

2 • HOW WE WILL DO IT • EVERY MONTH

01 • WEEK 1

Operate

- Patch and upgrade cycle
- Incident and ticket review
- Backup and recovery checks

02 • WEEK 2

Secure

- Posture scan and findings
- Access and key reviews
- Fix plan with owners

03 • WEEK 3

Optimize

- Cost and tagging review
- Right-size and commit decisions
- Savings tracked to target

04 • WEEK 4

Plan

- Health report for the CIO
- Roadmap and backlog session
- Next-month priorities

TEAM	Cloud lead · 25% Service owner	Platform engineer · 100% Operate, patch	Security engineer · 50% Posture, access	FinOps analyst · 50% Cost, tagging
------	-----------------------------------	--	--	---------------------------------------

Source: Sample figures for layout only; replace with current firm data before use.

A four-month data team, one \$96K monthly run-rate

1 • WHAT WE WILL DO

A standing team that ships a governed data platform in four months and leaves Northfield able to run it. We build, you learn by pairing.

Assumes the cloud tenant is ready in month 1; four-month minimum term.

TEAM MONTHLY RUN-RATE

\$96K

per month, whole team
Billed monthly

2 • HOW WE WILL DO IT • FOUR MONTHS

01 • MONTH 1	02 • MONTH 2	03 • MONTH 3	04 • MONTH 4
Frame and mobilize - Stand up the team and the working rhythm - Triage sources; agree standards - Design the target platform	Build the core - Build ingestion and the modeled core - Write data quality tests - Pair with Northfield engineers	Prove with reports - Deliver two priority reports - Validate numbers with Finance - Stand up catalog and ownership	Hand over - Run book and on-call guide - Train owners and report builders - Backlog for the next six months
TEAM Engagement lead · 25% Steering, quality	Data architect · 100% Platform design	Data engineers · 100% Pipelines, tests	Governance analyst · 50% Standards, catalog

Source: Sample figures for layout only; replace with current firm data before use.

Three ways to connect 14 systems; the bus wins

OPTION A

Keep point-to-point

PROS

- Known to every interface team
- No new platform to run

CONS

- Feeds grow with every new system
- Failures hard to trace

2.6 / 5

Fit 3 · Cost 2 · Risk 2

OPTION B

Managed event bus

PROS

- One place to monitor and replay
- New systems connect in weeks

CONS

- A new service to operate
- Event design skills needed

4.1 / 5

Fit 5 · Cost 3 · Risk 4

CHOSEN

OPTION C

Integration platform suite

PROS

- Rich tooling and connectors
- Vendor-supported runtime

CONS

- Highest licence cost
- Locks flows into one product

3.3 / 5

Fit 4 · Cost 2 · Risk 3

Why we chose it: Option B fits 14 systems at a manageable cost and removes the feed growth behind most incidents.

General availability

Make governance executable

Scale depends on clear decision rights, auditable controls and operational handoffs, not one-time enablement.

- Named approvers per decision
- SLA tracking on every request
- Access controls by pattern
- Adoption measured by team
- Exceptions reviewed quarterly

Governance runs through rollout and optimization

TWO CRITICAL PATHWAYS TO OPERATIONALIZE PLATFORM GOVERNANCE

01 Request intake: team to approved pattern access



02 Use-case intake: idea to governed build



Result: controlled scale, approval-only tool access and a repeatable model.

1 Designed to align with Northfield's governance structure, data enablement team and operational support functions.

Three patterns define the lakehouse role

Pattern 1: Lakehouse as the analytics layer over the existing warehouse

Curated warehouse → Semantic services → BI layer

ROLE: Native analytics and consumption over data governed in the warehouse

Pattern 2: Lakehouse as the consumption layer for the engineering platform

Engineering platform → Curated layer → BI layer

ROLE: Business consumption while engineering and curation stay put

Pattern 3: End-to-end lakehouse platform

Ingestion and engineering → Lakehouse → Semantic and reporting

ROLE: One platform for qualified net-new workloads

Key decisions to validate

- Data access versus movement
- Semantic model ownership
- Security and lineage

Key decisions to validate

- Open formats and shortcuts
- Metadata and lineage sync
- Curated layer ownership

Key decisions to validate

- Fit with placement criteria
- Capacity and isolation
- Engineering standards

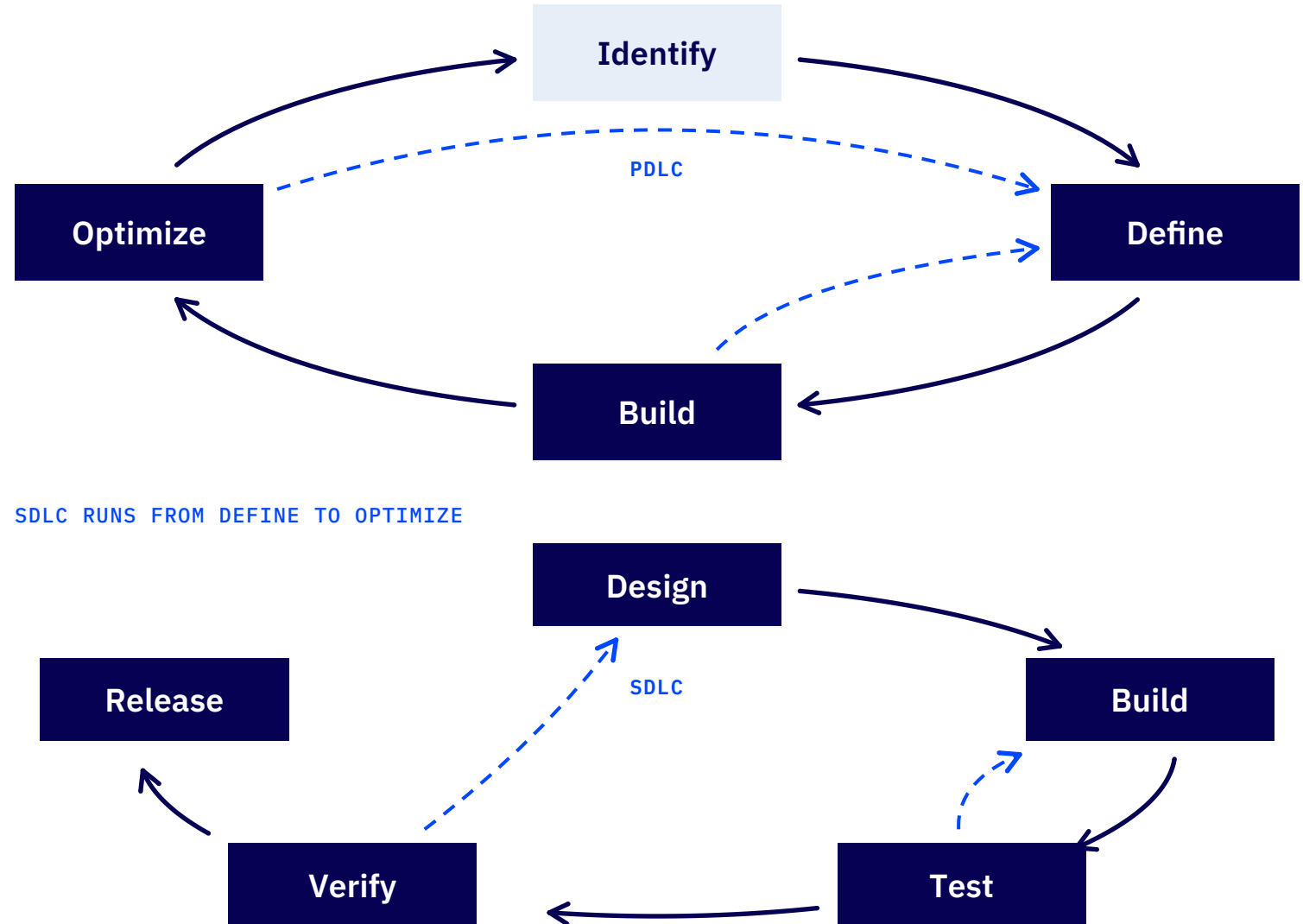
Source: West Monroe analysis of the Northfield data estate.

The SDLC is a slice of the PDL

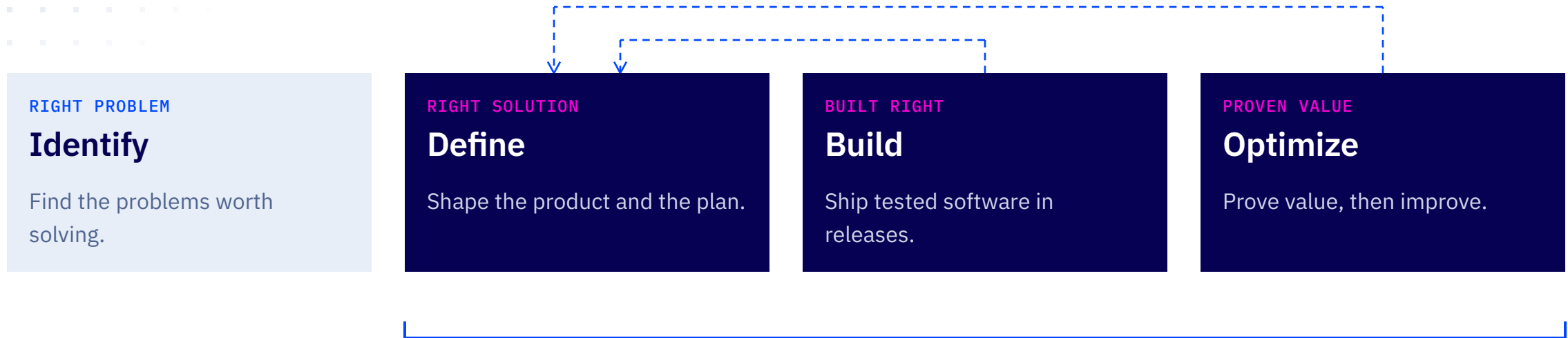
- **PDL.** The superset, from problem to proven value.
- **SDLC.** The subset that engineers and tests the product.

WHERE THE SDLC SITS

- Design in Define
- Build, Test, Verify in Build
- Release in Optimize



The SDLC is the slice of the PDLC from Define to Optimize

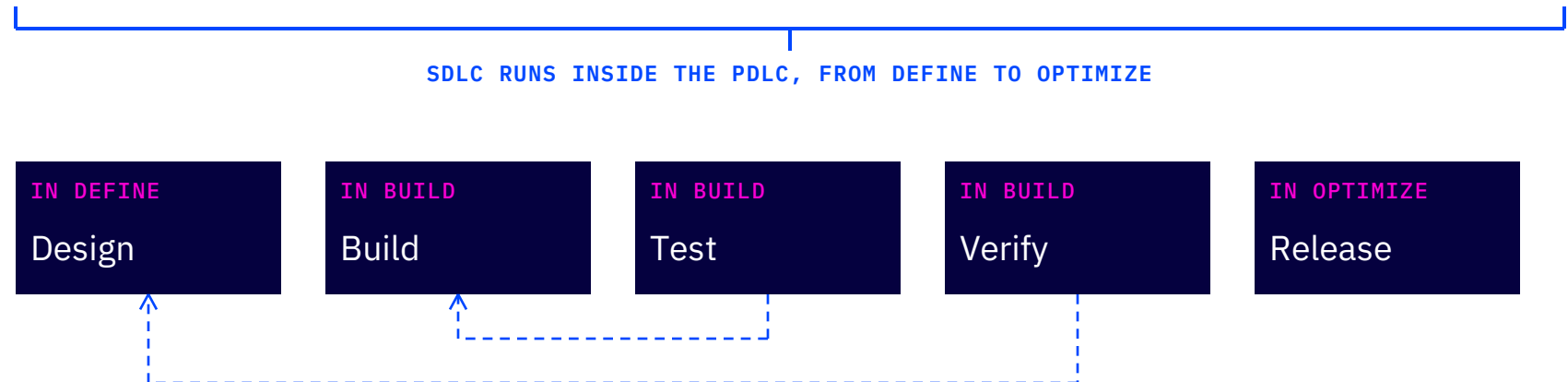


PDLC: THE SUPERSET

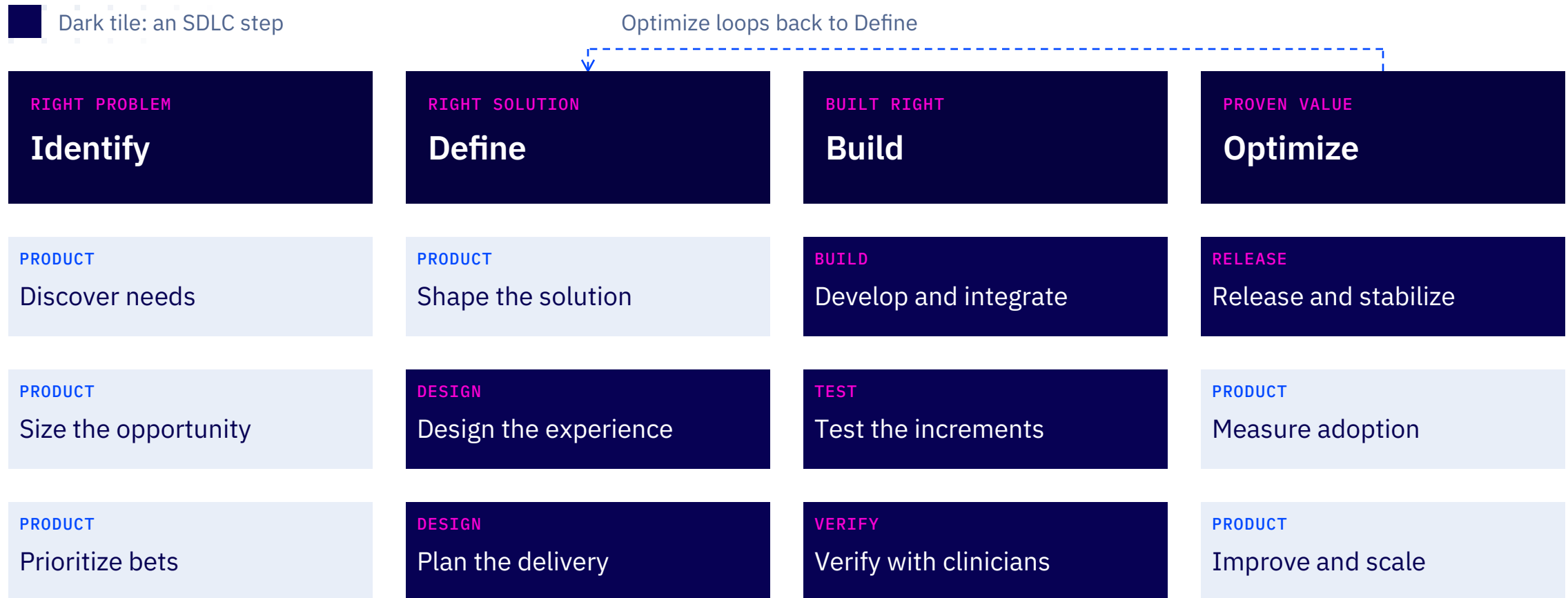
Four phases that loop back. Identify happens before engineering starts.

SDLC: A SUBSET

Five steps that loop back on defects.



Each phase has three sub-phases; the SDLC covers six



OUR APPROACH

Phase 01

Data Platform Foundation

Weeks 0–16



Early-adopter scorecard

STRATEGIC VALUE

Outcomes and sponsorship

PATTERN COVERAGE

Warehouse, lakehouse or both

TECHNICAL READINESS

Data, security, dependencies

TEAM READINESS

Owner, engineers, timeline

LEARNING VALUE

Reusable patterns and controls



Three workloads prove the platform patterns

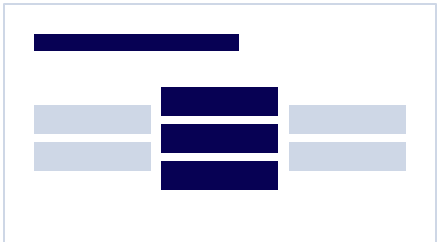
Selected to maximize enterprise learning and to test each architecture pattern once.

WAREHOUSE A TO PLATFORM

Analytics

Build analytics models on curated warehouse data.

Validates data movement and lineage.



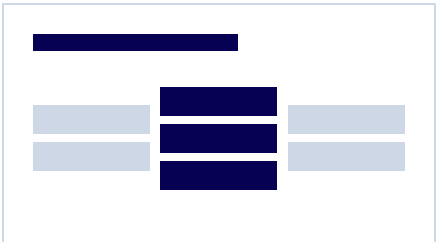
Warehouse A architecture

LAKEHOUSE B TO PLATFORM

Reporting

Rebuild reports and dashboards on curated data.

Validates ownership of curated layers.



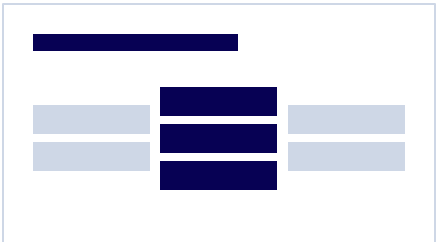
Lakehouse B architecture

END TO END ON PLATFORM

Engineering

Net-new ingestion, curation and BI for one use case.

Validates engineering standards.



End-to-end architecture

OUR APPROACH

Phase 01

Data Platform Foundation

Weeks 0–16



Objective

Stand up and validate the enterprise data platform: target architecture, security, governance and onboarding patterns for the first finance workloads.

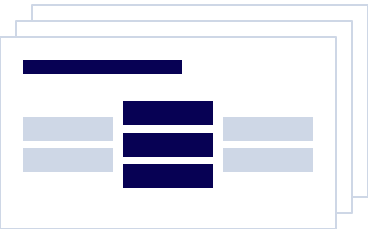
Phase 01 exit

- Architecture patterns approved
- Foundation environments live
- Integrations demonstrated
- Three pilot workloads chosen

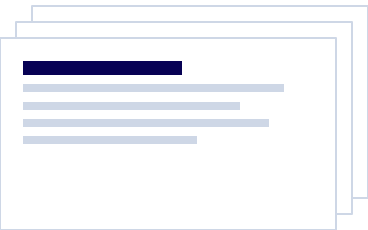


STREAM	ACTIVITIES	WORK PRODUCTS
Design	. Define target architecture and capacity . Create the placement framework	. Reference architecture . Placement framework
Build	. Configure environments and pipelines . Set up health and cost telemetry	. Configured environments . Telemetry baseline
Govern	. Define security and catalog patterns . Set lineage and classification rules	. Access control framework . Lineage standards
Launch	. Define the service catalog and onboarding . Set adopter criteria; plan Phase 02	. Service catalog and kit . Early-adopter cohort

Deliverables



Onboarding framework



Reference design

Assumptions

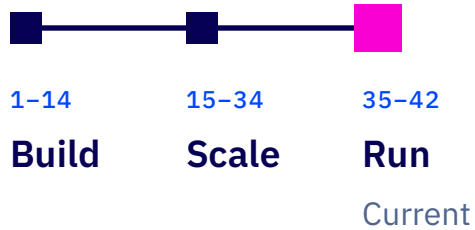
- Access is provided
- Pilots test patterns only

APPROACH

Phase 3 hands platform ownership to the Northfield team

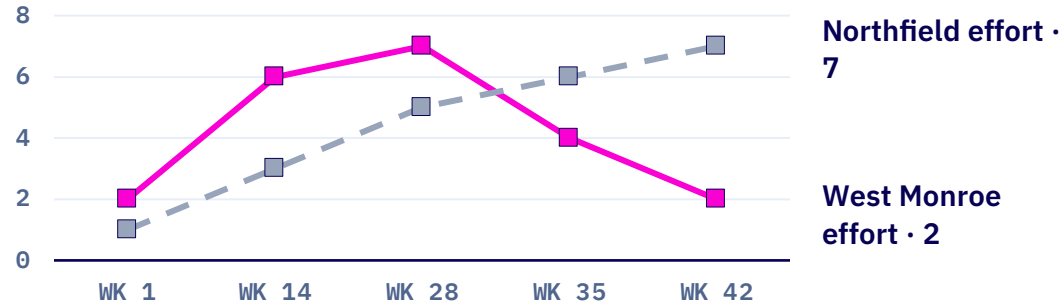
PHASE 03 · WEEKS 35-42

General availability and optimization



EXIT CRITERIA

- Runbooks accepted
- Northfield on call
- Benefits tracked
- Risks transferred



West Monroe tapers its driving role so the Northfield core team owns execution. Effort crosses at about week 30, when Northfield staff lead more work than West Monroe.

Phase 2 · Scalability and early adoption

Phase 3 · General availability

West Monroe team

Deploying provisioning and monitoring, and onboarding the first business units.

Optimizing performance, cost and adoption while transferring ownership and operations.

Northfield team

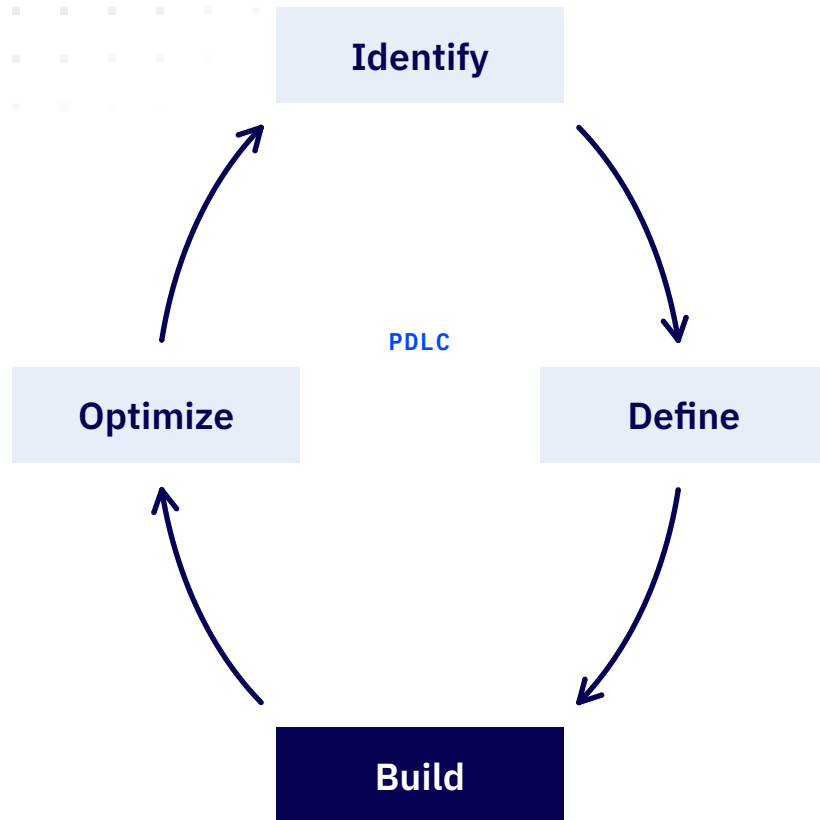
Refining standards and overseeing the early adopter program.

Dedicated staff are learning to support the platform and beginning to lead.

Evidence from three early adopters decides whether the platform is ready for general availability

01 Platform	02 Operations	03 Governance	04 Service	05 Financial
EVIDENCE REQUIRED - Patterns validated - Integrations reliable - Exceptions understood - Standards adopted	EVIDENCE REQUIRED - Provisioning automated - Telemetry active - Runbooks exercised - Alerts routed	EVIDENCE REQUIRED - Controls applied - Lineage and evidence - Access reviews done - Gaps closed	EVIDENCE REQUIRED - Ownership clear - Support tiers tested - Onboarding working - Handover agreed	EVIDENCE REQUIRED - Cost visible - Demand forecastable - Value demonstrated - Savings tracked
ILLUSTRATIVE EXIT Priority patterns approved for GA	ILLUSTRATIVE EXIT Core services supportable	ILLUSTRATIVE EXIT No unresolved critical gaps	ILLUSTRATIVE EXIT Support accepted by Northfield	ILLUSTRATIVE EXIT Sustainable operating basis
DECISION OWNER Chief architect	DECISION OWNER Platform owner	DECISION OWNER Governance lead	DECISION OWNER Service owner	DECISION OWNER Finance sponsor

Build ships tested software in small releases



Dark phase is the one shown on the right.

PHASE 3 OF 4 • BUILT RIGHT

Build

Deliver the patient app and scheduling platform in two-week sprints with automated checks.

ACTIVITIES

- Build in two-week sprints
- Automate tests and releases
- Verify with clinicians

OUTPUTS

- Working releases
- Test evidence
- Release notes

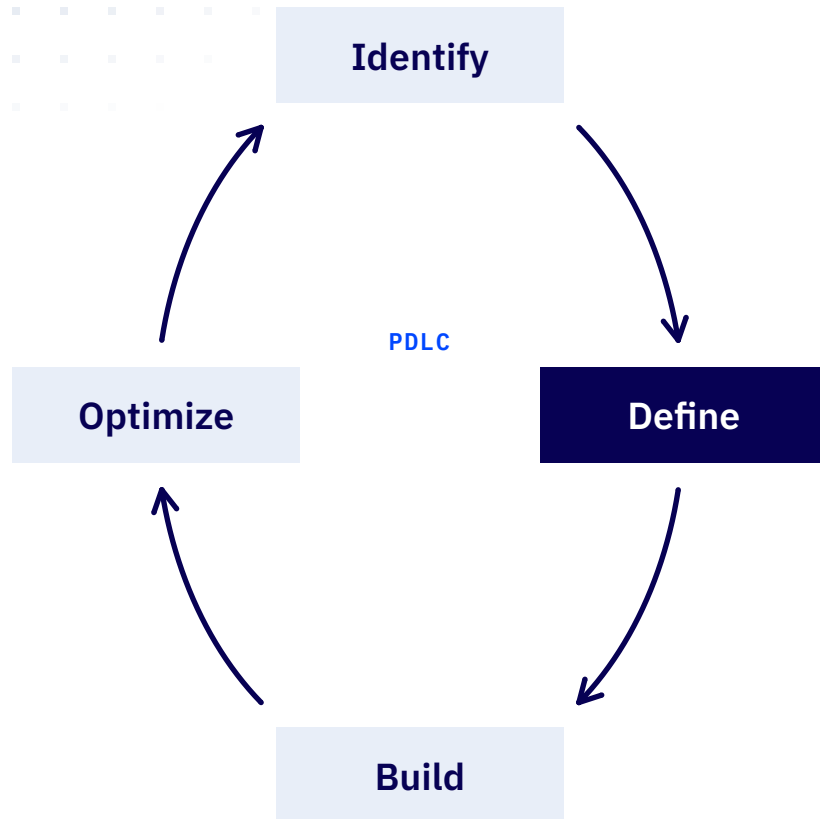
ROLES

- Engineering lead
- QA lead
- Release manager

EXIT GATE

Release approved for patients

Define turns a ranked problem into a plan



Dark phase is the one shown on the right.

PHASE 2 OF 4 • RIGHT SOLUTION

Define

Shape the product, the patient experience and the delivery plan with clinicians.

ACTIVITIES

- Shape the vision and prototypes
- Design the booking journeys
- Plan releases and risks

OUTPUTS

- Product vision
- Release plan
- Architecture

ROLES

- Product owner
- Solution architect
- UX designer

EXIT GATE

Approved scope and plan

Our **three-phase** approach turns a defined architecture into a Northfield-owned enterprise service



Source: Fixed platform scope is priced once; workload scope scales with the number of early adopters.

Three phases from baseline to a funded roadmap

Phase 1 · Baseline

6–8 weeks · establish the fact base

ACTIVITIES

- Gather data and benchmarks
- Interview finance leaders
- Size the opportunity by function
- Map workflows and handoffs
- Test findings with sponsors

DELIVERABLES

- Baseline and benchmark report
- Opportunity sizing model
- Fact-base readout

Phase 2 · Design

2–3 weeks · shape the options

ACTIVITIES

- Develop recommendations per area
- Set savings and value targets
- Define the target mix of roles
- Rank options by impact and effort
- Align leaders on the options

DELIVERABLES

- Prioritized value opportunities
- Target role-mix view
- Value-target summary

Phase 3 · Roadmap

1–2 weeks · agree the plan

ACTIVITIES

- Facilitate a trade-off workshop
- Build the integrated roadmap
- Sequence initiatives by value
- Name owners and dependencies
- Read out the priorities

DELIVERABLES

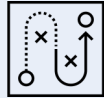
- Value-creation roadmap
- Initiative owner register
- Executive readout deck

AI-powered modernization compresses time and risk



Value first

- We rank work by business value, not technical elegance.
- Every decision weighs complexity.
- Return comes first.



Purpose built

- We choose retain, replatform, rewrite or retire per system.
- Stable components stay in place.
- Nothing is rebuilt for show.



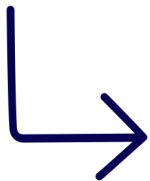
AI accelerated

- Agents scan and map codebases at scale.
- Evidence, planning and tooling stay connected.
- Months of analysis take days.



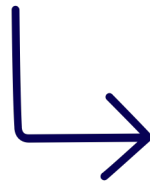
Co-build and scale

- Co-sourced teams sit with your stakeholders.
- Onshore and nearshore flex with demand.
- Knowledge transfer is built in.



WHY THIS MATTERS

Effort goes where return is highest, so promised value actually lands.



WHY THIS MATTERS

We design around your portfolio, keeping what works and replacing only what drives value.



WHY THIS MATTERS

Analysis that takes months is done in days, with less reliance on tribal knowledge.



WHY THIS MATTERS

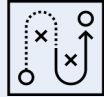
You get capacity without disruption, and a team that can run the result on its own.

AI-powered modernization compresses time and risk



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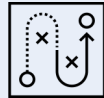


Value first

- Rank work by business value.
- Weigh cost, risk and revenue together.
- Fund quick wins first.
- Track value per release.
- Review progress with sponsors each quarter.

WHY THIS MATTERS

Effort goes where return is highest.



Purpose built

- Choose retain, replatform, rewrite or retire.
- Keep stable components in place.
- Fit the target to your portfolio.
- Nothing rebuilt for show.
- Plan system by system.

WHY THIS MATTERS

You keep what works, replace what drives value.



AI accelerated

- Agents scan and map codebases.
- Dependencies surface in days.
- Generated tests back each change.
- Engineers approve every merge.

WHY THIS MATTERS

Months of analysis done in days.



Co-build and scale

- Co-sourced teams sit with stakeholders.
- Onshore and nearshore flex with demand.
- Pairing builds your team's skills.
- Handover is planned from day one.

WHY THIS MATTERS

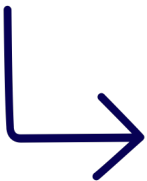
Capacity without disruption.

AI-powered modernization compresses time and risk



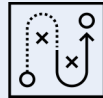
Value first

- We rank every system by business value, not technical elegance.
- Cost, risk and revenue exposure scored side by side.
- Quick wins funded first, so savings pay for later waves.



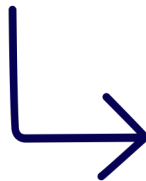
WHY THIS MATTERS

Effort goes where return is highest, so promised value lands.



Purpose built

- Retain, replatform, rewrite or retire, chosen per system.
- Stable components stay in place, untouched.
- Target architecture fits your portfolio and skills.



WHY THIS MATTERS

You keep what works and replace only what drives value.



AI accelerated

- Agents scan and map codebases at scale.
- Dependencies and dead code surface in days.
- Generated tests and docs back each change.



WHY THIS MATTERS

Months of analysis shrink to days, with less tribal knowledge.

AI-powered modernization compresses time and risk

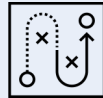


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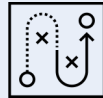


Value first

- **Ranked by value.** Cost, risk and revenue exposure scored per system.
- **Quick wins first.** Fund later waves.
- **Dependencies weighed.** Complexity enters every sequencing call.
- **Per release.** Value measured.
- **Business owned.** Quarterly sign-off.

WHY THIS MATTERS

Effort goes where return is highest, so promised value lands.



Purpose built

- **Four paths.** Retain, replatform, rewrite or retire, per system.
- **Stable stays put.** Working parts stay.
- **Fit to portfolio.** Target matches your skills and vendors.
- **Evidence based.** Usage, defects, cost.
- **No rebuild for show.** Return first.

WHY THIS MATTERS

You keep what works and replace only what drives value.



AI accelerated

- **Scan at scale.** Agents map code, data flows and dependencies.
- **Days, not months.** Discovery in weeks.
- **Tests generated.** Every change ships with tests and docs.
- **Humans decide.** Engineers review each merge.
- **Knowledge kept.** Findings in a shared record.

WHY THIS MATTERS

Months of analysis shrink to days, with less tribal knowledge.

OUR APPROACH

AI-powered modernization compresses time and risk

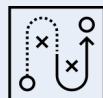


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WHY THIS MATTERS

Months of analysis shrink to days, with less tribal knowledge.

Why modernization has been slow and costly

Coding is only one part. The harder work is rediscovering, deciding and cutting over safely.

DISCOVER AND DEFINE

01 Reconstruct business behavior

Document current processes, rules and workarounds.

02 Decide what to retain

Separate essential controls from obsolete features.

03 Align business and technology

Turn operator intent into shared designs and tests.

DELIVER AND TRANSITION

04 Untangle dependencies

Map integrations, then clean duplicate and incomplete data.

05 Validate during cutover

Run acceptance tests while old and new systems coexist.

06 Execute a safe cutover

Sequence migration, rollback and controls to protect operations.

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Two options, and the one we recommend

OPTION A • DESIGN ONLY

Diagnose and design

Prove the case first, then decide on build.

- ☒ Close diagnostic
- ☒ Target design
- ☐ Wave 1 build

\$720K

fixed fee, 8 weeks

RECOMMENDED

OPTION B • DESIGN AND BUILD

Diagnose, design, build

Carry one team through to Wave 1.

- ☒ Close diagnostic
- ☒ Target design
- ☒ Wave 1 build

\$720K + \$210K/mo

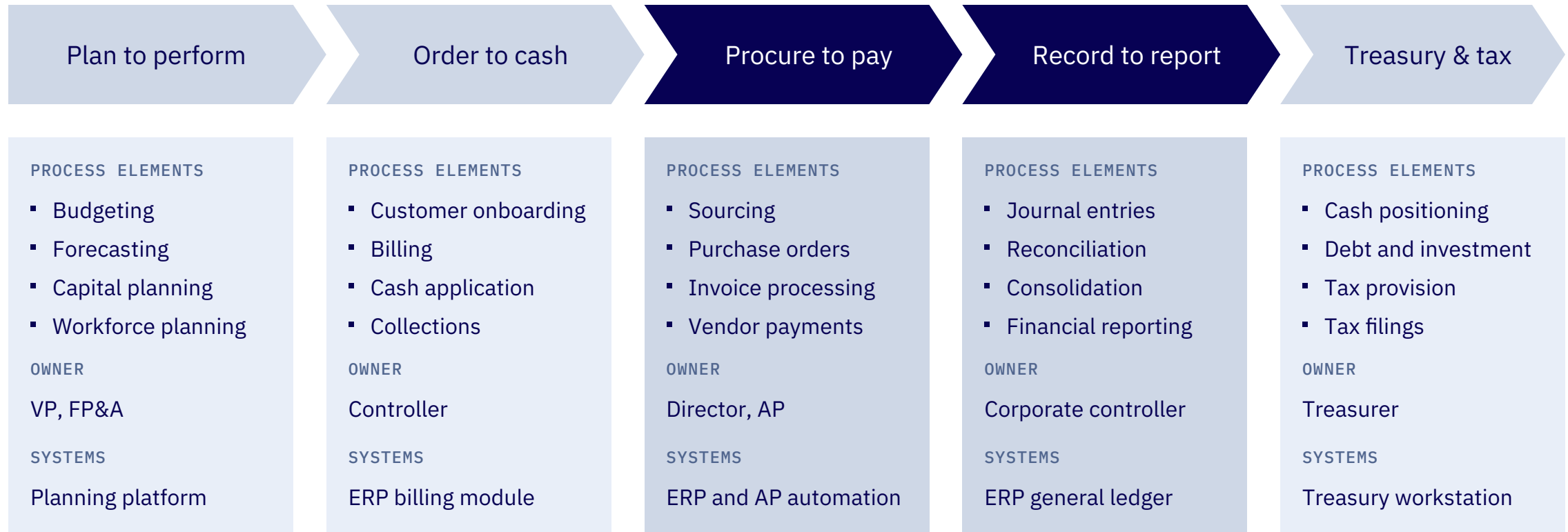
fixed, then run-rate

WHY OPTION B

- 01 No hand-off gap**
The design team stays to build what it designed.
- 02 Fixed where known**
Design is a fixed fee; build flexes with demand.
- 03 Earlier benefit**
Wave 1 goes live 12 weeks sooner.

Decision by 16 October keeps the Wave 1 start date.

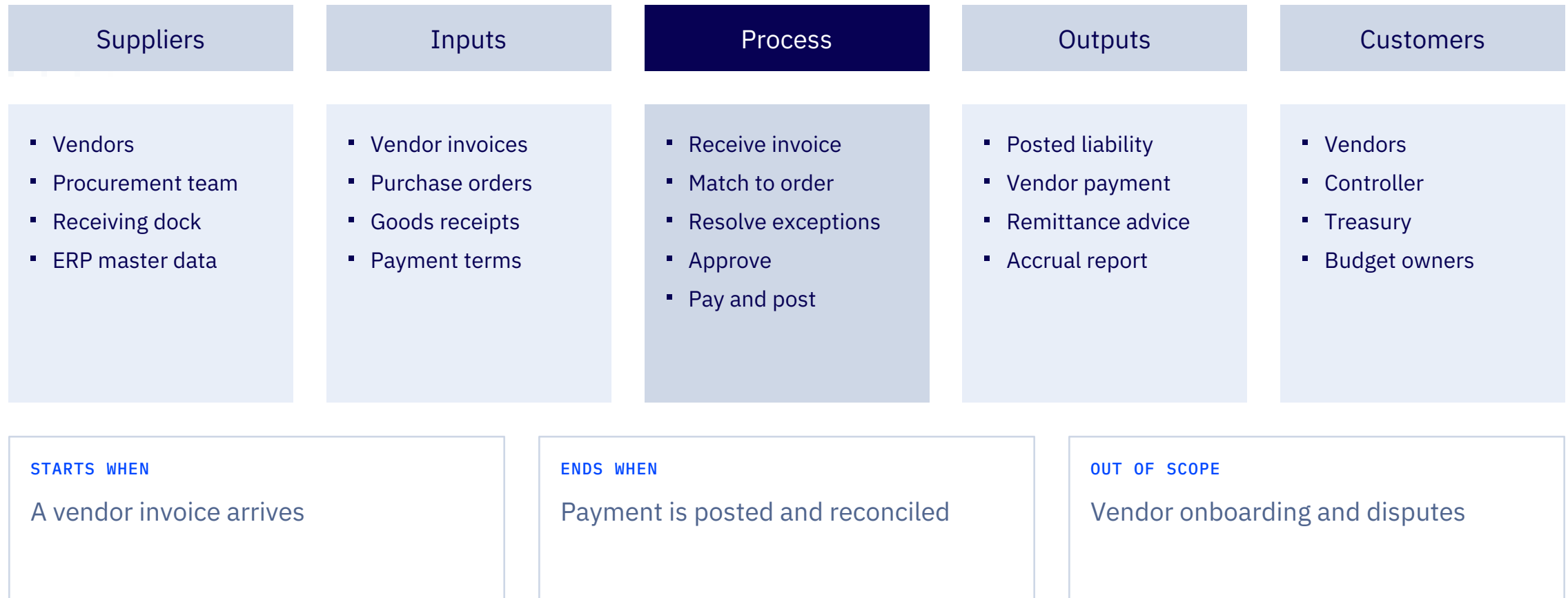
Wave one covers two of five end-to-end finance processes



Dark chevrons are in scope for wave one. Enablers: shared services, finance IT and master data.

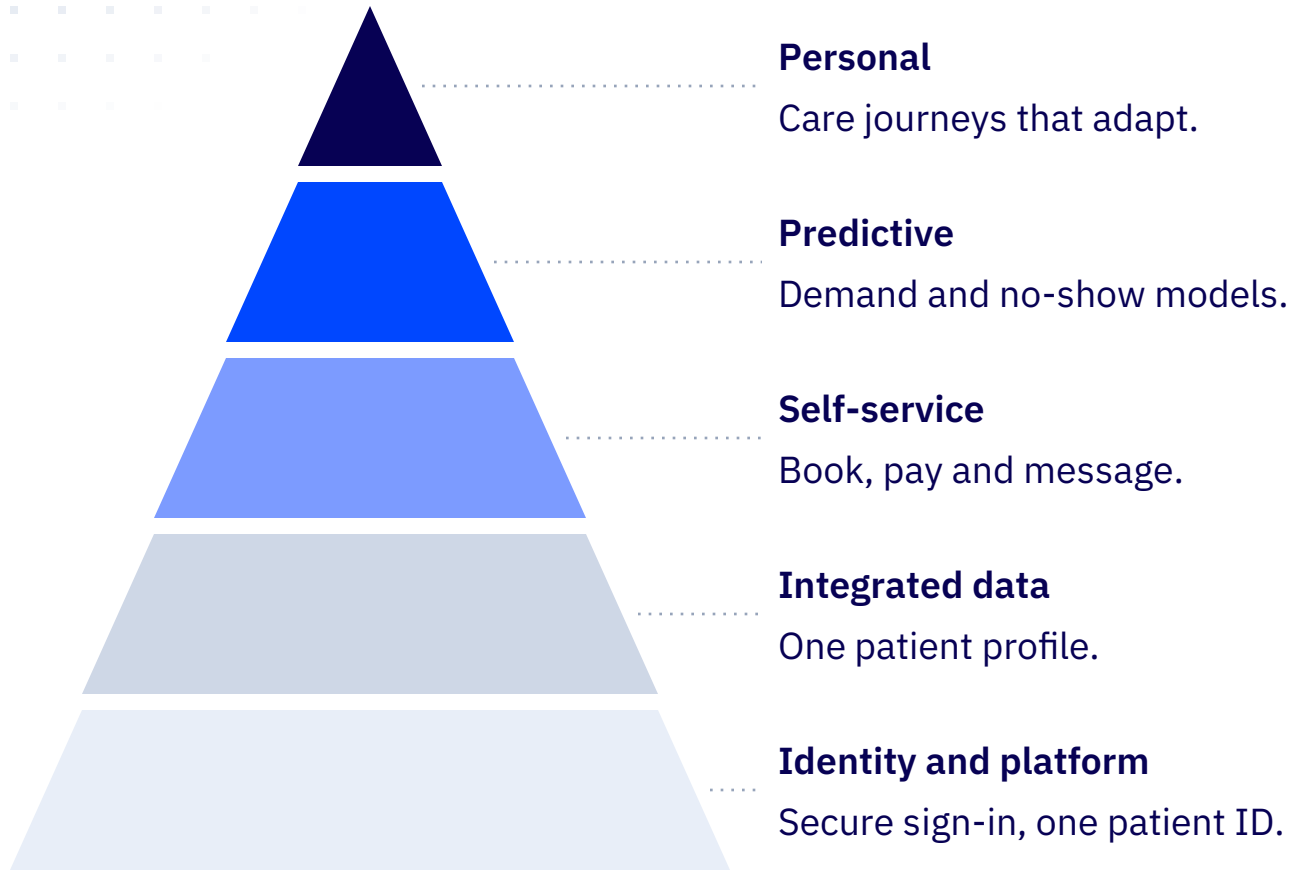
Source: Northfield Health System process inventory, Sep 2026.

A SIPOC fixes the scope of invoice to pay before we map it



Source: Northfield Health System scoping workshop, Sep 2026.

Each layer of the digital front door rests on the one below



BUILD ORDER

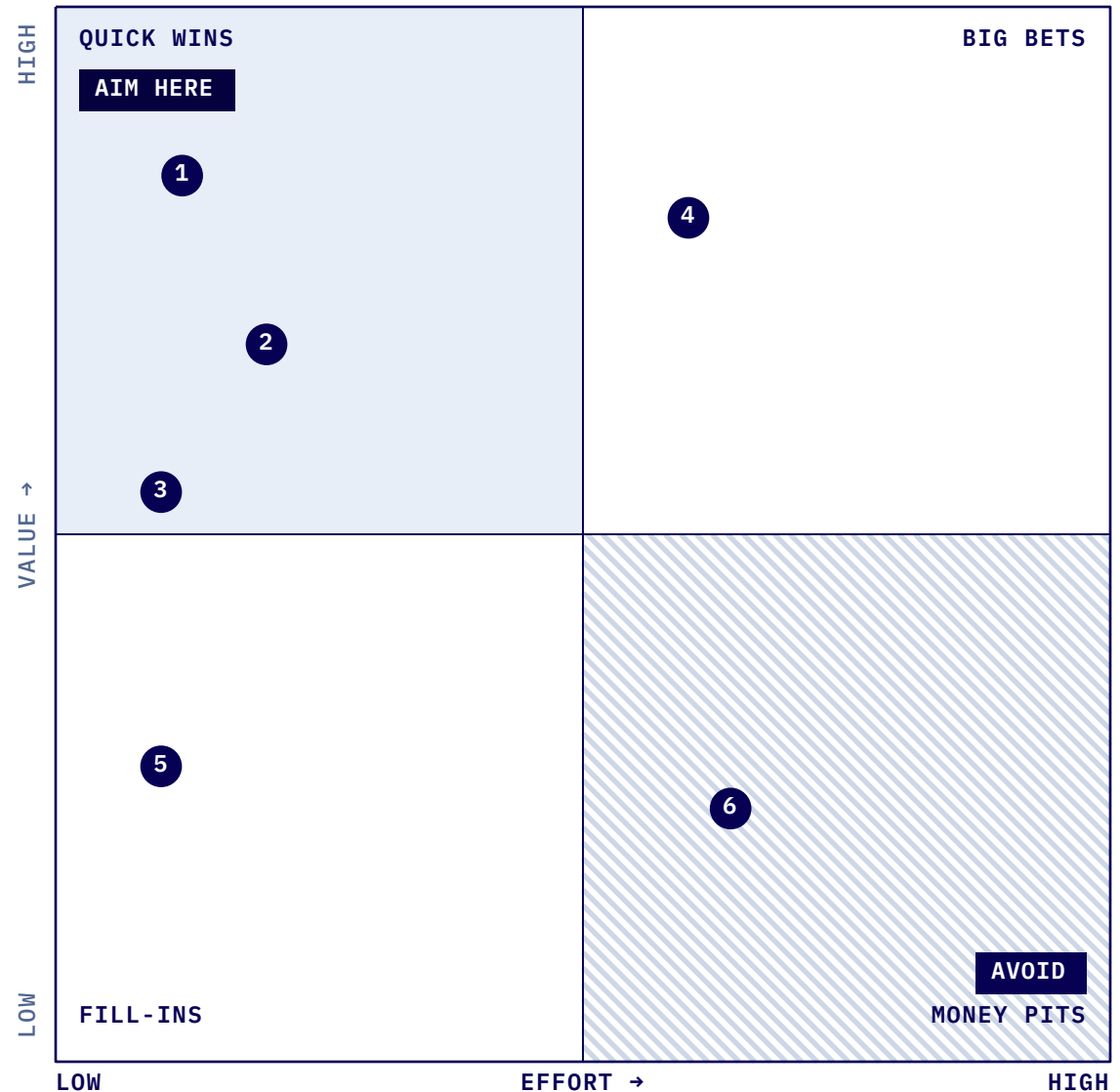
- 01 Lay the base first**
Identity and a secure platform come before any new service.
- 02 Connect the data**
Scheduling, billing and clinical records feed one profile.
- 03 Then add intelligence**
Prediction needs clean, connected data.

Source: Northfield Health System digital front door program, Sep 2026.

PRIORITIZATION

Six initiatives, ranked by value and effort

- 01 Reconciliations**
Automated matching, wave 1.
- 02 Rules-based review**
Replaces manual checks.
- 03 Close calendar**
One shared timetable.
- 04 Consolidation**
Highest value, most effort.
- 05 Account rationalization**
Fewer accounts to maintain.
- 06 Custom ERP reports**
Cost outruns the return.



Source: Northfield finance workshops, Sep 2026.

Four sources of resistance, each with a planned response

01 Fear of job loss

WHAT WE HEAR

Rumors that automated scheduling cuts roles

RESPONSE

Publish role cards and a no-layoff statement

OWNER

HR director

02 Workload

WHAT WE HEAR

No time to train during busy clinic days

RESPONSE

Protect two hours per person; backfill coverage

OWNER

Practice managers

03 Tool distrust

WHAT WE HEAR

Past rollouts failed and were rolled back

RESPONSE

Sandbox, peer demos and a visible fix list

OWNER

Change lead

04 Loss of control

WHAT WE HEAR

Physicians lose their personal template rules

RESPONSE

Template sign-off by each clinic lead

OWNER

Medical director

Monitor: a two-question pulse survey every two weeks.

Source: Northfield Health System readiness interviews, Sep 2026.

The platform is generally available when Northfield can run it as one owned service

Ready for GA when Northfield can confidently

say yes to the next qualified workload

and fulfill the request through one standard enterprise service.

Architecture approved

- Role in the data estate is clear
- Placement criteria documented
- Approved patterns established

Automated and repeatable

- Environments provisioned consistently
- Policies embedded in automation
- Manual activities kept to a minimum

Adoptable and consumable

- Clear intake and onboarding
- Documentation and training ready
- Teams know when to use it

Secure and governed

- Identity and workspace segmentation
- Lineage and catalog integration
- Controls tested, remediation defined

Observable and supportable

- Health, usage and cost visible
- Alerting and runbooks in place
- Ownership and support established

Owned and sustainable

- Roles and decision rights clear
- Northfield teams can operate it
- Chargeback mechanisms in place

GA means Northfield runs it as one owned service

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One risk is both certain and critical

LIKELIHOOD	IMPACT (1 MINOR TO 5 CRITICAL)				
	1	2	3	4	5
5 Almost certain			R9		R1
4 Likely		R7		R3	R10
3 Possible		R6	R2	R4	
2 Unlikely			R8		R5
1 Rare					

Low 1–3
Watch 4–9
Medium 10–15
High 16–21
Extreme 22–25

R1, SCORE 25

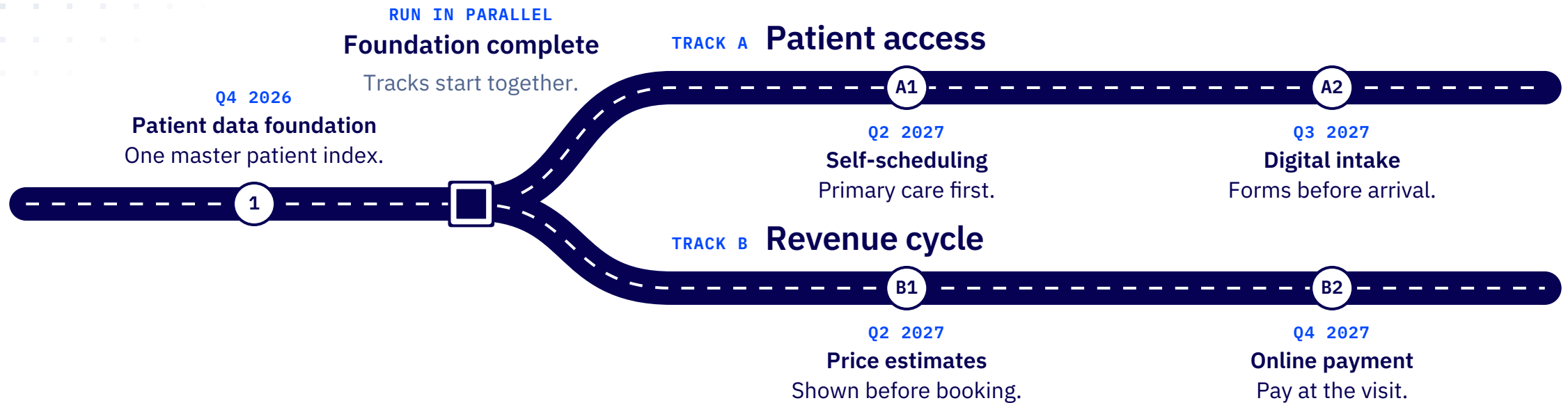
Interface failure at cutover

Owner: CIO. Mitigation: dual-run the lab and pharmacy interfaces for two weeks.

- R10 and R3 follow at scores of 20 and 16.
- Both share the same migration team and cutover window.
- After the dual run, R1 drops to a score of 15.

Source: Northfield program risk register, Sep 2026.

Each track has its own outcome, owner and dependency



TRACK A • PATIENT ACCESS

- **Outcomes.** 70% of primary care visits booked online.
- **Owner.** Chief patient access officer, with the access squad.
- **Depends on.** Master patient index; single sign-in.

TRACK B • REVENUE CYCLE

- **Outcomes.** Estimates on 90% of scheduled visits.
- **Owner.** VP revenue cycle, with the billing squad.
- **Depends on.** Payer contract data loaded.

Source: Northfield Health System digital front door program.

ROADMAP

Three tracks share one foundation

The foundation must land first. After that, access, revenue and clinical teams ship independently.

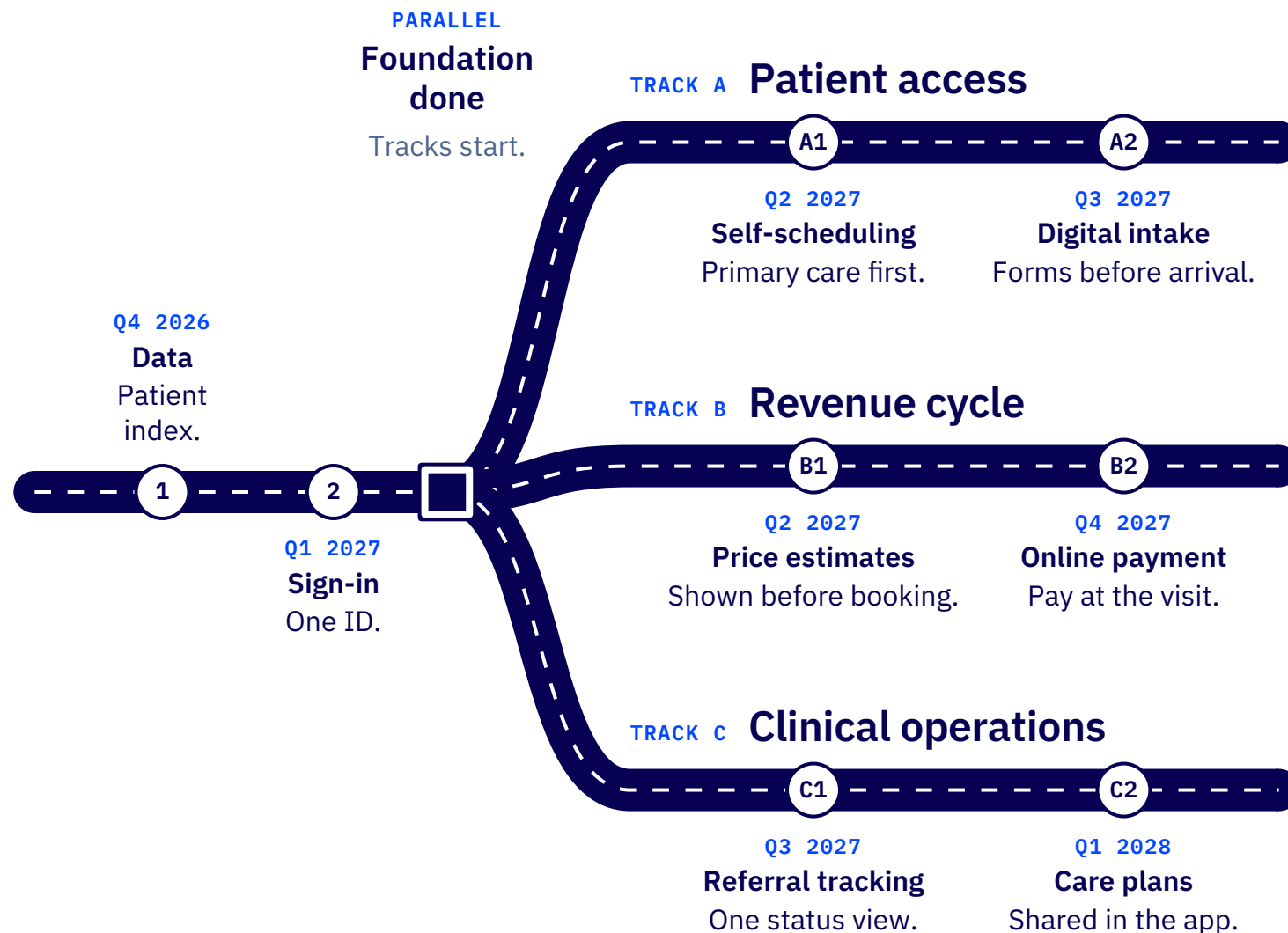
TRACK OWNERS

- **Access.** Chief patient access officer.
- **Revenue.** VP revenue cycle.
- **Clinical.** Chief nursing informatics officer.

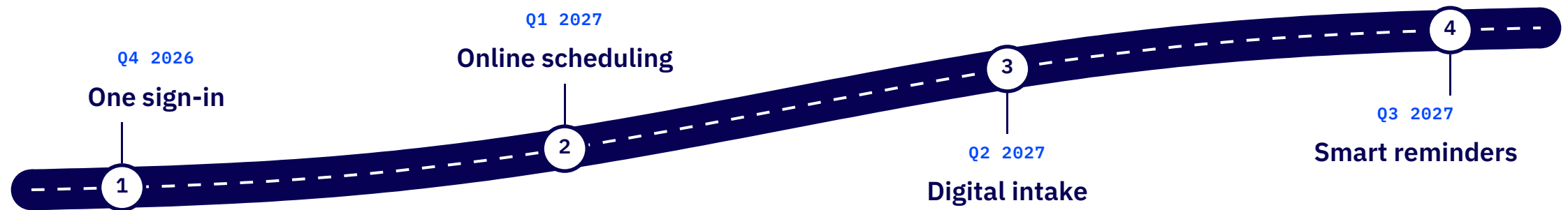
BIGGEST DEPENDENCY

Single sign-in gates all three tracks.

Source: Northfield Health System digital front door program.



The first four milestones land inside 12 months



01 Sign-in

Replaces three logins; 60% migrate in 90 days.

02 Self-booking

Primary care first, then specialty clinics.

03 Digital intake

Forms and coverage checks before arrival.

04 Reminders

Two-way text cuts no-shows by a quarter.

Source: Sample figures for layout only; replace with current firm data before use.

DECISION	BY	OWNER	WHAT IT UNLOCKS
Fund the data foundation	NOW	Executive team	Wave 1 starts
Approve patient ID policy	NOW	Privacy officer	Single sign-in at every site
Choose buy, build or partner	NEXT	Chief digital officer	Vendor selection and contracts
Set price estimate rules	NEXT	VP revenue cycle	Estimates shown before booking
Agree clinical workflow changes	LATER	Clinical council	Referral tracking and care plans
Decide on enterprise scale-out	LATER	Executive team	Funding for 2028

DIGITAL FRONT DOOR ROADMAP

Six decisions set the pace of the roadmap

A roadmap is a series of choices, each made by a named owner before work can start.

HOW DECISIONS ARE MADE

- **Forum.** Access council, monthly.
- **Rule.** No wave starts without its decision logged.
- **Escalation.** Late decisions move to the executive team within two weeks.

Source: Northfield Health System digital front door program.

Three horizons build one front door

HORIZON	OBJECTIVE	KEY INITIATIVES	OUTCOMES AND KPIS	DEPENDENCIES	INVESTMENT
Now Q4 2026 to Q1 2027	Lay the data foundation so every system agrees who the patient is.	- Master patient index - Single sign-in, patients and staff - Scheduling event stream	- Duplicates down 40% - One ID at 100% of sites	- EHR release 4.2 - Privacy sign-off	\$4.2M
Next Q2 to Q3 2027	Open the front door: patients book, register and see prices online.	- Self-scheduling in primary care - Digital intake and consent - Price estimates before booking	60% of visits booked online - Call volume down 25%	- Patient index live - Payer contract data loaded	\$6.8M
Later Q4 2027 to 2028	Extend access across the care journey and personalize it.	- Referral tracking end to end - Care plans in the app - Virtual front desk and chat	- No-show rate under 6% - App use by 50% of patients	- Event stream - Clinical capacity	\$5.5M

Source: Sample figures for layout only; replace with current firm data before use.

Why the roadmap runs in this order

Patients feel the front door, but it only works once the data underneath it is trusted.

- **Identity first.** Every later feature needs one patient ID.
- **Access before payment.** Booking creates the demand that estimates and billing serve.
- **Care last.** Clinical workflows change slowest, so we start them early but ship them last.

Source: Northfield Health System digital front door program.

NOW • Q4 2026 TO Q1 2027

WHAT

One master patient index and one sign-in across all six hospitals.

WHY NOW

Two registration flows create duplicates and denials today.

UNLOCKS

Self-service for every digital feature that follows.

NEXT • Q2 TO Q3 2027

WHAT

Online scheduling, digital intake and price estimates.

WHY NOW

Patients ask for it, and competitors already offer it.

UNLOCKS

Fewer calls, fewer no-shows and cleaner claims.

LATER • Q4 2027 TO 2028

WHAT

Referral tracking, care plans and a virtual front desk.

WHY NOW

Not yet: it needs the event stream and clinical capacity.

UNLOCKS

Care that continues between visits.

Each wave starts and ends on evidence, not on the calendar

WAVE	SCOPE	ENTRY CRITERIA	EXIT CRITERIA	DECISION GATE
Wave 1 Foundation	- Master patient index - Single sign-in	- Funding approved - Privacy review signed	- Duplicates down 40% - Sign-in live at all sites	Access council confirms the data is trusted
Wave 2 Access	- Self-scheduling - Digital intake	- Wave 1 exit met - Two pilot clinics named	60% of visits booked online - Intake done for 50%	Executive team chooses buy or build
Wave 3 Revenue	- Price estimates - Online payment	- Payer data loaded - Billing squad staffed	- Estimates on 90% of visits - Denials down 15%	CFO signs off on rollout to all sites
Wave 4 Clinical	- Referral tracking - Care plans	- Event stream at scale - Clinical lead named	- Every referral tracked - Care plans in 3 service lines	Clinical council approves workflow changes

Source: Northfield Health System digital front door program.

Each capability group moves through three horizons

	Access	Care	Billing	Platform
Now NEXT 90 DAYS	- Self-scheduling - Waitlist - Provider search	- Secure messaging triage - Lab alerts - Visit summaries	- Pay online - Payment plans - Statement redesign	- EHR hardening - Release pipeline - Crash reporting
Next TWO QUARTERS	- Pre-visit forms - Virtual visit v2 - Reminders v2	- Care plans - Remote monitoring - Risk flags	- Cost estimates - Card capture - Refund tracking	- Event stream - Observability - Test automation
Later BEYOND SIX MONTHS	- Multilingual booking - Kiosk check-in - Group visits	- Medication adherence - Proxy access - Wearable sync	- Price transparency API - Estimate accuracy - Coverage checks	- Data store re-platform - Zero-downtime release - Cost dashboards

Source: Northfield product council roadmap, Sep 2026.

Three horizons take the patient app from booking to care

- **Read across** to see what ships and when.
- **Read down** to see how the product matures.
- **Move items** only through the product council.

Horizons are planning commitments, not dates.

Now

NEXT 90 DAYS

- Self-scheduling
- Waitlist
- Messaging triage
- Pay online
- Payment plans
- EHR hardening

Outcome: bookings complete without a call

Next

TWO QUARTERS

- Care plans
- Remote monitoring
- Cost estimates
- Pre-visit forms
- Event stream
- Observability

Outcome: care continues between visits

Later

SIX MONTHS AND BEYOND

- Proxy access
- Multilingual booking
- Kiosk check-in
- Medication adherence
- No-show outreach
- Data store re-platform

Outcome: access reaches every community

Three themes organise six bets across the horizons

THEME 1

Frictionless access

Patients book, pay and message without a call.

NOW • KEY BET

Self-scheduling

Primary care first, then specialties.

NEXT

Cost estimates

Prices shown before the visit is booked.

THEME 2

Care between visits

The app carries the care plan home.

NOW

Messaging triage

Nurses route every message in one queue.

NEXT • KEY BET

Remote monitoring

Pilot with chronic-care cohorts.

THEME 3

A platform that scales

One data and release foundation for every product.

NOW

EHR hardening

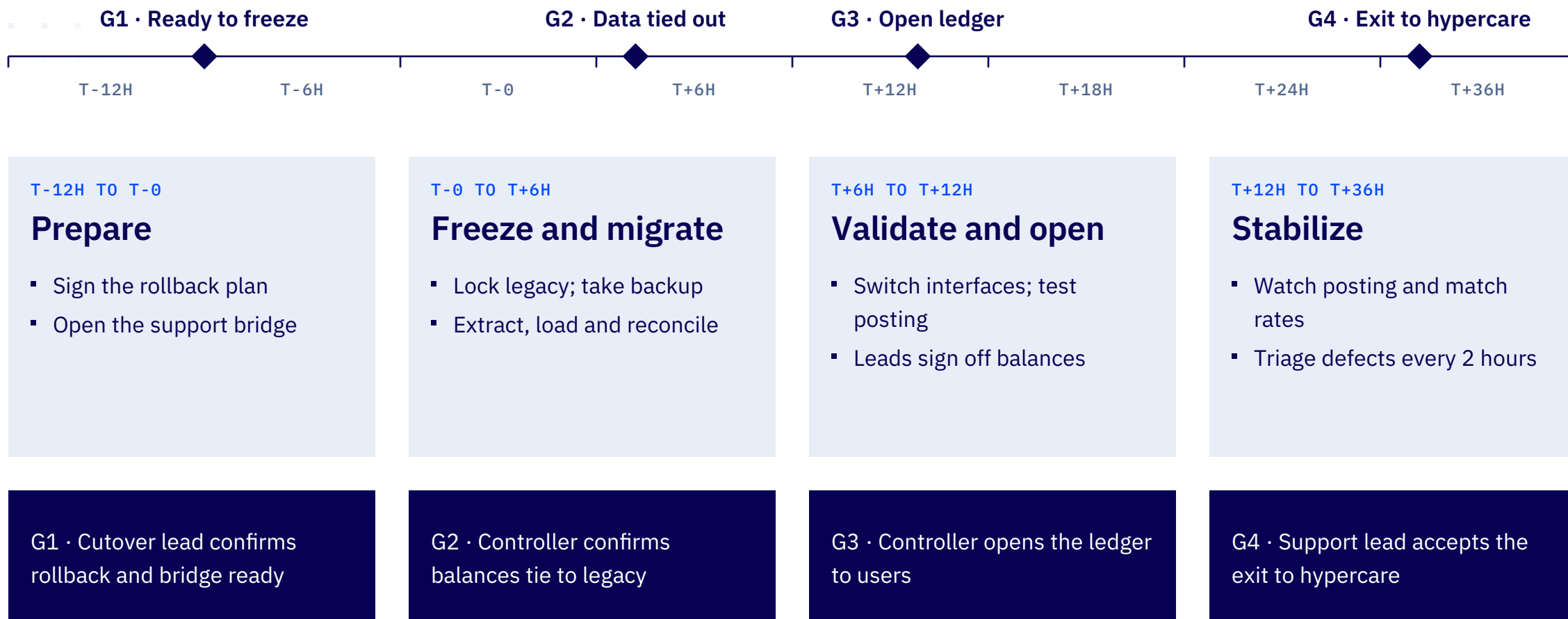
Two integrations stable at volume.

LATER

Data store re-platform

Move to the shared patient data store.

Four go/no-go gates pace the weekend cutover



Four levels put every issue in front of a decision-maker

Each level owns one class of decision. If a level has not acknowledged within its threshold, page the next level.

The clock runs from the first alert, not from the escalation.

L4 • STEERING COMMITTEE • 60+ MIN

- Trigger: rollback invoked or go-live date at risk
- Decides: reset the date; approve a new plan



L3 • CONTROLLER AND CIO • 30–60 MIN

- Trigger: balances do not tie, or a gate is at risk
- Decides: go or no-go; invoke full rollback



L2 • CUTOVER LEAD • 15–30 MIN

- Trigger: L1 cannot clear it, or the window slips
- Decides: re-sequence steps; extend a window



L1 • WORKSTREAM LEAD • 0–15 MIN

- Trigger: a step runs late or a check fails
- Decides: retry the step or apply its rollback

One runbook governs every wave 1 cutover decision

PURPOSE

Move close and consolidation to the new ledger over one weekend, with a tested way back at every step.

SCOPE

- General ledger and subledgers
- Bank and intercompany interfaces
- Excludes planning and reporting

ROLE	HELD BY	ACCOUNTABLE FOR
Cutover lead	Program director	Runs the bridge; calls go or no-go
Ledger owner	Controller	Signs balances; opens the ledger
Technical lead	Finance IT	Runs loads and interface switch
Support lead	Shared services	User comms and hypercare

ENTRY CRITERIA

- Dress rehearsal signed off
- Rollback plan approved

EXIT CRITERIA

- Day 1 postings reconciled
- Hypercare handover accepted

ESCALATION PATH



CUTOVER RUNBOOK

One runbook governs every wave 1 cutover decision

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ESCALATION PATH



Step 1 freezes every legacy posting before any data moves

STEP 1

Freeze legacy postings

Stop every posting to the legacy ledger so the extract is one stable cut of the books.

OWNERS

- Controller · accountable
- Finance IT · executes
- Internal audit · observes

WINDOW

Fri 18:00 · 30 min

SUB-STEPS

1. Announce the freeze on the bridge
2. Close and post open batches
3. Lock subledgers and the calendar
4. Run the subledger lock report
5. Publish the freeze notice

RULES

- No manual journals after freeze
- Exceptions need the Cutover lead
- Late items are queued, not posted
- Record the freeze time on the log

GATE CLEARANCE

- Lock report shows no open batches
- Controller confirms the freeze on the log
- Cutover lead releases step 2

Tick every reconciliation before step 4 loads balances

STEP 4

Load balances to the new ledger

Load the extracted open balances by entity and prove they match the legacy books.

OWNERS

- Finance IT · executes
- Data lead · reconciles
- Controller · accountable

WINDOW

Fri 22:30 · 2 h

DONE	RECONCILIATION CHECK	LEGACY	TOLERANCE	NEW LEDGER
☐	Row count by entity	412,806	Zero	
☐	Debit and credit totals	\$1.284bn	\$0	
☐	Open items by entity	9,418	±0.1%	
☐	Intercompany balances	\$38.2m	±0.5%	

SUB-STEPS

- Load by entity, parents last
- Run the count and total checks
- Park rejected rows
- Log each variance

ACCEPTANCE

- Every check inside tolerance
- Variances explained and signed
- Data lead releases step 5

Tick every criterion before step 7 opens the ledger

STEP 7

Hold the go or no-go decision

Review the evidence for every criterion and decide whether the ledger opens to users.

OWNERS

- Cutover lead · decides
- Controller · signs balances
- CIO · endorses

WINDOW

Sat 03:30 · 30 min

GO	GO CRITERION	EVIDENCE	APPROVER
☐	Balances tie to legacy	Reconciliation report	Controller
☐	Interfaces pass end to end	Interface test pack	Integration lead
☐	Rollback fits the window	Timed rollback record	Technical lead
☐	Support bridge staffed	Signed rota	Support lead

IF GO

- Controller opens the ledger
- Support lead sends the notice
- Log the decision and time

IF NO-GO

- Cutover lead invokes rollback
- Escalate to level 3
- Reset the date at steering

Steps 1 to 4 lock the legacy ledger and move its balances

1 Freeze legacy postings

- Close open batches
- Lock subledgers
- Publish the freeze notice

Owner: Controller
Fri 18:00 · 30m

2 Take the final backup

- Snapshot the legacy ledger
- Verify checksums

Owner: Finance IT
Fri 18:30 · 1h

3 Extract open balances

- Run the extract jobs
- Tie to the trial balance
- Log variances

Owner: Data lead
Fri 19:30 · 3h

4 Load the new ledger

- Load by entity
- Check counts and totals
- Park rejected rows

Owner: Finance IT
Fri 22:30 · 2h

Step 4 · page 1 of 2 · Continued on next page

RUNBOOK PURPOSE

Lock the legacy ledger and move its balances to the new platform in one window.

RUNBOOK OWNER

Cutover lead

Window: Fri 18:00 to Sat 02:00

Step 4 · page 1 of 2 · continued on next page



CUTOVER RUNBOOK

Steps 1 to 4 lock the legacy ledger and move its balances

1 Freeze legacy postings

Controller · Fri 18:00 · 30m

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3 Extract open balances

Data lead · Fri 19:30 · 3h

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Finance IT · Fri 22:30 · 2h

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Steps 1 to 4 lock the legacy ledger and move its balances

Each step has an owner and a window. Work to the actions in order and log each one as it completes.

Step 4 · page 1 of 2 · Continued on next page

1 Freeze legacy postings

Owner: Controller
Fri 18:00 · 30m

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Fri 22:30 · 2h

- Load by entity
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Sixteen defects are open and none is critical

SEVERITY	NEW	TRIAGE	FIX	RETEST	CLOSED	FIX BY
Sev 1 · Critical	0	0	0	0	1	4 h
Sev 2 · High	0	1	1	1	6	1 day
Sev 3 · Medium	2	3	2	1	12	3 days
Sev 4 · Low	1	2	1	1	9	Release 2

TRIAGE RULES

- Triage twice a day at 09:00 and 16:00.
- Sev 1 and 2 go straight to the fix team.
- Sev 3 needs a workaround before go-live.
- Sev 4 moves to the post-go-live backlog.

Exit rule: no Sev 1 or 2 open, and every Sev 3 has an approved workaround.

16

open defects

1.8

days mean time to fix

3

awaiting retest

The test lead triages with the process owner. The Controller signs the exit.

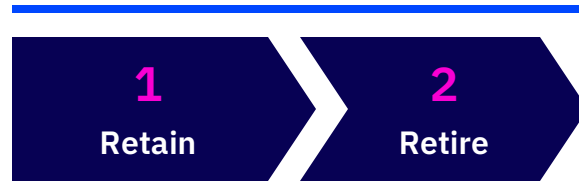
Three evidence streams become one validated model



¹ Observation requires consent and appropriate privacy, security and retention controls.

Seven options, one common decision language

KEEP OR RETIRE



Keep and manage deliberately

Remove obsolete software or work

MOVE AND IMPROVE



Move infrastructure; limit change

Change the runtime; keep behavior

Improve structure; keep function

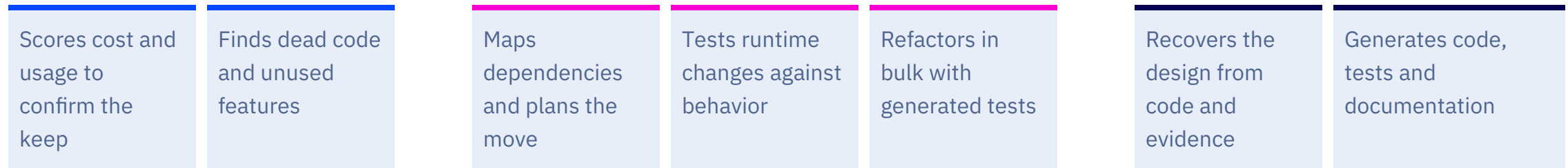
REBUILD THE DESIGN



Change structure and boundaries

Redesign and rewrite the capability

Where AI accelerates each option



The framework still guides the decision; AI broadens the feasible set, and leaders remain accountable for the choice.

Expect a facilitated build, not a passive demo

HOW THE SESSION RUNS

-  **Frame together** 15 MIN
Facilitators set the objective and decision criteria; teams bring their own context and constraints.
-  **Work in role** 30 MIN
Product and engineering build in focused tracks; facilitators keep time.
-  **Reconnect at gates** 30 MIN
Teams share artifacts, challenge ambiguity and agree what is ready before the next block.
-  **Iterate and close** 15 MIN
The group reviews evidence, records limits and turns learning into owned next actions.

HOW FACILITATION WORKS

Guide the process without taking over the work

- Short prompts, then participant work
- Named checkpoints and timeboxes
- Active coaching and unblocking
- Human approval at every gate
- Shared notes captured live
- Open risks logged for follow-up

What we need from participants: real context, active decisions, work in progress shared and candor about constraints.

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-  **Iterate and close** 15 MIN
Review evidence and turn learning into owned actions.

Bring real context and share work in progress.

WHAT PARTICIPANTS WILL LEAVE WITH

Six outcomes define the session

Frame the right opportunity
Pick a feasible problem

Turn evidence into intent
Stories and criteria

Agents across the SDLC
Plan, code, test, release

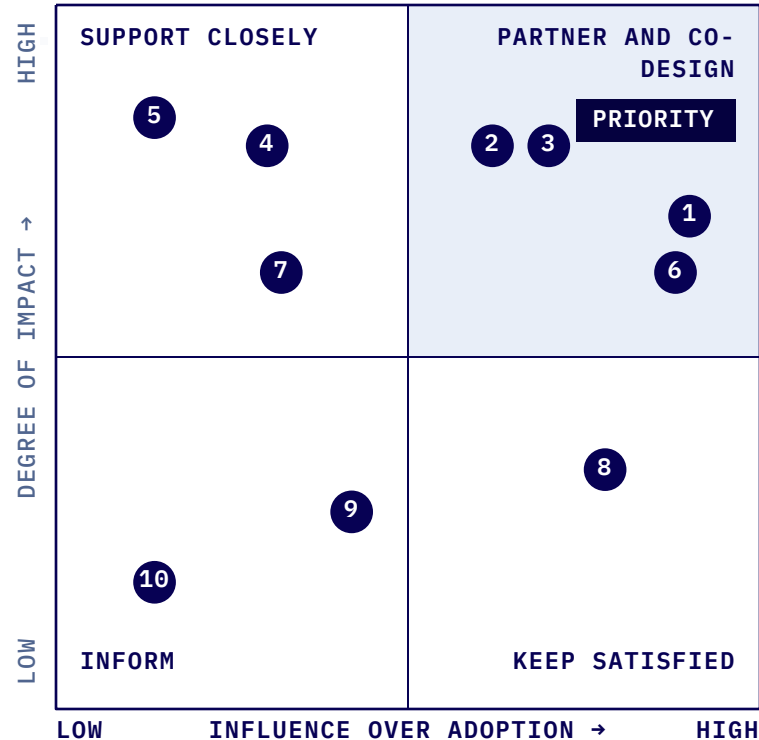
Product-to-R&D interface
Agree a demonstrable slice

Compress feedback loops
Prototype in parallel

Scale AI-enabled delivery
Adoption and governance

Seven stakeholders need active engagement before go-live

Influence over adoption and degree of impact



01

Partner

Chief medical officer, physician group and practice managers co-design the rollout.

02

Support

Schedulers and agents get coaching and a named champion.

03

Inform

Union and payer contacts get briefings ahead of each wave.

Source: Northfield Health System stakeholder interviews, Sep 2026.

One schedule slip puts the program at risk

OVERALL STATUS

At risk

STATUS BY AREA

AREA	STATUS	THIS PERIOD
Scope	On track	Two change requests approved
Schedule	At risk	Wave 2 cutover slips six days
Budget	On track	\$0.2M under plan to date
Risks	At risk	Two high risks, both owned

COMPLETED THIS PERIOD

- Wave 1 entities live
- Journal rules signed off
- Close calendar published
- Training cohort one done

PLANNED NEXT PERIOD

- Wave 2 data migration
- User acceptance testing
- Cutover go or no-go
- Cockpit design review

RISK REGISTER

RISK	LIKELIHOOD	OWNER	MITIGATION	STATUS
Late data extracts	High	Finance IT	Daily extract check	At risk
Reviewer capacity	Medium	Controller	Add two reviewers	On track
Cutover freeze clash	Medium	PMO lead	Reset freeze date	At risk

DECISIONS NEEDED

- Approve new cutover date CFO, by 9 Oct
- Fund two extra reviewers Controller, by 16 Oct
- Confirm freeze window CIO, by 16 Oct
- Release Wave 3 budget CFO, by 30 Oct

Source: Northfield program office status report, 30 Sep 2026.

ADR-014 replaces the 2024 choice of point-to-point feeds

SUPERSEDED • ADR-009 • MAR 2024

Point-to-point HL7 feeds

- Each system connects directly to the others
- Chosen when Northfield ran 6 systems
- Retired in 3 waves from Q1 2027



CURRENT • ADR-014 • SEP 2026

Shared event bus

- Systems publish and subscribe to events
- Fits 14 systems and growing
- Owned by the platform team

WHY IT CHANGED

The estate outgrew it

Systems grew from 6 to 14 and feeds from 9 to 61.

Incidents followed feeds

Most clinical interface incidents traced to one broken feed.

Cost per feed rose

Each new feed took a quarter and a test cycle across teams.

Clinicians rate the new system lowest

- **Clinicians struggle most.** Physicians and nurses rate time savings, workflow fit and alerts lowest.
- **Leaders are upbeat.** Their scores run well above those of the people doing the work.
- **Act on the gap.** Fix alert noise and order-set layout first; both are configuration, not build.

STATEMENT	PHYSICIANS	NURSES	FRONT DESK	LEADERS
I understand why we change	4.1	3.9	3.6	4.6
I have the training I need	3.2	2.8	3.0	3.9
The system saves me time	2.4	2.6	2.9	3.8
Leaders support the change	3.5	3.7	3.4	4.7
I know who to ask for help	3.3	3.8	3.1	4.2
Workflows fit how I work	2.1	2.5	3.2	3.5
Data in the system is trusted	3.1	3.2	3.0	4.0
I would recommend it to a peer	2.9	3.1	3.3	4.4
Alerts help more than they interrupt	2.2	2.4	3.4	3.6
Reports meet my needs	3.0	3.3	2.8	4.1

4.5–5.0
3.5–4.4
2.5–3.4
1.5–2.4
Under 1.5

Source: Northfield go-live pulse survey, n = 412, Sep 2026.

Agents cut our team and Northfield's maintenance

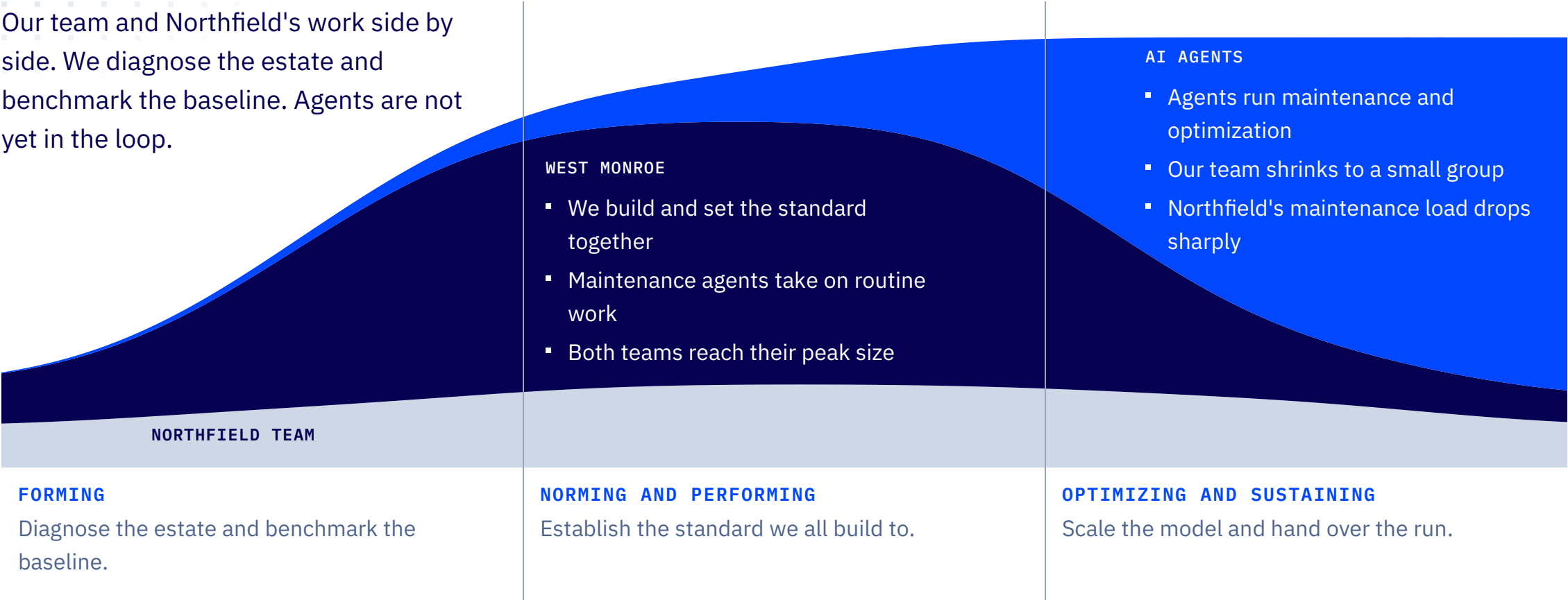
Our team and Northfield's work side by side. We diagnose the estate and benchmark the baseline. Agents are not yet in the loop.



OUR APPROACH

Agents cut our team and Northfield's maintenance

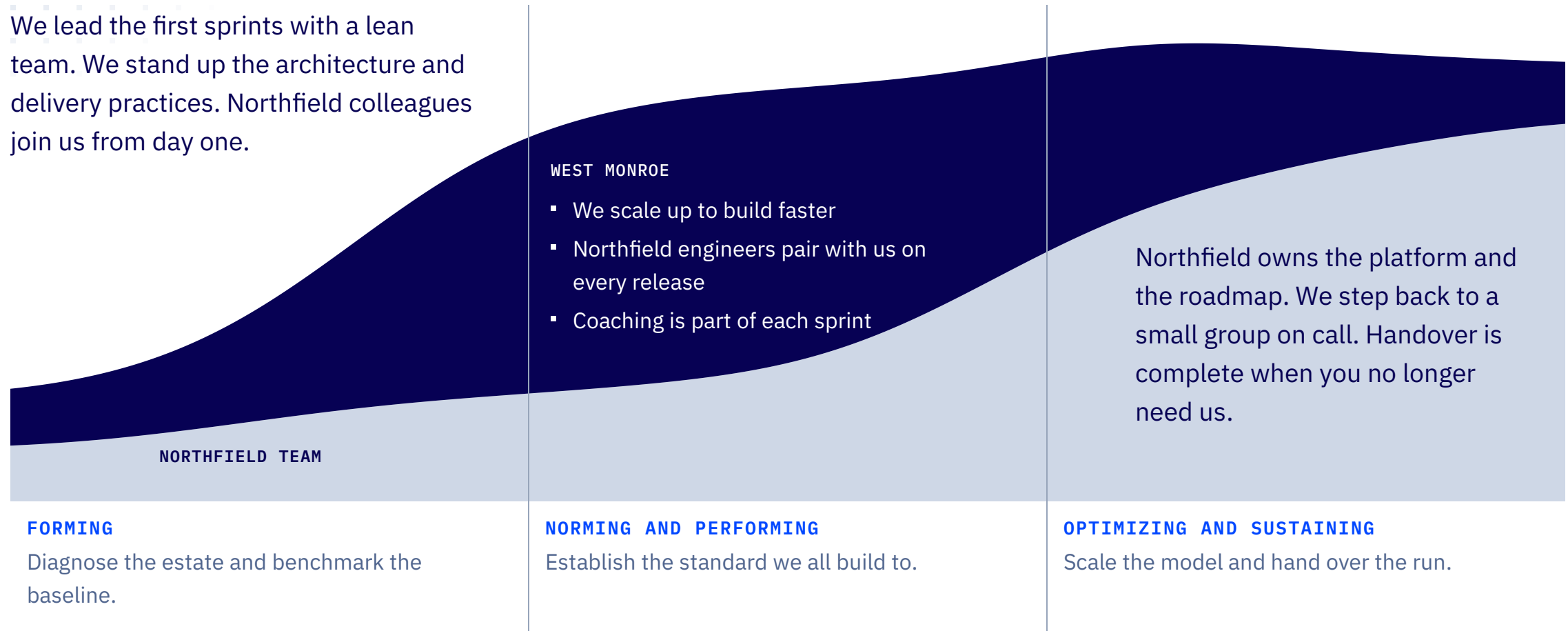
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OUR APPROACH

We build together, then Northfield takes the lead

We lead the first sprints with a lean team. We stand up the architecture and delivery practices. Northfield colleagues join us from day one.



A core team backed by matched specialists

Leadership and core team



Dana Whitfield
Executive sponsor

KM

Kiran Mehta
Executive sponsor

JA

Julia Ansel
Engagement lead

WHAT THE CORE TEAM DOES

- Own the sponsor relationship and quality
- Bring healthcare expertise to the work
- Align both teams and transfer knowledge

MO

Mike Odell
Platform architect

JS

Jag Soni
Enterprise architect

AR

Albert Reyes
Infrastructure lead

Subject matter experts and support specialists

SA

Scott Alder
SME, payer data

DC

Dan Calloway
SME, life sciences

BM

Brett Marlow
SME, provider

WHAT THE SPECIALISTS DO

- Advise on architecture and governance
- Assess processes and organization
- Lead training and knowledge transfer

AG

Anneke Groot
SME, governance

TZ

Tanner Zell
SME, design

GM

Greta Movsky
SME, cybersecurity

A core team backed by matched specialists

Leadership and core team



Dana Whitfield
Executive sponsor

KM

Kiran Mehta
Executive sponsor

JA

Julia Ansel
Engagement lead

MO

Mike Odell
Platform architect

JS

Jag Soni
Enterprise architect

AR

Albert Reyes
Infrastructure lead

WHAT THE CORE TEAM DOES

- Own the sponsor relationship and quality
- Bring healthcare expertise to the work
- Align both teams and transfer knowledge

Subject matter experts and support specialists

SA

Scott Alder
SME, payer data

DC

Dan Calloway
SME, life sciences

BM

Brett Marlow
SME, provider

AG

Anneke Groot
SME, governance

TZ

Tanner Zell
SME, design

GM

Greta Movsky
SME, cybersecurity

WHAT THE SPECIALISTS DO

- Advise on architecture and governance
- Assess processes and organization
- Lead training and knowledge transfer

Core team, matched specialists

Leadership and core team



Dana Whitfield
Executive sponsor

JA

Julia Ansel
Engagement lead

JS

Jag Soni
Enterprise architect

KM

Kiran Mehta
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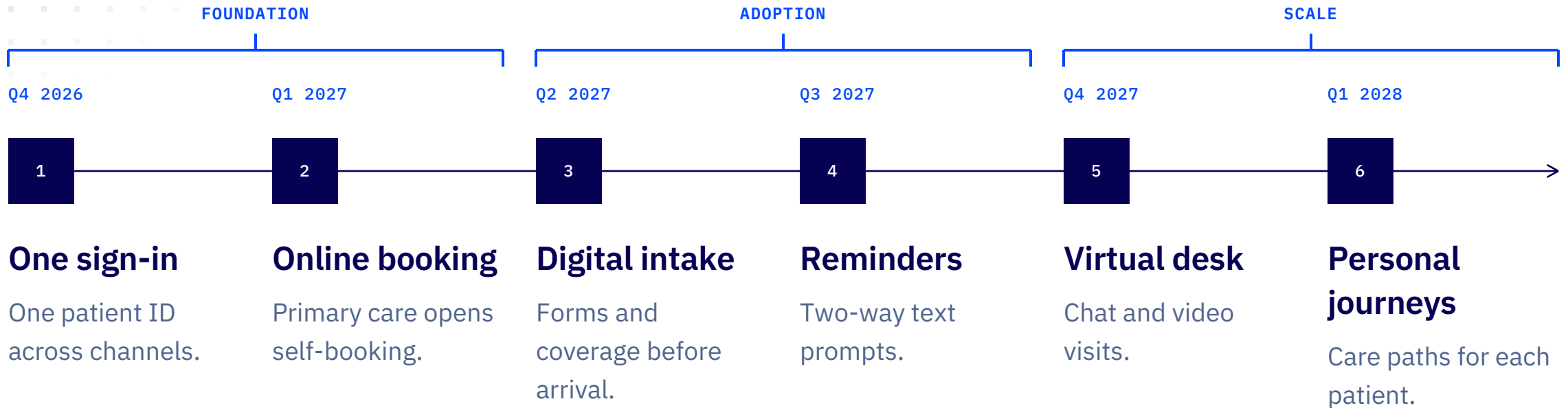
Tanner Zell
SME, design

GM

Greta Movsky
SME, cybersecurity

Advise on architecture and governance, assess processes and lead training.

Six milestones in three phases carry the program to 2028



PHASE 1 • FOUNDATION

Exit: 60% on one sign-in

PHASE 2 • ADOPTION

Exit: 35% book online

PHASE 3 • SCALE

Exit: 20% fewer calls

Source: Sample figures for layout only; replace with current firm data before use.

Our understanding on one page

CONTEXT

- Seven hospitals, 64 clinics, three schedulers
- Three acquisitions in four years
- EHR go-live in Q3 next year
- 11% of denials start at registration
- Days in A/R run above peers

SCOPE

- In: scheduling, registration, estimates
- In: edits, claims, denial work queues
- In: EHR interface design, training
- Out: EHR build, documentation redesign
- To confirm: wave one, data access

OBJECTIVES

- One access experience at every site
- Fewer denials, faster cash
- Cleaner registration data
- Lower cost to collect
- A sustained benefits model

CONSTRAINTS

- EHR cutover date is fixed
- Phase one capped at FY27 capital
- Estimates and consent rules apply
- Two legacy schedulers retire
- Access and billing decide separately

STAKEHOLDERS

- J. Alvarez, VP Revenue Cycle
- R. Chen, Patient Access
- EHR program lead
- Compliance and privacy
- Finance, capital planning

SUCCESS MEASURES

- Days in A/R: 52 to 44
- Denial rate: 9.4% to 6.0%
- Access score: 68 to 80
- Cost to collect: 3.4% to 2.8%
- Adoption: 90% at six months

Source: Sample names and figures for layout only; replace with engagement stakeholders and data before use.

Four constraints decide how we answer the ask

TIMELINE

EHR go-live in Q3

Access workflows must be ready for cutover, which leaves about nine months to design and train.

BUDGET

Funded in two phases

Phase one is capped at the FY27 capital line; phase two depends on measured results.

THE ASK

One access and revenue cycle design

Join scheduling, registration, estimates, claims and denials into one design that Northfield can run, measure and extend.

- Seven hospitals
- 64 clinics
- Three scheduling tools

REGULATORY

No Surprises Act, HIPAA

Good faith estimates and consent must work in the new flow from the first day.

SYSTEMS

Three tools, one EHR

Two legacy schedulers retire; interfaces to the clearinghouse and EHR stay stable.

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

Four constraints, and what each means for the answer

The ask: one access and revenue cycle design that Northfield can run, measure and extend across seven hospitals and 64 clinics.

TIMELINE

EHR go-live in Q3

- About nine months to design and train
- Sites go live in waves

IMPLICATION

Sequence waves around cutover.

BUDGET

Funded in two phases

- Phase one capped at the FY27 capital line
- Phase two depends on results

IMPLICATION

Prove value in phase one.

REGULATORY

Estimates and consent

- No Surprises Act and HIPAA apply
- Good faith estimates from day one

IMPLICATION

Build compliance into the flow.

SYSTEMS

Three tools, one EHR

- Two legacy schedulers retire
- Clearinghouse interfaces stay stable

IMPLICATION

Retire tools without breaking claims.

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

THE ASK

One access and revenue cycle design

Join scheduling, registration, estimates, claims and denials into one design that Northfield can run, measure and extend.

- Seven hospitals
- 64 clinics
- Three scheduling tools

It must deliver

- One front door at every site
- Fewer front-end denials
- Faster cash

OUR UNDERSTANDING

Four constraints decide how we answer the ask

TIMELINE

EHR go-live in Q3

About nine months to design and train before cutover.

BUDGET

Funded in two phases

Phase one is capped at the FY27 capital line.

REGULATORY

Estimates and consent

No Surprises Act and HIPAA rules apply from day one.

SYSTEMS

Three tools, one EHR

Two legacy schedulers retire; interfaces stay stable.

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

OUR UNDERSTANDING

We read the scope as the front end through to denials

IN SCOPE

What we will deliver

- Scheduling, registration, estimates
- Front-end edits and claims
- Denial work queues
- EHR interface design
- Training and adoption

OUT OF SCOPE

What stays with others

- EHR configuration and build
- Clinical documentation redesign
- Payer contract negotiation
- Network and hardware refresh

Boundary rule: any change to the EHR build is a change request.

TO CONFIRM

Assumptions

- Wave one covers four hospitals and 20 clinics
- We receive read access to all three schedulers
- One leader owns access and billing decisions
- The Q3 go-live date is fixed
- Phase one funding is approved
- Northfield names one executive sponsor

Source: Sample content for layout only; replace with the client's RFP and engagement facts before use.

Three scheduling tools and one EHR date shape the ask

SITUATION

Where Northfield stands

- Seven hospitals and 64 clinics run three scheduling tools
- Registration quality varies by site
- Three acquisitions in four years

3

scheduling tools

71

sites to align

COMPLICATION

What is getting in the way

- Front-end errors reach the claim
- Patients repeat the same details at each step
- Access and billing answer to different leaders

11%

denials at intake

52

days in A/R

WHAT YOU ASKED FOR

What the RFP requires

- A unified access and revenue cycle design
- A phased roadmap that fits the EHR date
- Named owners and measures for each benefit

7

measures to track

Q3

EHR go-live

Our reading: the complication is organizational as much as technical, so the answer has to join access, billing and the EHR plan.

Source: Sample figures for layout only; replace with the client's RFP and engagement facts before use.

What sits behind the ask

SITUATION

Where Northfield stands

- Seven hospitals and 64 clinics run three scheduling tools
- Registration quality varies by site
- Three acquisitions in four years

COMPLICATION

What is in the way

- Front-end errors reach the claim
- Patients repeat the same details at each step
- Access and billing answer to different leaders

3

scheduling tools

71

sites to align

11%

denials at intake

52

days in A/R

Our reading: the complication is organizational as much as technical.

Source: Sample figures for layout only; replace with the client's RFP and engagement facts before use.

WHAT YOU ASKED FOR

One joined-up design

- A unified access and revenue cycle design
- A roadmap that fits the EHR date
- Named owners and measures
- Seven measures baselined in two weeks

FIXED POINT

EHR go-live in Q3

OUR UNDERSTANDING

Situation, complication and ask in one reading

How we read it

The complication is organizational as much as technical, so the ask has to join access, billing and the EHR plan.

WHERE CASH WAITS

52 days

in A/R, above peers, is the cost of the front-end gap

SITUATION

Where we start

- Seven hospitals, 64 clinics, three scheduling tools
- Registration varies by site
- Three acquisitions in four years

3

scheduling tools

COMPLICATION

In the way

- Front-end errors reach the claim
- Patients repeat the same details
- Access and billing split by leader

11%

of denials start at registration

THE ASK

What you need

- One access and revenue cycle design
- A roadmap that fits the EHR date
- Named owners and measures

7

measures to baseline

Source: Sample figures for layout only; replace with the client's RFP and engagement facts before use.

RFP SECTION 1.2

“We want a partner who owns the outcome, not the work plan.”

NORTHFIELD TODAY

- Seven hospitals, 64 clinics
- Three scheduling tools
- 11% of denials start at registration
- EHR go-live in Q3 next year

OUR UNDERSTANDING

You asked us to modernize patient access and the revenue cycle

Northfield wants one partner to redesign how patients are scheduled, registered and billed, and to own the result.

01 One front door

A single scheduling and registration experience across all seven hospitals and 64 clinics.

02 One revenue cycle

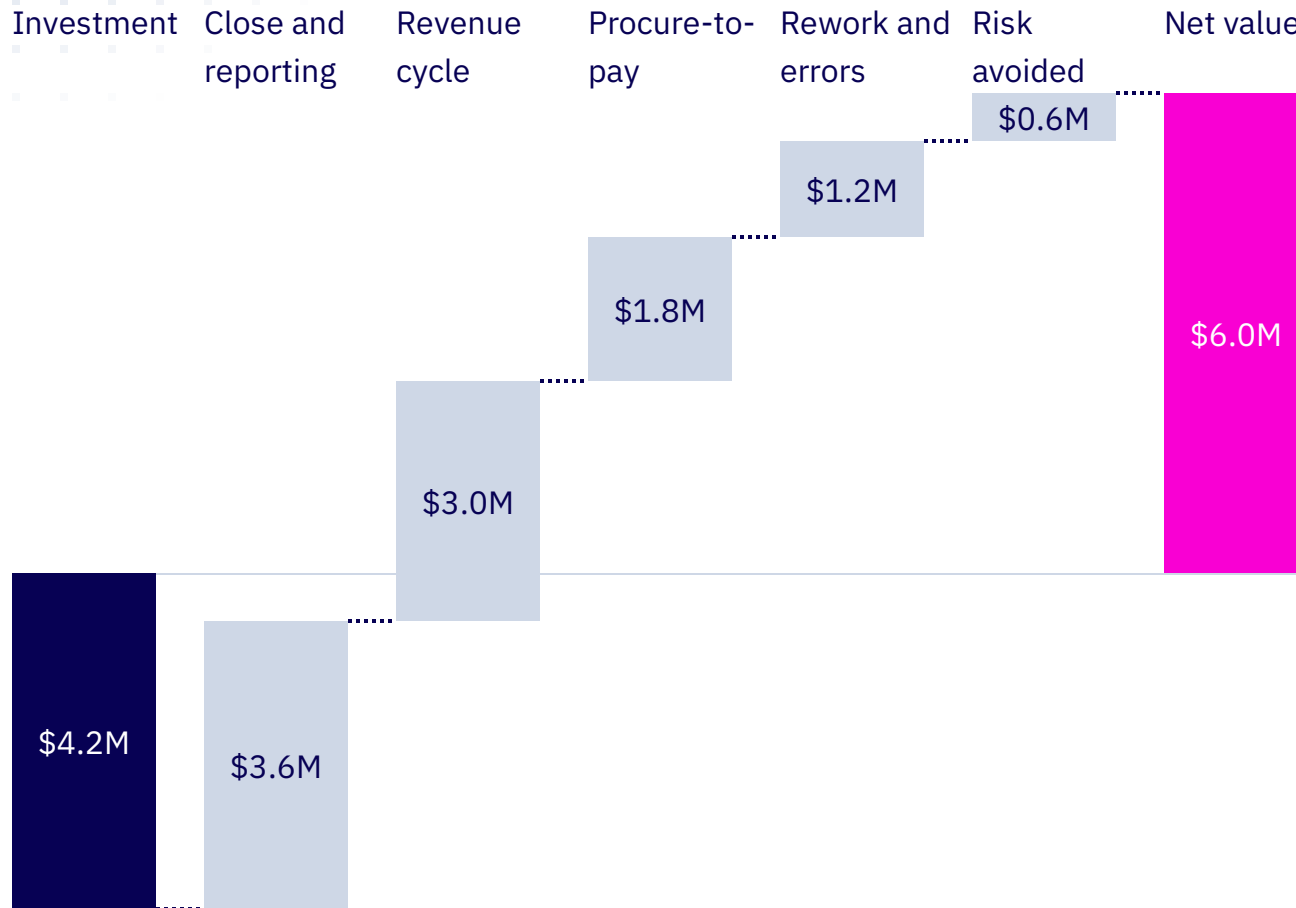
Common front-end, claims and denial processes that cut days in A/R and rework.

03 One owner

A partner accountable from design through adoption.

Source: Sample RFP language and figures for layout only; replace before use.

Five levers turn \$4.2M of spend into \$6.0M net



READING THE BRIDGE

- **Close and reporting** 35% of five-year value
- **Revenue cycle** 29%, the fastest to start
- **Procure-to-pay** 18%
- **Rework and risk** 18% together

Bars show five-year value in \$M. The investment hangs below the zero line.

2.4x 143%

value per dollar

five-year ROI

Source: Sample figures for layout only; replace with current program data before use.

INVESTMENT AND VALUE

Five levers turn \$4.2M of spend into \$6.0M net

SHARE OF FIVE-YEAR VALUE

- Close and reporting 35%
- Revenue cycle 29%
- Procure-to-pay 18%
- Rework and risk 18%

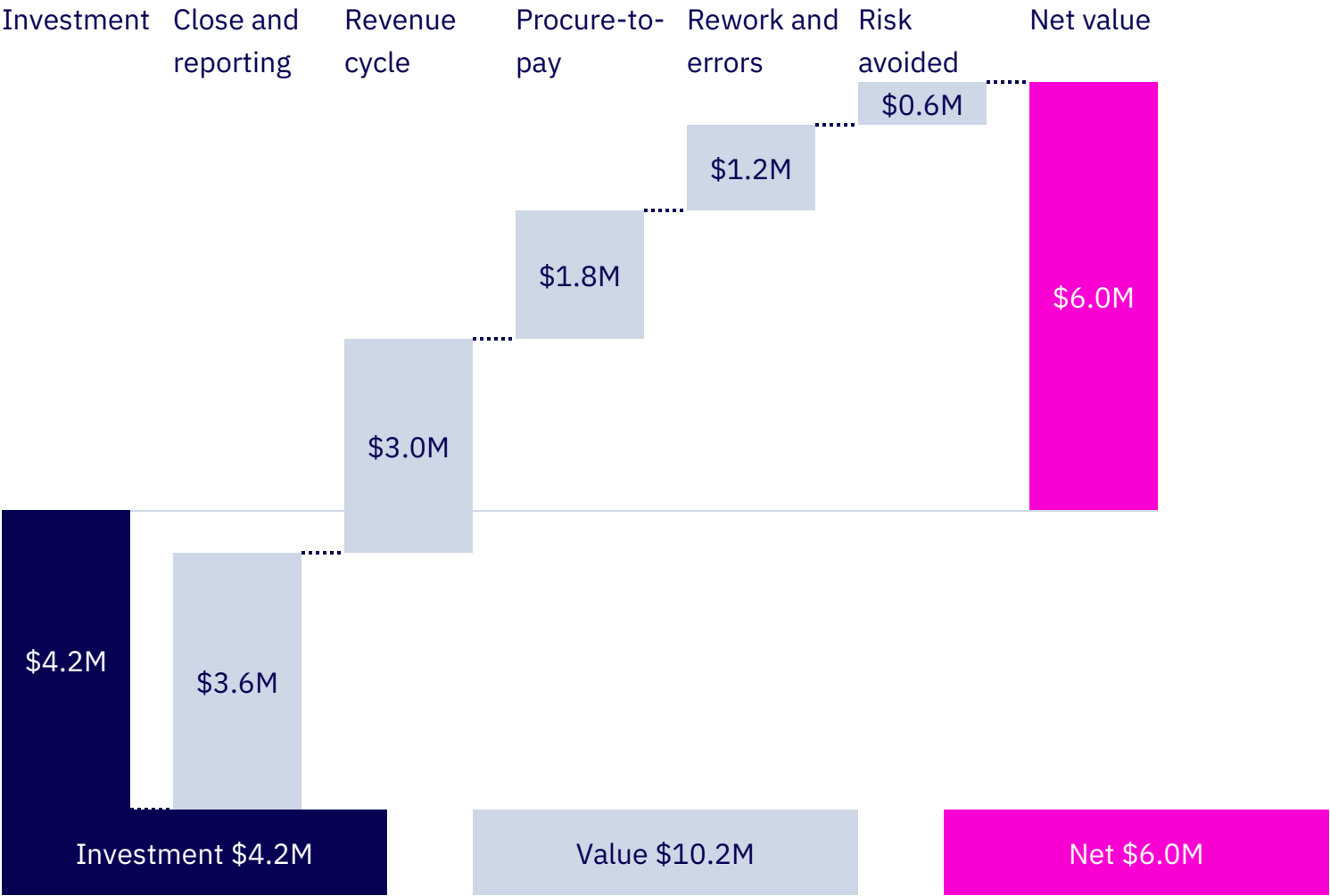
Value per dollar 2.4x

Five-year ROI 143%

Payback month 31

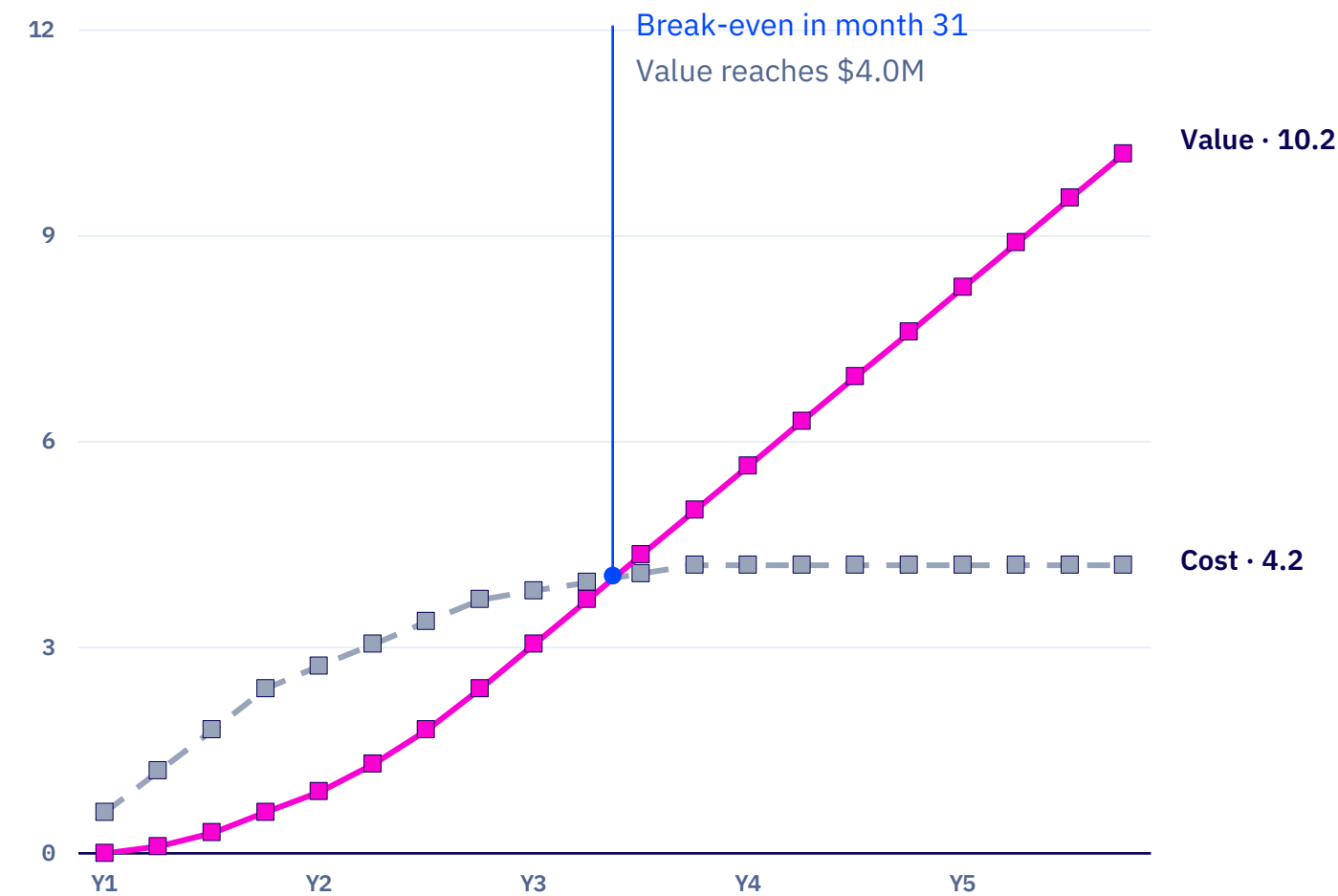
NPV at 8% \$4.1M

Source: Sample figures for layout only; replace with current firm data before use.



Cumulative cost and value

\$M, BY QUARTER, FIVE YEARS



INVESTMENT AND VALUE

Value overtakes cost in month 31

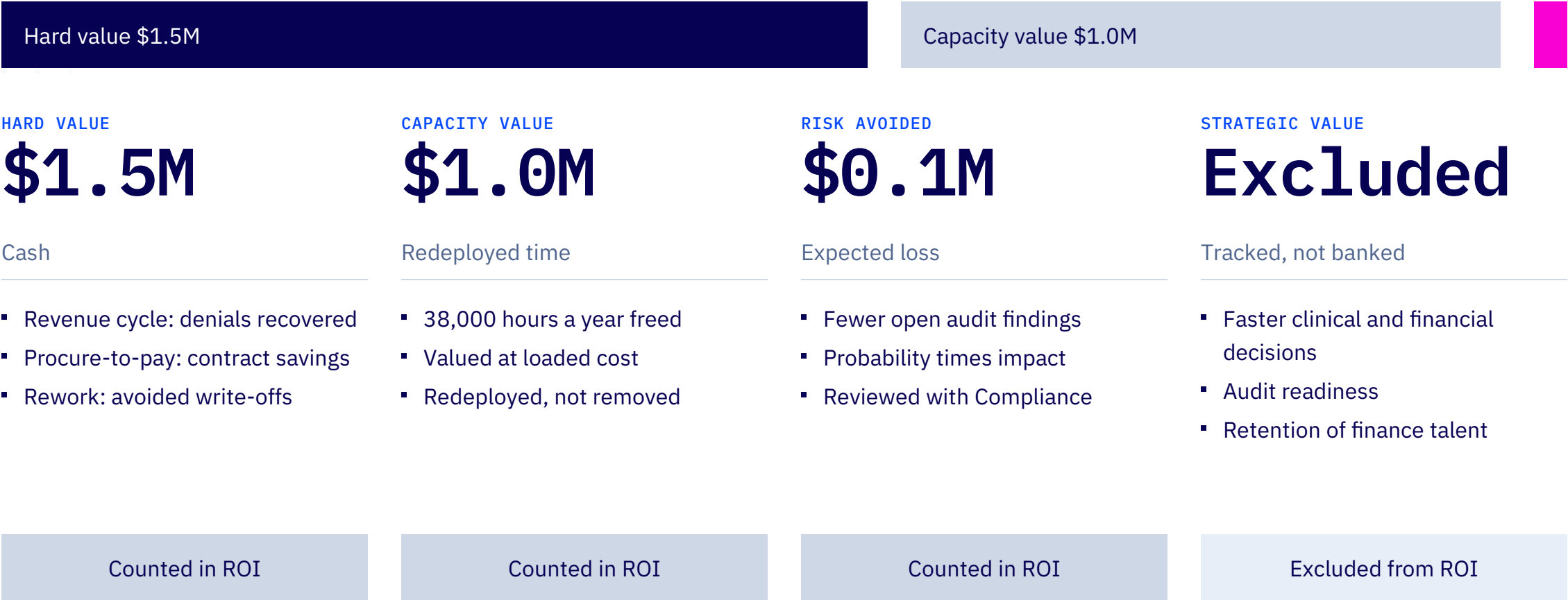
READING THE CURVES

- Spend peaks in year one at \$0.6M a quarter
- Value ramps from \$0.6M to \$2.6M a year
- Spend stops after year three

Total cost	\$4.2M
Five-year value	\$10.2M
Net value	\$6.0M
Payback	month 31

Source: Sample figures for layout only; replace with current firm data before use.

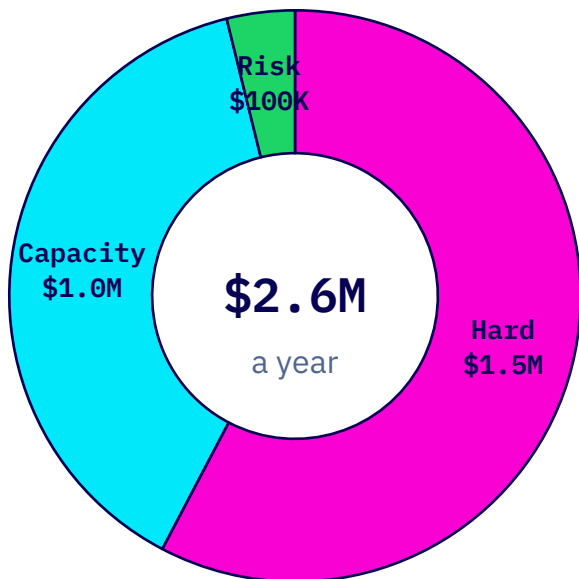
Hard and capacity value make up 96% of the case



Source: Sample figures for layout only; replace with current program data before use.

Cash and capacity are 96% of value

Annual value by type



WHAT EACH TYPE MEANS

Hard value shows up in the ledger: collected revenue, contract savings and avoided write-offs.

Capacity value is time released, valued at loaded cost and redeployed.

Risk avoided is expected loss: probability times impact.

58%

hard value

38%

capacity value

NOT IN THE CASE

- Faster clinical and financial decisions
- Audit readiness
- Retention of finance talent
- A platform for later waves

Tracked every quarter, never counted toward ROI or payback.

HOW IT IS TRACKED

A quarterly pulse survey and decision-cycle times, reported beside the value tracker.

TREATMENT

Finance validates against the ledger; Compliance reviews the risk estimate yearly.

Source: Sample figures for layout only; replace with current program data before use.

Vendor B leads the weighted scorecard

CRITERION	WEIGHT	VENDOR A	VENDOR B	VENDOR C
Functional fit	25%	4	5	3
Integration effort	20%	3	4	5
Security and compliance	20%	4	4	3
Total cost of ownership	15%	3	3	5
Vendor viability	10%	4	4	3
Delivery support	10%	2	4	3
Weighted total	100%	3.45	4.10	3.70

RECOMMENDED

RECOMMENDATION

Select Vendor B

Vendor B scores highest overall and leads on functional fit and integration.

- Confirm security controls before contract
- Negotiate the cost of ownership terms
- Pilot with one Northfield business unit

4.10

Vendor B total, out of 5

Scores run 1 to 5, with 5 best. Weights were agreed with the steering committee and total 100%.

Three steps narrowed twelve vendors to one



Each step tightens the test: must-haves first, then fit, then total value.

Three vendors, each with a clear trade-off

VENDOR A LOGO

Established suite with broad coverage

STRENGTHS

- Wide module coverage
- Large partner network

WATCH-OUTS

- Heavy configuration effort
- Slower release cadence

FIT	INDICATIVE COST
■■■□	\$\$ · 9 months

VENDOR B LOGO

RECOMMENDED

Modern platform built for fast integration

STRENGTHS

- Strong ERP integration
- Quick time to value

WATCH-OUTS

- Thinner security depth
- Smaller support team

FIT	INDICATIVE COST
■■■■	\$\$ · 6 months

VENDOR C LOGO

Low-cost option for focused needs

STRENGTHS

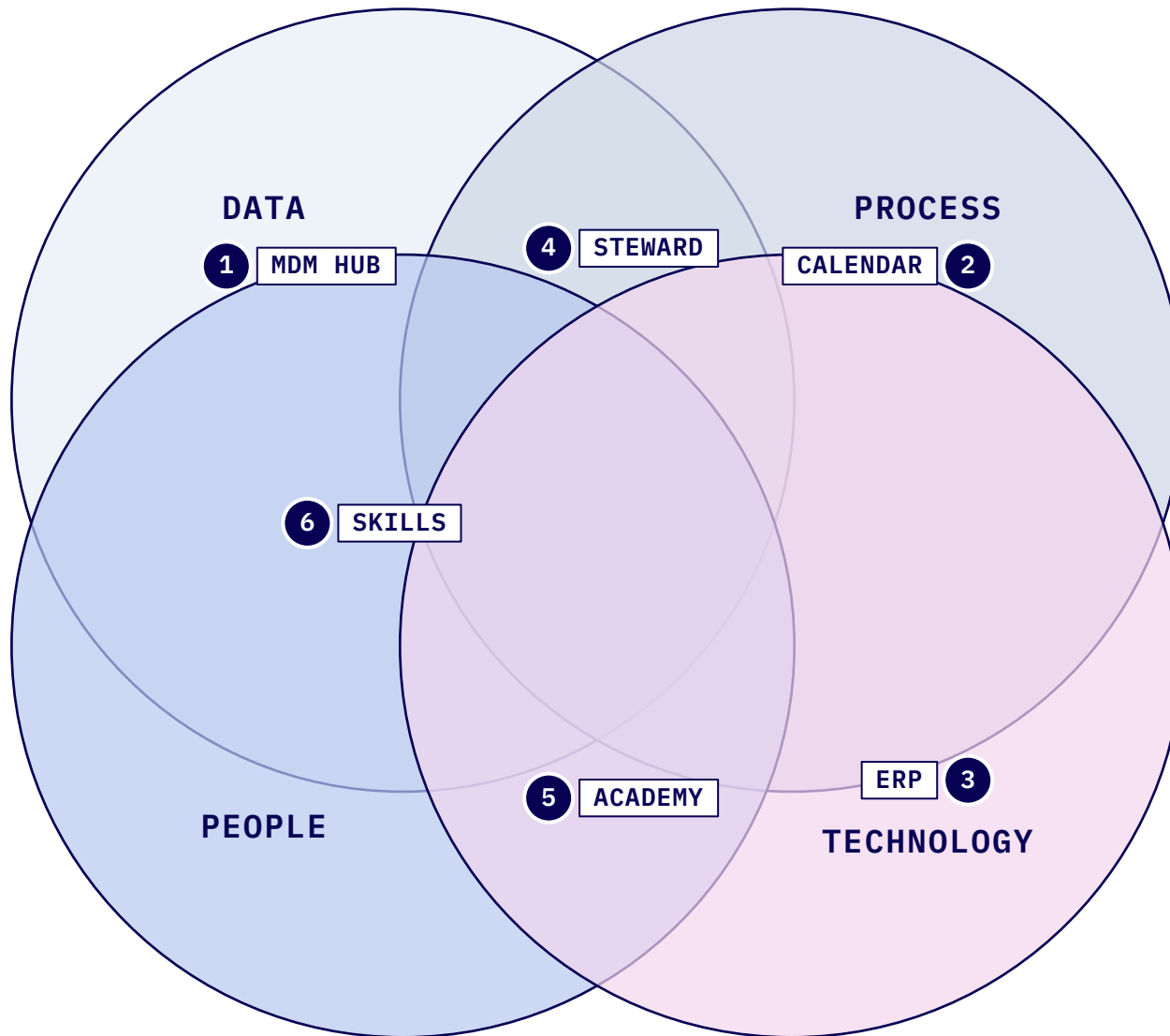
- Lowest license cost
- Simple to administer

WATCH-OUTS

- Limited workflow depth
- Few healthcare references

FIT	INDICATIVE COST
■■□□	\$ · 5 months

Six initiatives across four capabilities

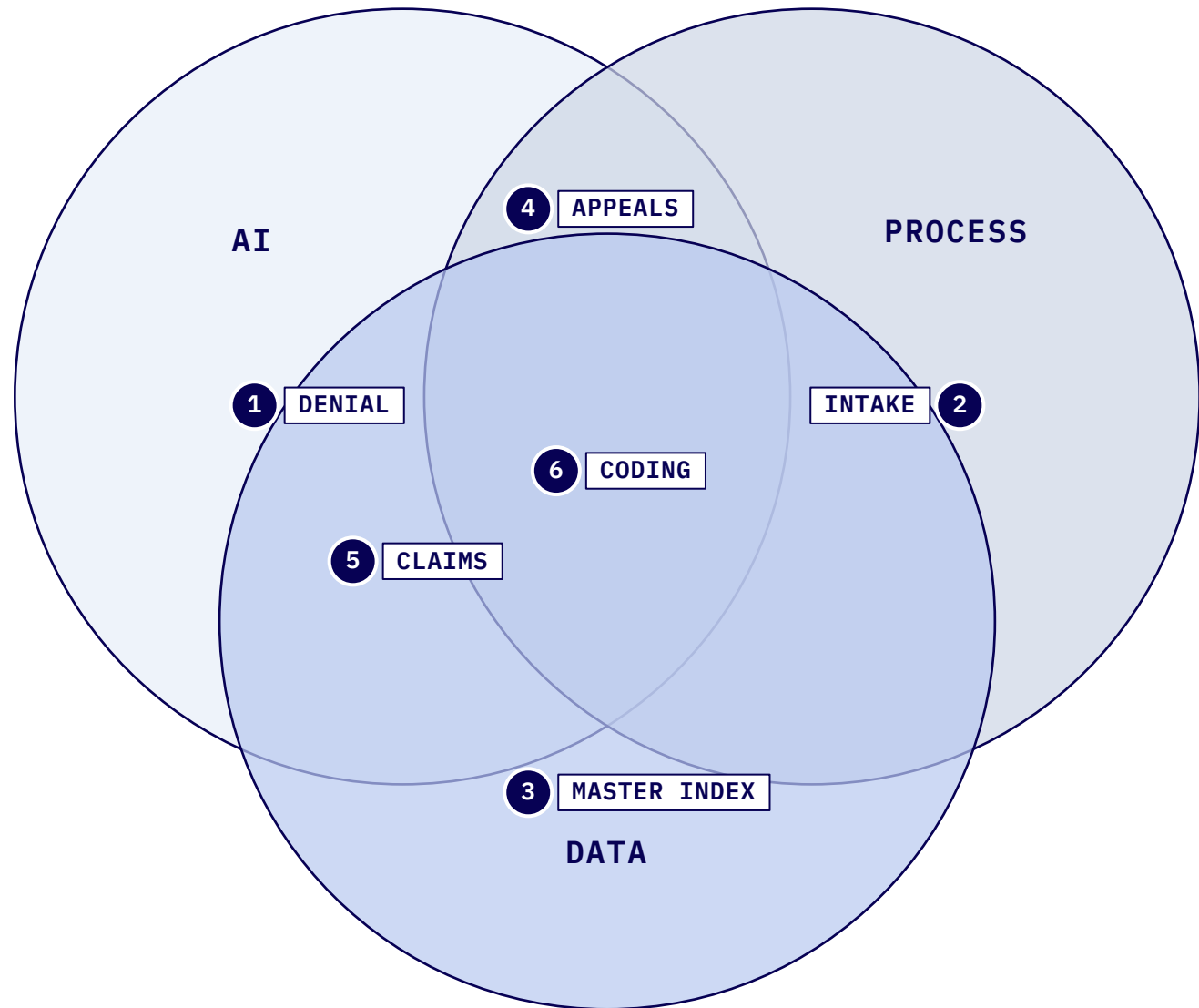


- 01 Master data hub**
Data only.
- 02 Close calendar**
Process only.
- 03 ERP upgrade**
Technology only.
- 04 Data stewardship**
Data and process.
- 05 Finance academy**
People and technology.
- 06 Skills inventory**
Data and people.

Source: Northfield finance and operations workshops, Sep 2026.

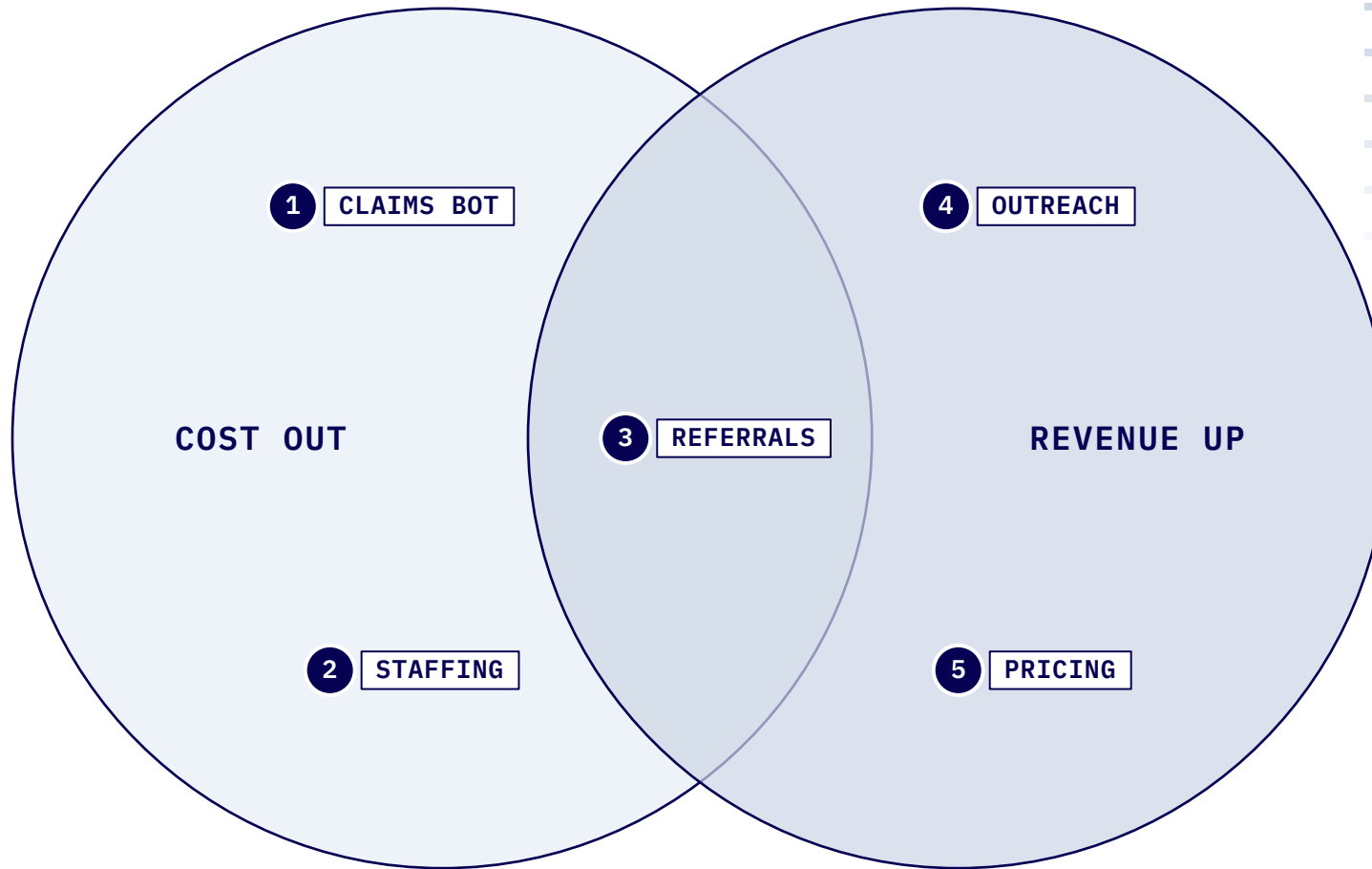
Six initiatives mapped to three capabilities

- 01 Denial prediction**
AI only.
- 02 Intake redesign**
Process only.
- 03 Patient master index**
Data only.
- 04 Appeal drafting**
AI and process.
- 05 Claims analytics**
AI and data.
- 06 Autonomous coding**
All three.



Source: Northfield finance and operations workshops, Sep 2026.

Most AI initiatives cut cost, few grow revenue



- 01 Claims status bot**
Cost out only.
- 02 Staffing forecast**
Mostly cost out.
- 03 Referral matching**
Both, at the overlap.
- 04 Outreach scoring**
Mostly revenue.
- 05 Pricing analytics**
Revenue up only.

Source: Northfield finance and operations workshops, Sep 2026.

Breakout table guide: one prompt per focus area

TABLE 1 • SCHEDULING

- **Prompt:** Draw one front door and one template for every service line.
- **Materials:** Canvas, template samples
- **Time:** 20 min · 2-minute report-back: three steps, one gap

ASK THIS TABLE _____

- Where does a request wait today?
- Which template could every clinic use?
- Who owns the no-show list?

TABLE 2 • REGISTRATION

- **Prompt:** Capture insurance and consent once, before the patient arrives.
- **Materials:** Canvas, form samples
- **Time:** 20 min · 2-minute report-back: data captured, owner

ASK THIS TABLE _____

- What do we ask that we already know?
- Who verifies, and when?
- Which consent is missed most?

TABLE 3 • ESTIMATES

- **Prompt:** Give a price a patient can rely on before they book.
- **Materials:** Canvas, price list sample
- **Time:** 20 min · 2-minute report-back: services quoted first

ASK THIS TABLE _____

- Which ten services first?
- What data is missing today?
- What happens when the price is high?

TABLE 4 • DENIALS

- **Prompt:** Stop front-end causes of denial before the claim is sent.
- **Materials:** Canvas, top denial codes
- **Time:** 20 min · 2-minute report-back: top three causes

ASK THIS TABLE _____

- Which codes recur weekly?
- Which start at the front desk?
- How fast does feedback return?

Listen for the signal, watch for the flag

SESSION OBJECTIVE

Design the future state for
scheduling, registration, estimates
and denials.

GROUND RULES

- Timeboxes hold; the parking lot takes the rest
- Every decision gets an owner and a date
- Problems before solutions
- Every table speaks at playback
- Video tables get equal airtime

	IF YOU HEAR	THEN ASK
SIGNALS	“We work around it.”	“Show me the workaround. What does it cost?”
	“That depends on the clinic.”	“Which clinics differ, and is there a reason?”
	“One person always fixes it.”	“What happens when they are away?”
RED FLAGS	“We need a new system.”	“What does the process need first?”
	“That is not our problem.”	“Who owns it, and are they here?”
	Silence or quick agreement	“What would make this fail?”
	One voice answering for all	“Let us hear from the other tables.”

Question ladder for the estimates topic

TOPIC _____

- Estimates: a price patients can rely on before they book.

IF THE ANSWER IS VAGUE _____

- “Give me one example from this week.”
- “What would a new hire see on day one?”
- “How many times a month is that?”
- “Who would answer that differently?”

RULE OF THUMB _____

- No challenge until the story is told.

Open RUNG 1

“Walk me through the last time a patient asked what a visit would cost.” Intent: get the real story, not the policy. Move on when you hear a named step.

Probe RUNG 2

“What happened next, and who did you hand it to?” Intent: trace handoffs and waiting time. Move on when each wait has an owner.

Challenge RUNG 3

“If the estimate came before booking, what would break?” Intent: test assumptions and find the true constraint. Move on when the constraint is named.

Confirm RUNG 4

“So the blocker is missing charge data. Is that right, and who owns it?” Intent: check we heard it and agree an owner. Move on when the table says yes.

SIGNS THE LADDER IS WORKING

- The table names steps, owners and numbers, not opinions
- Quiet participants are speaking by rung 3
- Disagreement is stated out loud, not smoothed over

Design by area: guide for the 60-minute segment

OBJECTIVE • 9:45 TO 10:45

Leave with a future-state sketch for each area and the gaps that block it.

TIME BOX

60 min

QUESTIONS TO ASK

- **Open:** Which step costs a patient the most time today?
- **Probe:** Where does the work wait, and who is it waiting on?
- **Probe:** What do you do when the system says no?
- **Probe:** Which step would you remove first, and what breaks?
- **Probe:** Who outside this room feels this change?
- **Probe:** How often does this happen, and how do we know?
- **Test:** If we did only this, would you stand behind it on Monday?

LISTEN FOR

- Workarounds nobody wrote down
- Handoffs with no owner
- Quick win: one fix, one owner, 30 days
- Disagreement between areas

SCRIBE CAPTURES

- Decisions, owners, open questions
- Pain points in their own words

WATCH-OUTS

- One voice filling the time
- Solutions before the pain is clear
- Drift into another area
- Quiet tables never heard

OUTPUT AT 10:45

- One canvas for each area
- A ranked list of gaps

Each day narrows the work: open up, shape, commit

OVERALL GOAL

Agree, test and commit to one future-state design for how patients schedule, register, get estimates and avoid denials.



DAY 1 • 9:00 TO 17:00

Diverge: open up the problem

- 9:00 Goals and ground rules
- 10:00 Journey mapping from every angle
- 13:00 Data, pain points, root causes
- 16:30 Playback, nothing ruled out

OUTPUT BY END OF DAY

Shared current state



DAY 2 • 9:00 TO 17:00

Converge: shape one design

- 9:00 Design principles
- 9:45 Four design tables
- 13:00 Cross-area hand-offs
- 15:00 Scenario tests, narrowing options

OUTPUT BY END OF DAY

One design



DAY 3 • 9:00 TO 15:00

Commit: decide and assign

- 9:00 Resolve the last gaps
- 10:30 Score and rank
- 13:00 Owners, dates, funding asks
- 14:00 Readout to sponsors

OUTPUT BY END OF DAY

Commitments

Discover, design and decide in three days

OVERALL GOAL

Agree, test and commit to one future-state design for how patients schedule, register, get estimates and avoid denials.

DAY 1 • TUE NOV 4

Discover

- 9:00 Welcome and goals
- 10:00 Current-state walk-through
- 13:00 Journey mapping
- 15:00 Root-cause tables

Output: validated current state

DAY 2 • WED NOV 5

Design

- 9:00 Design principles
- 9:45 Four design tables
- 13:00 Hand-offs across areas
- 15:30 Scenario tests

Output: draft future-state design

DAY 3 • THU NOV 6

Decide

- 9:00 Close the gaps
- 10:30 Score and rank
- 13:00 Roadmap and owners
- 14:00 Decisions readout

Output: ranked roadmap and decisions

BY THE END OF DAY 3

- One agreed future-state design
- Owners and dates for each area
- A ranked roadmap, signed off

Day 2 designs the future state, area by area

OVERALL GOAL

Agree and commit to one future-state design for patient access by Thursday afternoon.

THREE DAYS AT A GLANCE

- **Day 1 Discover.** Current state and pain points.
- **Day 2 Design.** Future state for each area.
- **Day 3 Decide.** Rank, assign, commit; ends 15:00.

ATTENDEES

- Access, registration, revenue cycle
- IT owners and a finance partner

DAY 2 AGENDA • 9:00 TO 17:00

- 9:00** Recap and principles
- 9:45** Design by area
- 11:30** Table playbacks
- 12:30** Working lunch
- 13:00** Cross-area hand-offs
- 15:00** Scenario tests
- 16:30** Day 2 playback
- 17:00** Close

ROLES IN THE ROOM

- Two facilitators, one scribe
- One owner per area table

OUTPUT BY END OF DAY 2

- Future-state design for each area
- Open gaps list for day 3
- Decisions log, draft

ROOM AND LOGISTICS

- Wed Nov 5, HQ room 4B
- Four tables, one per area
- Video bridge for remote leads

BEFORE DAY 3

- Sponsors hold 14:00 to 15:00
- Owners confirm their area

PATIENT ACCESS WORKSHOPS

Three days from discovery to decisions

OVERALL GOAL

Agree one future-state design for patient access and commit to who does what by when.

YOU WILL LEAVE WITH

- One design for four areas
- Owners and dates
- A ranked roadmap

LOGISTICS

- Nov 4 to 6, Northfield HQ, 4B
- Video bridge for remote leads

DAY 1 • DISCOVER • TUE NOV 4 • 9:00 TO 17:00

9:00	Welcome and goals	13:00	Data review
9:45	Current-state walk-through	15:00	Root-cause tables
11:15	Journey mapping	16:30	Playback

Output: validated current state, top pain points.

DAY 2 • DESIGN • WED NOV 5 • 9:00 TO 17:00

9:00	Design principles	13:00	Cross-area hand-offs
9:45	Design by area	15:00	Scenario tests
11:30	Table playbacks	16:30	Playback

Output: draft future-state design for four areas.

DAY 3 • DECIDE • THU NOV 6 • 9:00 TO 15:00

9:00	Close design gaps	13:00	Roadmap and owners
10:30	Score and rank	14:00	Readout to sponsors
12:30	Working lunch	15:00	Close

Output: ranked roadmap, owners and decisions.

Two days, three decisions for Northfield leaders

OVERALL GOAL

Two days to agree how patients get in, know the cost and get paid.

DAY 1 • NOV 4 • 9:00 TO 17:00

See access as patients do

- Morning: walk the patient journey
- Afternoon: review data and root causes
- 16:30: playback to the room

DAY 2 • NOV 5 • 9:00 TO 16:30

Design and decide

- Morning: design the future state by area
- Afternoon: test it on real patient cases
- 15:30: readout and decisions

DECISIONS YOU WILL BE ASKED TO MAKE

01 Front door

Which front door and booking template to adopt.

02 Ownership

Who owns each of the four areas.

03 First wave

Which five opportunities start first.

DAY 1 • NOV 4

DAY 2 • NOV 5

TIME	SESSION	WHERE	OUTPUT
9:00	Welcome and goals	In person	Validated current state
9:45	Current-state walk-through	In person	Ranked pain points
11:15	Journey mapping	In person	Journey map
13:00	Data and pain points	Remote	Root-cause list
15:00	Playback and root causes	In person	Day 1 playback
9:00	Recap and principles	Remote	Agreed principles
9:45	Design by area	In person	Four area designs
13:00	Cross-area hand-offs	Remote	Hand-off agreements
15:00	Decisions and owners	In person	Decisions log
16:00	Readout to sponsors	Hybrid	Signed-off readout

PATIENT ACCESS WORKSHOPS

Two days, in the room and on video

OVERALL GOAL

Agree and commit to one future-state design for patient access.

LOGISTICS

- In person: Northfield HQ, room 4B
- Remote: video bridge, shared board
- Hybrid blocks run both at once

WHO JOINS AND PRE-WORK

- Core group of 14, plus eight remote leads
- Test the video link by Oct 30
- Bring one failed-visit example

A half day on how patients get access

In one morning, Northfield leaders agree how patients get in, know the cost and get paid.

01 Getting patients in

Scheduling and registration: one front door and one booking template.

02 Knowing the cost

Estimates: a reliable price before the patient books.

03 Getting paid

Denials: stop front-end causes before the claim is sent.

AGENDA

- 9:00** Welcome and goals
- 9:45** Design by area
- 11:40** Decisions
- 12:00** Close

YOU WILL LEAVE WITH

- One future-state design
- Owners for each area
- A shortlist to prioritize

WHO JOINS

- Executive sponsors and area leads
- Oct 28 · half day · HQ and video

A half day to agree how patients get access

PURPOSE AND OBJECTIVES

Agree how patients will schedule, register, get estimates and avoid denials at Northfield.

- One future-state journey, end to end
- Owners named for each area
- A shortlist ready to prioritize
- Agreed focus for session 3

EXPECTED OUTCOMES

- Future-state process for each area
- Decisions log with owners
- Ranked list of 12 to 15 opportunities
- Draft target measures

AGENDA • HALF DAY

- 9:00** Welcome and goals
- 9:15** Current-state recap
- 9:45** Future-state design by area
- 10:45** Break
- 11:00** Playback and gaps
- 11:40** Decisions and next steps
- 12:00** Close

WHO SHOULD ATTEND

- Patient access and scheduling leaders
- Revenue cycle and denials managers
- Registration supervisors
- IT application owners

AREAS OF FOCUS

1. Scheduling. One front door and one template for every service line.
2. Registration. Capture insurance and consent once, before arrival.
3. Estimates. A price patients can rely on before they book.
4. Denials. Stop front-end causes before the claim is sent.

PRE-WORK

- Review the Discovery readout (10 min)
- Bring one example of a failed visit
- Confirm your area's top two pain points
- Join by Oct 21 to test video

Leave the morning with one design for patient access

PURPOSE AND OBJECTIVES

Agree how patients schedule, register, get estimates and avoid denials, with one owner for each area.

AGENDA

- 9:00** Welcome and goals
- 9:15** Current-state recap
- 9:45** Future-state design by area
- 10:45** Break
- 11:00** Playback and gaps
- 11:40** Decisions and next steps
- 12:00** Close

PRE-WORK

Read the Discovery readout.

AREAS OF FOCUS

1. Scheduling. One front door and one template for every service line.
2. Registration. Capture insurance and consent once, before arrival.
3. Estimates. A price patients can rely on before they book.
4. Denials. Stop front-end causes before the claim is sent.

WHO SHOULD ATTEND

- Patient access and scheduling
- Revenue cycle and denials
- Registration supervisors
- IT application owners

WHAT YOU WILL LEAVE WITH

Five decisions, one shared design

- A future-state process for each of the four areas
- Named owner and target measure per area
- A decisions log signed off in the room
- A ranked list of 12 to 15 opportunities
- Pre-work for the prioritization session

Oct 28 · 9:00 to 12:00 · Northfield HQ and video

Workshop 2 designs the future state of access

PURPOSE

Turn the Discovery findings into one future-state design for each access area.

LOGISTICS

- Oct 28 · 9:00 to 12:00
- Northfield HQ, room 4B, and video
- Two facilitators, one scribe

WHO SHOULD ATTEND

- Access, registration, revenue cycle
- IT application owners, finance partner

AGENDA

- 9:00** Welcome and goals
- 9:15** Current-state recap
- 9:45** Future-state design by area
- 10:45** Break
- 11:00** Playback and gaps
- 11:40** Decisions and next steps
- 12:00** Close

AREAS OF FOCUS

1. Scheduling. One front door and one template for every service line.
2. Registration. Capture insurance and consent once, before arrival.
3. Estimates. A price patients can rely on before they book.
4. Denials. Stop front-end causes before the claim is sent.

OUTCOMES

- Future-state process per area
- Decisions log with owners
- Ranked opportunity list

PRE-WORK

- Read the Discovery readout
- Bring one failed-visit example

Four tables, one morning: design the future state of access

Agree how patients will schedule, register, get estimates and avoid denials, with named owners by Oct 28.

AREAS OF FOCUS

- **Scheduling.** One front door and one template for every service line.
- **Registration.** Capture insurance and consent once, before arrival.
- **Estimates.** A price patients can rely on before they book.
- **Denials.** Stop front-end causes before the claim is sent.

WHAT YOU WILL LEAVE WITH

- Future-state process per area
- Decisions log and a ranked list

WHO ATTENDS, AND PRE-WORK

- Access, revenue cycle, IT and registration leads
- Read the Discovery readout first

- ☐ **Welcome and goals** 9:00
Objectives and ground rules.
- ☐ **Current-state recap** 9:15
What Discovery found.
- ☐ **Design by area** 9:45
Four breakout tables.
- ☐ **Playback and gaps** 11:00
Each table reports back.
- ☐ **Decisions** 11:40
Owners and next steps.
- ☐ **Close** 12:00
Confirm pre-work for session 3.

FACILITATION PLAN

Facilitation plan for workshop 2

ROLES

- Lead: runs the room and the clock
- Co-facilitator: tables and scribes
- Area facilitators: one per table

MATERIALS CHECKLIST

- Journey map, printed A0
- Canvas and sticky notes per table
- Dots, markers, name tents
- Decisions log template

GROUND RULES

- Timeboxes hold; parking lot for the rest
- Every decision gets an owner and a date

TIME	SEGMENT	ACTIVITY OR METHOD	LEAD
8:15	Room setup	Four tables, journey map on the wall, timer visible	Co-facilitator
9:00	Welcome and goals	Sponsor opens; objectives, ground rules, parking lot	Lead facilitator
9:15	Current-state recap	Journey-map walk-through; dot vote on top pain points	Lead facilitator
9:45	Design by area	Four tables, future-state canvas, 15-minute timeboxes	Area facilitators
10:45	Break	Tables reset; scribes consolidate notes	Co-facilitator
11:00	Playback and gaps	5-minute playback per table; gap callouts on the board	Co-facilitator
11:40	Decisions and next steps	Fist-to-five consensus; owners and dates	Lead facilitator
12:00	Close and debrief	Confirm pre-work; one-word check-out; team debrief	Lead facilitator

The room agreed a design; targets are still open

Covered: Discovery recap, four design tables, playback of gaps, decisions and next steps. 22 attendees, 3 hours.

KEY LEARNINGS

- 1. Patients meet three tools to book once.
- 2. Registration errors drive most denials.
- 3. Estimates are rarely given before a visit.
- 4. Leads agree on goals, not on order.

DECISIONS MADE

- One scheduling template for all
- Eligibility checked at booking
- Estimates start with imaging

OPEN QUESTIONS

- Who owns the estimate tool?
- One consent form across systems?
- Which denial codes come first?

NEXT STEPS

NEXT STEP	OWNER	DUE	STATUS
Publish the decisions log	Program office	Oct 30	In progress
Draft registration target measures	Registration lead	Nov 4	Not started
Confirm estimate data sources	Finance partner	Nov 4	Not started
Size the top denial causes	Revenue cycle	Nov 6	In progress

OUTCOMES AGAINST OBJECTIVES

OBJECTIVE	RESULT
One future-state journey	Met
Owners named per area	Partly met
Shortlist ready to rank	Met
Target measures drafted	Not met

Source: Sample figures for layout only; replace with current firm data before use.

Two opportunities go to Now after the dot vote

OPPORTUNITY	VOTES (OF 5)	PRIORITY	OWNER
Eligibility check at booking	● ● ● ● ●	NOW	Registration lead
One scheduling template	● ● ● ● ●	NOW	Access director
Pre-visit estimate for imaging	● ● ● ● ●	NEXT	Finance partner
Shared consent capture	● ● ● ● ●	NEXT	Compliance lead
Denial root-cause dashboard	● ● ● ● ●	NEXT	Revenue cycle
Real-time insurance cross-check	● ● ● ● ●	NEXT	IT application owner
Self-service rebooking	● ● ● ● ●	LATER	Digital lead

KEY LEARNINGS

Front-end fixes drive most value; estimates and denials depend on the same registration data.

Source: Sample figures for layout only; replace with current firm data before use.

OUTCOMES AGAINST OBJECTIVES

OBJECTIVE	RESULT
One future-state journey	Met
Owners named per area	Partly met
Shortlist ready to rank	Met
Target measures drafted	Not met

DECISIONS AND OPEN QUESTIONS

- Decided: top two go to Now
- Decided: estimates start with imaging
- Open: who owns the estimate tool?
- Open: consent form scope

KEY LEARNINGS

- 1. Patients meet three scheduling tools before they can book one visit.
- 2. Registration errors cause most of the front-end denials we see.
- 3. Estimates are rarely given before the visit, so patients arrive unsure of cost.
- 4. Area leads agree on the goals but not yet on the order of work.
- 5. Insurance checks run too late in the flow to prevent a denial.

OPEN QUESTIONS

- Who owns the estimate tool?
- One consent form across systems?
- Which denial codes come first?

WHAT WE COVERED

- Discovery recap and four design tables
- Playback, gaps and decisions

NEXT STEPS

NEXT STEP	OWNER	DUE	STATUS
Publish the decisions log	Program office	Oct 30	In progress
Draft registration target measures	Registration lead	Nov 4	Not started
Confirm estimate data sources	Finance partner	Nov 4	Not started

WORKSHOP 2 READOUT

Five learnings shape the design

DECISIONS MADE

- One scheduling template
- Eligibility checked at booking
- Estimates start with imaging

OUTCOMES AGAINST OBJECTIVES

OBJECTIVE	RESULT
One future-state journey	Met
Owners named per area	Partly met
Shortlist ready to rank	Met
Target measures drafted	Not met

Source: Sample figures for layout only; replace with current firm data before use.

In their words: front-end fixes come first

“ “ “

One phone number to book one visit.

DIRECTOR, PATIENT ACCESS

KEY LEARNINGS

- 1. Patients meet three tools to book once.
- 2. Registration errors drive most denials.
- 3. Estimates are rarely given before a visit.
- 4. Leads agree on goals, not on order.

A wrong estimate becomes our denial.

REVENUE CYCLE MANAGER

OUTCOMES AGAINST OBJECTIVES

OBJECTIVE	RESULT
One future-state journey	 Met
Owners named per area	 Partly met
Shortlist ready to rank	 Met
Target measures drafted	 Not met

Registration is where claims are won.

REGISTRATION SUPERVISOR

DECISIONS AND NEXT STEPS

- **Decided.** One scheduling template.
- **Oct 30.** Program office publishes the log.
- **Nov 4.** Registration lead drafts measures.
- **Open.** Who owns estimates?

Source: Sample quotes for layout only; replace with comments from the session before use.

Each workshop has a focus, a room and an output

OCT 14 • HALF DAY

01 Discovery

Baseline scheduling, registration, estimates and denials.

Who: 18 access, revenue cycle and IT leads.

Output: current-state map and pain-point list.

- Booking and intake walk-through
- Estimate and denial data review

OCT 28 • HALF DAY

02 Future state

Design the target process for each of the four areas.

Who: 22 area leads and IT owners.

Output: future-state design and decisions log.

- Four breakout design tables
- Playback and decisions

NOV 11 • HALF DAY

03 Prioritization

Score and rank opportunities by value and effort.

Who: 14 sponsors and area leads.

Output: ranked list in Now, Next, Later.

- Value and effort scoring
- Dot vote and Now, Next, Later

NOV 18 • HALF DAY

04 Roadmap

Sequence the work, owners and funding asks.

Who: 12 sponsors and finance.

Output: 18-month roadmap and funding request.

- Sequencing and dependencies
- Owners and funding asks

Source: Sample figures for layout only; replace with current firm data before use.

Five workstreams take the platform from project to service

01 Define the service catalog

Name who the platform serves and what it offers, so every team knows what is central and what is theirs.

KEY RISKS

- The platform becomes an IT silo
- Catalog goes stale within months

02 Set the standards

Agree the patterns, workspace rules and data sharing standards that every environment follows.

KEY RISKS

- Standards are ignored in practice
- Exceptions outnumber patterns

03 Industrialize delivery

Automate provisioning, release and monitoring so a new workload goes live without a project.

KEY RISKS

- It stays a project, not a service
- Manual steps slow every launch

04 Drive adoption and ownership

Open intake, training and a self-service path, then hand day-to-day ownership to Northfield teams.

KEY RISKS

- Adoption stalls at capacity limits
- Owners are never named

05 Prove value and cost

Baseline what workloads cost today, track savings against plan and report value to sponsors.

KEY RISKS

- Costs rise faster than usage
- Value is never shown to funders

Design general availability in from the start

